PSYCHIATRY PRACTICE HANDBOOK

# 550

## BOARD-STYLE PRACTICE QUESTIONS

## The Online Question Bank

Five options. One best answer.
A full explanation for every question.
Companion to the 3rd Edition.

MARCUS T. HALLBROOK

# How to Use This Bank

This file holds 550 board-style practice questions that go with the *Psychiatry Practice Handbook, 3rd Edition*. Every question has five options and one best answer. The questions follow the format of the book′s chapter-end questions and cover its 25 chapters.

- **Open this file in your browser or PDF app.** The buttons and links jump between pages. They work in Chrome, Safari, Edge, and Acrobat. Some in-app viewers, such as a Google Drive preview, do not support them.
- **Start at the contents.** Each row jumps to the questions or the answers for one subject.
- **Answer a question, then press Answer.** The button under each question jumps to its explanation. Press Back to question to return.
- **Use the footer.** The line at the foot of every page links to Contents, Questions, Answers, and How to Use.
- **Read the explanation, not only the letter.** Each one says why the best answer is right and why the others are not.
- **Check before you act.** The questions are for study. Confirm any dose or threshold against the current label before using it in practice.

This bank is an independent educational work. It is not affiliated with, endorsed by, or sponsored by the American Board of Psychiatry and Neurology. ABPN is a registered mark of its owner and is used only to describe the content outline the questions follow. The questions are original and are not actual examination questions.

# Contents


## PART I. ASSESSMENT FOUNDATIONS

| | | | |
|---|---|---|---|
| 1 | The Psychiatric Interview, Mental Status Examination, and Formulation<br>BQ-001 to BQ-016 | Questions p.6 | Answers p.215 |
| 2 | Diagnosis, Rating Scales, and the Medical Workup<br>BQ-017 to BQ-038 | Questions p.12 | Answers p.219 |
| 3 | Neuroscience and Mechanisms of Disease<br>BQ-039 to BQ-065 | Questions p.20 | Answers p.224 |

## PART II. TREATMENT

| | | | |
|---|---|---|---|
| 4 | Principles of Psychopharmacology<br>BQ-066 to BQ-087 | Questions p.31 | Answers p.230 |
| 5 | Antidepressants, Anxiolytics, and Hypnotics<br>BQ-088 to BQ-112 | Questions p.39 | Answers p.235 |
| 6 | Antipsychotics, Mood Stabilizers, and Other Agents<br>BQ-113 to BQ-137 | Questions p.48 | Answers p.241 |
| 7 | Psychotherapies<br>BQ-138 to BQ-159 | Questions p.58 | Answers p.246 |
| 8 | Neuromodulation and Interventional Psychiatry<br>BQ-160 to BQ-175 | Questions p.66 | Answers p.251 |

## PART III. MOOD, ANXIETY, AND TRAUMA

| | | | |
|---|---|---|---|
| 9 | Depressive Disorders<br>BQ-176 to BQ-202 | Questions p.72 | Answers p.255 |
| 10 | Bipolar and Related Disorders<br>BQ-203 to BQ-224 | Questions p.82 | Answers p.261 |
| 11 | Anxiety and Obsessive-Compulsive Disorders<br>BQ-225 to BQ-251 | Questions p.90 | Answers p.266 |
| 12 | Trauma, Stressor, and Dissociative Disorders<br>BQ-252 to BQ-273 | Questions p.100 | Answers p.272 |

## PART IV. PSYCHOSIS, SUBSTANCE USE, AND PERSONALITY

| | | | |
|---|---|---|---|
| 13 | Schizophrenia Spectrum and Other Psychotic Disorders<br>BQ-274 to BQ-300 | Questions p.108 | Answers p.277 |

| | | | |
|---|---|---|---|
| **14** | Substance-Related and Addictive Disorders<br>BQ-301 to BQ-327 | **Questions p.118** | **Answers p.282** |
| **15** | Personality, Impulse-Control, and Conduct Disorders<br>BQ-328 to BQ-349 | **Questions p.128** | **Answers p.287** |
| **PART V. BRAIN, BODY, AND BEHAVIOR** | | | |
| **16** | Neurocognitive Disorders and Delirium<br>BQ-350 to BQ-374 | **Questions p.136** | **Answers p.291** |
| **17** | Neurologic Disorders in Psychiatric Practice<br>BQ-375 to BQ-396 | **Questions p.145** | **Answers p.296** |
| **18** | Somatic Symptom and Eating Disorders<br>BQ-397 to BQ-418 | **Questions p.153** | **Answers p.301** |
| **19** | Sleep-Wake, Sexual, Gender, and Paraphilic Disorders<br>BQ-419 to BQ-437 | **Questions p.162** | **Answers p.306** |
| **PART VI. ACROSS THE LIFE CYCLE** | | | |
| **20** | Child and Adolescent Psychiatry<br>BQ-438 to BQ-461 | **Questions p.169** | **Answers p.310** |
| **21** | Perinatal, Reproductive, and Geriatric Psychiatry<br>BQ-462 to BQ-477 | **Questions p.178** | **Answers p.315** |
| **PART VII. EMERGENCIES, MEDICAL SETTINGS, LAW, AND PRACTICE** | | | |
| **22** | Suicide, Violence, and Psychiatric Emergencies<br>BQ-478 to BQ-499 | **Questions p.186** | **Answers p.319** |
| **23** | Consultation-Liaison Psychiatry<br>BQ-500 to BQ-518 | **Questions p.195** | **Answers p.324** |
| **24** | Forensic Psychiatry, Ethics, and the Law<br>BQ-519 to BQ-534 | **Questions p.203** | **Answers p.328** |
| **25** | Professionalism, Communication, Cultural Care, and Systems of Practice<br>BQ-535 to BQ-550 | **Questions p.209** | **Answers p.332** |

SUBJECT 1

# The Psychiatric Interview, Mental Status Examination, and Formulation

Questions BQ-001 to BQ-016

**BQ-001**

**A 72-year-old man is brought by his wife for forgetfulness. On a brief screen he recalls two of three words after the clock task, and his clock has all the numbers in the right places but both hands point to the wrong times. What is the best interpretation?**

**A.** Negative screen, so cognitive impairment is excluded

**B.** Positive screen; a fuller evaluation is indicated

**C.** Established diagnosis of Alzheimer disease

**D.** Delirium, because the clock was abnormal

**E.** Normal aging, so no further evaluation is needed

Answer Contents

**BQ-002**

**A 62-year-old woman with 10 years of formal education scores 25 on the Montreal Cognitive Assessment. Using the original scoring, which statement is most accurate?**

**A.** The score of 25 is the final score and is normal

**B.** The score is invalid because of her education

**C.** One point is subtracted, giving a final score of 24

**D.** One point is added, giving 26, which is normal

**E.** The test cannot be scored without an interpreter

Answer Contents

**BQ-003**

**A 29-year-old man says that someone is putting thoughts into his head that are not his own. He is not intoxicated and is alert. Which term describes this symptom?**

**A.** Thought insertion
**B.** Thought broadcasting
**C.** Thought blocking
**D.** Obsession
**E.** Flight of ideas

Answer Contents

**BQ-004**

**A 41-year-old woman laughs and smiles while she describes the death of her mother last month. Her mood, in her words, is "very sad." Which description of her affect is most accurate?**

**A.** Blunted
**B.** Flat
**C.** Labile
**D.** Constricted
**E.** Incongruent

Answer Contents

**BQ-005**

**A hospitalized 68-year-old man tells the night nurse that a stranger is standing in his room. In the morning light he sees that it is a coat hanging on the door and laughs. Which term best fits the night experience?**

**A.** Auditory hallucination
**B.** Delusion of reference
**C.** Illusion
**D.** Depersonalization
**E.** Pseudologia fantastica

Answer Contents

**BQ-006**

**A healthy 24-year-old student hears his name called just as he falls asleep. He has no other symptoms, uses no substances, and sleeps well. What is the best interpretation?**

**A.** Early schizophrenia, requiring antipsychotic treatment

**B.** A hypnagogic experience that is a normal variant

**C.** Command hallucinations

**D.** Temporal lobe epilepsy

**E.** Alcohol withdrawal

Answer Contents

**BQ-007**

**A 33-year-old woman is distressed by repeated thoughts that she may have harmed someone while driving. She knows the thoughts are her own and excessive, tries to resist them, and checks the road behind her. Which term best describes the thoughts?**

**A.** Delusion

**B.** Overvalued idea

**C.** Phobia

**D.** Thought insertion

**E.** Obsession

Answer Contents

**BQ-008**

**A 36-year-old man is brought in with rapid, pressured speech. He changes topics before finishing sentences and says he needs no sleep. Which interviewing approach is best?**

**A.** Use short, focused questions, redirect gently, and obtain collateral

**B.** Ask long open-ended questions and wait in silence until he slows down

**C.** Challenge his statements about sleep with a detailed medical explanation

**D.** Conduct the interview in a busy area so he is not isolated

**E.** Postpone all questions until he agrees that he is ill

Answer Contents

**BQ-009**

**A 60-year-old woman has not spoken for two days. When her arm is raised she holds it in the air for minutes and resists being repositioned. Vital signs are normal. What is the best next step?**

**A.** Start a high-dose antipsychotic at once to treat the mutism

**B.** Label the behavior as malingering and discharge her

**C.** Wait a week for spontaneous improvement before acting

**D.** Examine for catatonia and search for a medical cause

**E.** Begin insight-oriented psychotherapy twice a week

Answer Contents

**BQ-010**

**A 9-year-old boy is described by his parents as well behaved but by his teacher as disruptive and inattentive. Which approach to assessment is best?**

**A.** Rely on the parents, who know him best at home

**B.** Rely on a single rating scale completed at home

**C.** Gather information from child, parents, and school

**D.** Interview only the teacher who sees him daily

**E.** Defer assessment until he is older and can explain

Answer Contents

**BQ-011**

**An 82-year-old woman with hearing loss gives answers unrelated to your questions and seems confused. Before concluding that she has cognitive impairment, what is the best first step?**

**A.** Start a cholinesterase inhibitor for presumed dementia

**B.** Ensure aids are in use, reduce noise, and repeat the questions

**C.** Obtain an immediate brain scan to look for a lesion

**D.** Diagnose delirium on the basis of the unrelated answers

**E.** Stop the interview and call her family to explain

Answer Contents

**BQ-012**

**A 55-year-old man refuses to let you speak with his wife. His wife telephones to tell you he has been drinking heavily and talking about harming himself. What is the best response?**

**A.** Refuse to listen, because she is not the patient and privacy rules forbid it

**B.** Tell her everything in his chart so that she will cooperate with you

**C.** Ignore her information because collateral is generally unreliable

**D.** Ask her to sign a statement promising that she will keep him safe

**E.** Listen and record, but share information only as law or consent allows

Answer Contents

**BQ-013**

**A 26-year-old woman with a history of assault becomes silent and distant when you close the door and ask for details of the event. What is the most appropriate response?**

**A.** Slow down, offer choices, and avoid pressing for details

**B.** Continue with detailed questions to complete the history

**C.** Tell her that she must answer all questions to receive care

**D.** End the visit and tell her to return when she is ready to talk

**E.** Ask her partner to answer the questions in her place

Answer Contents

**BQ-014**

**In the Cultural Formulation Interview, which section covers what the patient has already tried and who, including a religious or traditional healer, has helped before?**

**A.** Cultural definition of the problem

**B.** Perceptions of cause, context, and support

**C.** Current help-seeking

**D.** Self-coping and past help-seeking

**E.** The role of the clinician's own culture

Answer Contents

**BQ-015**

**A trainee writes only "major depressive disorder" as the entire assessment. Which statement best describes what a case formulation would add?**

**A.** A list of every symptom the patient has ever reported

**B.** A second diagnosis chosen from the same manual

**C.** An explanation of why this person has this problem now

**D.** A prediction of the exact date of recovery

**E.** A summary of the insurance coverage available

Answer Contents

**BQ-016**

**A patient tells you he has thoughts of seriously harming his former business partner. According to the APA evaluation guideline, which additional element should you assess?**

**A.** His religious affiliation and frequency of worship

**B.** His level of education and academic record

**C.** His employment history and current job title

**D.** His marital status and number of children

**E.** Access to firearms and any identified targets

Answer Contents

SUBJECT 2

## Diagnosis, Rating Scales, and the Medical Workup

Questions BQ-017 to BQ-038

**BQ-017**

**A 45-year-old man completes the 10-item Alcohol Use Disorders Identification Test in primary care and scores 18. In which World Health Organization risk zone does this score fall?**

**A.** Zone I

**B.** Zone III

**C.** Zone II

**D.** Zone IV

**E.** The test assigns no zones

Answer Contents

**BQ-018**

**A 16-year-old answers no to all three opening questions about alcohol, cannabis, and other drug use in the past 12 months. The clinician then asks the car question, and she says she rode with a driver who had been using. Which is the correct way to proceed?**

**A.** Ask all six CRAFFT items before assigning a score

**B.** Record a positive screen, since any yes to the car question counts

**C.** Add one point for each opening question to reach a score of 2

**D.** Ask only the alcohol-specific items next, then score

**E.** Stop after the car question; the score of 1 is below the cutoff

Answer Contents

**BQ-019**

**Six weeks after delivery, a woman completes the Edinburgh Postnatal Depression Scale and scores above the usual cutoff. Which statement is most accurate?**

**A.** The score flags possible depression; assess clinically, including self-harm

**B.** The score establishes major depressive disorder and the need for medication

**C.** The scale reliably separates depression from posttraumatic stress disorder

**D.** The scale replaces the clinical interview in perinatal care settings

**E.** A score above cutoff means treatment should start without further assessment

Answer Contents

**BQ-020**

**A 38-year-old veteran completes the PTSD Checklist for DSM-5 and scores 33 out of a possible 80. Which interpretation is best?**

**A.** The score is below every proposed cutoff for the checklist

**B.** The scale has a possible range of 0 to 100 points

**C.** The score confirms the diagnosis of posttraumatic stress disorder

**D.** The score is in the cutoff range; arrange a structured assessment

**E.** The checklist requires a license fee before it can be scored

Answer Contents

**BQ-021**

**A hospital system wants to adopt the Columbia Suicide Severity Rating Scale and asks the trainer which total score defines high risk. What is the best answer?**

**A.** A total score of 4 or more on the six-question screener

**B.** Any yes answer to the first question on the screener

**C.** There is no total score; organizations set their own thresholds

**D.** A total score of 10 or more summed across all items

**E.** High risk is defined only by a licensed psychologist's rating

Answer Contents

**BQ-022**

**A 68-year-old retired lawyer with 18 years of formal education has a Montreal Cognitive Assessment score of 25. Which statement is most accurate?**

**A.** No education point is added, so 25 falls in the impaired range

**B.** One point is added for age, giving a normal score of 26

**C.** The score is normal because 25 is within one point of the cutoff

**D.** The score cannot be used without a certified rater

**E.** Two points are subtracted for advanced education, giving 23

Answer Contents

**BQ-023**

**A hospitalized 80-year-old man has an acute change from baseline that fluctuates through the day, and he cannot sustain attention. He is alert, and his speech is organized. What is the result on the confusion assessment method algorithm?**

**A.** Positive, because acute onset plus inattention is sufficient

**B.** Positive, because inattention alone is sufficient for delirium

**C.** Negative; no disorganized thinking and no altered consciousness

**D.** Positive, because acute onset alone is sufficient for delirium

**E.** The algorithm cannot be applied to hospitalized patients

Answer Contents

**BQ-024**

**A 34-year-old woman scores 12 on the GAD-7. Which statement is most accurate?**

**A.** The score is below the threshold for further evaluation

**B.** The score is in the severe range, above the cut point of 15

**C.** The score establishes the diagnosis of generalized anxiety disorder

**D.** The scale ranges from 0 to 28, so 12 is a low score

**E.** Moderate range and above the cutoff of 10, so interview next

Answer Contents

**BQ-025**

**A 30-year-old man with no known risk factors presents with a first episode of psychosis. His workup is under way. Which statement about HIV testing is most consistent with United States Preventive Services Task Force guidance?**

**A.** HIV testing is recommended only for persons who report injection drug use

**B.** One-time screening is recommended at ages 15 to 65, which fits here

**C.** HIV testing is unnecessary without a history of sexually transmitted infection

**D.** HIV testing should wait until the psychosis has resolved fully

**E.** HIV testing is recommended only for adults older than age 65

Answer Contents

**BQ-026**

**A 58-year-old man has eight months of personality change and a new cranial nerve palsy. Treponemal and nontreponemal serologic tests are both reactive. What is the best next step?**

**A.** Start an antidepressant and reassess in four weeks

**B.** Repeat the serologies in six months

**C.** Treat as latent syphilis without further testing

**D.** Examine the cerebrospinal fluid before treatment

**E.** Obtain a PHQ-9 and defer other testing

Answer Contents

**BQ-027**

**A 78-year-old woman has three months of apathy, withdrawal, weight loss, and a fine tremor. Her PHQ-9 is in the moderate range. Which test should be ordered first?**

**A.** TSH

**B.** Urine drug screen

**C.** Head MRI

**D.** EEG

**E.** HIV test

Answer Contents

**BQ-028**

**A 45-year-old man taking prednisone for three weeks develops insomnia, elation, and pressured speech. He has no prior psychiatric history. What is the most appropriate first step?**

**A.** Diagnose bipolar I disorder and begin lithium treatment now

**B.** Diagnose panic disorder and start a scheduled benzodiazepine

**C.** Suspect a steroid effect and call the prescriber about the dose

**D.** Stop prednisone abruptly without consulting the prescriber

**E.** Reassure him that this is expected and take no action

Answer Contents

**BQ-029**

**A 12-year-old with asthma develops vivid nightmares, irritability, and aggression three weeks after starting montelukast. Which response is most appropriate?**

**A.** Attribute the symptoms to adolescence and observe for several months

**B.** Start an SSRI and continue montelukast at the same dose

**C.** Order a head MRI and a sleep study before any other step

**D.** Diagnose oppositional defiant disorder and refer for parent training

**E.** Suspect the boxed-warning drug effect and call the prescriber

Answer Contents

**BQ-030**

**A 9-year-old started on levetiracetam for epilepsy becomes irritable and aggressive within weeks. Which label finding is most relevant?**

**A.** Behavioral effects on the label are limited to adults

**B.** At ages 4 to 16, 38% on drug versus 19% on placebo had symptoms

**C.** Behavioral effects occur at the same rate as with placebo

**D.** The label reports no psychiatric or behavioral effects

**E.** Behavioral effects occur only above the maximum dose

Answer Contents

**BQ-031**

**A patient who has not responded to other antipsychotics has a baseline QTc of 470 msec. A colleague proposes thioridazine. What does the label indicate?**

**A.** Start at half the usual dose and recheck the ECG in one year

**B.** Proceed, because only a QTc above 500 msec matters here

**C.** Proceed if the potassium and magnesium levels are normal

**D.** Do not give it; a QTc above 450 msec precludes thioridazine

**E.** Combine it with a CYP2D6 inhibitor to lower the dose

Answer Contents

**BQ-032**

**A 60-year-old man with schizophrenia had a myocardial infarction three weeks ago. A resident proposes ziprasidone. What does the label state?**

**A.** It is contraindicated with a recent acute myocardial infarction

**B.** It is preferred because it has no effect on the QT interval

**C.** It is allowed if the ECG is repeated every week afterward

**D.** It is allowed if given with another QT-prolonging drug

**E.** It is contraindicated only for patients under age 18

Answer Contents

**BQ-033**

**A 35-year-old woman who takes a pseudoephedrine product has a positive amphetamine immunoassay. She denies stimulant use. What is the best next step?**

**A.** Report illicit stimulant use to the treating team

**B.** Discharge her from the program for stimulant use

**C.** Request confirmatory mass spectrometry before acting

**D.** Repeat the same immunoassay and act on two positives

**E.** Assume the sample was switched by the patient

Answer Contents

**BQ-034**

**In a 2023 meta-analysis of neuroimaging in first-episode psychosis, about what proportion of patients had a clinically relevant abnormality, meaning one that changed diagnosis or management?**

**A.** About 1%

**B.** About 6%

**C.** About 26%

**D.** About 50%

**E.** About 90%

Answer Contents

**BQ-035**

**Which presentation most clearly calls for an EEG?**

**A.** A 30-year-old with a first episode of major depression

**B.** A 25-year-old with generalized anxiety and normal examination

**C.** A 35-year-old with attention-deficit/hyperactivity disorder

**D.** A 50-year-old with uncomplicated alcohol use disorder

**E.** A 40-year-old with staring spells and fluctuating confusion

Answer Contents

**BQ-036**

**In the DSM-5 field trials of test-retest reliability, which diagnosis had the highest pooled kappa among the following?**

**A.** Major depressive disorder

**B.** Generalized anxiety disorder

**C.** Schizophrenia

**D.** Posttraumatic stress disorder

**E.** Bipolar I disorder

Answer Contents

**BQ-037**

**A man with long-standing schizophrenia reports chest pain, and the team attributes it to anxiety without an examination. Which reasoning error is this?**

**A.** Diagnostic overshadowing

**B.** Cross-sectional labeling

**C.** Confirmation of a screening score

**D.** Premature closure on a medication cause

**E.** Base rate neglect about bipolar disorder

Answer Contents

**BQ-038**

**A medical emergency department is about to transfer a patient to a psychiatric unit with a note reading "medically cleared." According to the emergency psychiatry task force, what is the best improvement?**

**A.** Add a urine drug screen for every transferred patient

**B.** Order a full laboratory battery before every transfer

**C.** Use a note stating what was assessed, found, and the stability

**D.** Delay all transfers until neuroimaging is complete for each

**E.** Remove the vital signs from the transfer documentation

Answer Contents

SUBJECT 3

# Neuroscience and Mechanisms of Disease

Questions BQ-039 to BQ-065

**BQ-039**

**A 29-year-old man with schizophrenia is stable on an antipsychotic for hallucinations, but apathy and poor planning persist. His family asks whether the medicine is causing this. Which statement best reflects current understanding?**

**A.** D2 blockade in the mesolimbic pathway is proven to cause negative symptoms

**B.** Low prefrontal dopamine is suspected to contribute; D2 blockade may not help

**C.** Negative symptoms prove that the tuberoinfundibular pathway is overactive

**D.** Negative symptoms arise only from nigrostriatal blockade by the antipsychotic

**E.** Excess prefrontal dopamine is the established cause of negative symptoms

Answer Contents

**BQ-040**

**Two days after starting a high-potency antipsychotic, a 22-year-old man develops sudden painful twisting of his neck and upward deviation of his eyes. Blockade of D2 receptors in which pathway best explains this reaction?**

**A.** Mesolimbic

**B.** Mesocortical

**C.** Tuberoinfundibular

**D.** Nigrostriatal

**E.** Thalamocortical

Answer Contents

**BQ-041**

**A pharmacology student notes that ketamine and other NMDA receptor antagonists can produce schizophrenia-like symptoms in healthy volunteers. Which hypothesis does this observation most directly supports?**

**A.** Reduced NMDA activity on interneurons disinhibits cortical glutamate neurons

**B.** Excess GABA release from interneurons silences the cortex and drives psychosis

**C.** Muscarinic receptor overactivation in the striatum directly causes hallucinations

**D.** A serotonin deficiency in the raphe nuclei causes delusions in schizophrenia

**E.** Loss of nicotinic receptors on pyramidal neurons causes disorganized thinking

Answer Contents

**BQ-042**

**A trainee asks why clozapine can help patients who failed other antipsychotics, even though it occupies D2 receptors less than many agents. Which answer is most accurate?**

**A.** Its effect is fully explained by agonism at serotonin 5-HT2A receptors

**B.** It occupies D2 receptors more completely than any other antipsychotic

**C.** Its label establishes that it works through a D1-selective mechanism

**D.** It acts only on glutamate receptors in the prefrontal cortex

**E.** Its mechanism is unknown; D2 and 5-HT2A blockade are proposed

Answer Contents

**BQ-043**

**A 34-year-old woman with schizophrenia is considered for xanomeline-trospium. Her psychiatrist explains how the combination was designed. Which statement is accurate?**

**A.** Both components block D2 receptors in the mesolimbic pathway of the brain

**B.** Xanomeline is a muscarinic antagonist and trospium is a central agonist

**C.** Xanomeline is an M1 and M4 agonist; trospium is a peripheral antagonist

**D.** The combination is a 5-HT2A inverse agonist that spares D2 receptors

**E.** The combination is a metabotropic glutamate receptor 2/3 agonist

Answer Contents

**BQ-044**

**A 58-year-old man with chronic back pain takes an opioid analgesic and is prescribed alprazolam for anxiety. His pharmacist asks about the mechanism and the risk. Which statement is most accurate?**

**A.** It blocks D2 receptors in the striatum, and the combination is safe

**B.** It inhibits serotonin reuptake, so the combination lowers opioid effect

**C.** It activates orexin receptors, and the combination is stimulating

**D.** It enhances GABA-A inhibition, and opioids add respiratory depression

**E.** It blocks NMDA receptors, and the combination commonly causes mania

Answer Contents

**BQ-045**

**A 29-year-old woman three weeks postpartum with severe depression is prescribed zuranolone. Which counseling point is accurate?**

**A.** It positively modulates GABA-A; the boxed warning concerns driving

**B.** It blocks NMDA receptors; the boxed warning concerns sedation and dissociation

**C.** It blocks D2 receptors; the boxed warning concerns mortality in dementia

**D.** It inhibits serotonin reuptake; the boxed warning concerns serotonin syndrome

**E.** It blocks orexin receptors; the boxed warning concerns worsened narcolepsy

Answer Contents

**BQ-046**

**A 27-year-old nurse who rotates between day and night shifts asks how her body clock stays synchronized. Which statement is most accurate?**

**A.** The pineal gland alone is the master clock, through nightly melatonin release

**B.** The suprachiasmatic nucleus is the master clock, set by light

**C.** The amygdala sets circadian timing through its daily release of cortisol

**D.** The striatum sets circadian timing through daily changes in dopamine release

**E.** Melatonin released by the pituitary gland sets the master clock timing

Answer Contents

**BQ-047**

**A 30-year-old woman starting lithium for bipolar disorder asks how it works. According to the current label, which answer is most accurate?**

**A.** It selectively blocks D2 receptors in the mesolimbic pathway

**B.** It inhibits serotonin reuptake at the presynaptic transporter

**C.** It is a positive allosteric modulator at GABA-A receptors

**D.** It is a noncompetitive blocker of NMDA receptors in cortex

**E.** The way it acts as a mood stabilizer is not known

Answer Contents

**BQ-048**

**A 40-year-old woman asks why her SSRI, which inhibits the serotonin transporter from the first doses, took several weeks to lift her mood. Which inference is most reasonable?**

**A.** A serotonin deficiency takes weeks to be corrected by the drug

**B.** The drug is absorbed slowly, so it reaches the brain only after weeks

**C.** Downstream adaptation, not transporter block alone, probably matters

**D.** Adult neurogenesis has been proven to be the cause of the delay

**E.** The delay shows that the drug is acting mostly as a placebo

Answer Contents

**BQ-049**

**A 49-year-old man cites a news story that a review "proved antidepressants do not work." You have read the umbrella review of the serotonin theory. Which statement is accurate?**

**A.** It tested antidepressant efficacy in trials and found no benefit

**B.** It asked whether low serotonin causes depression, not if drugs work

**C.** It proved that an excess of serotonin activity causes depression

**D.** It found low serotonin levels in every patient that it studied

**E.** It showed that SSRIs raise the long-term risk of depression

Answer Contents

**BQ-050**

**A 72-year-old man taking a tricyclic antidepressant develops dry mouth, constipation, urinary hesitancy, and new confusion. Which receptor effect best explains the cluster?**

**A.** Muscarinic receptor blockade

**B.** D2 receptor blockade

**C.** 5-HT2A receptor agonism

**D.** Orexin receptor blockade

**E.** GABA-A receptor potentiation

Answer Contents

**BQ-051**

**A 41-year-old woman with treatment-resistant depression has a CRP of 6 mg/L, and a colleague proposes infliximab. Which response best reflects the randomized evidence?**

**A.** Clear benefit in all participants, regardless of their baseline CRP

**B.** Proven benefit in the subgroup with a CRP above 5 mg/L

**C.** Infliximab is approved to treat major depression in adults

**D.** No overall benefit; the high-CRP subgroup finding was exploratory

**E.** Significant worsening of depression in the treated group

Answer Contents

**BQ-052**

**A 36-year-old teacher has had difficulty sleeping for eight months but has no current depressive episode. She asks whether it matters. Which statement is best supported?**

**A.** Insomnia and later depression are unrelated in longitudinal studies

**B.** Insomnia is only a symptom of depression and not a risk factor

**C.** Insomnia protects against later mood episodes in most adults

**D.** Insomnia matters only in older adults, not in younger ones

**E.** Insomnia predicted later depression, with an odds ratio above 2

Answer Contents

**BQ-053**

**A 47-year-old man with treatment-resistant depression asks about esketamine. Which statement from the current label is accurate?**

**A.** It is dispensed for home use with pharmacy refills
**B.** It carries no boxed warning but needs a REMS
**C.** It is an NMDA antagonist given in certified settings and not shown to prevent suicide
**D.** It is a serotonin reuptake inhibitor given at home
**E.** Its antidepressant mechanism is fully established

Answer Contents

**BQ-054**

**A student reads that ketamine may act by blocking NMDA receptors on GABA interneurons. Which statement about this interneuron disinhibition hypothesis is accurate?**

**A.** It is the established antidepressant mechanism of ketamine in humans
**B.** It is contested, since mice lacking these receptors still responded
**C.** It has been conclusively disproved in controlled human studies
**D.** It explains the antidepressant effect of the benzodiazepines too
**E.** It applies only to opioid receptors on cortical interneurons

Answer Contents

**BQ-055**

**A 25-year-old genetics trainee is asked how complement C4 relates to schizophrenia. Which statement is most accurate?**

**A.** C4 variants cause schizophrenia in all of the people who carry them
**B.** C4 is irrelevant to brain tissue because it acts only in blood
**C.** C4 variation influences dopamine synthesis in the striatum only
**D.** Higher C4A expression and synapse loss in mice suggest a mechanism
**E.** C4 shows that schizophrenia is an autoimmune disease of the brain

Answer Contents

**BQ-056**

**A resident reads that a large genome-wide study found 287 loci for schizophrenia. Which interpretation is most accurate?**

**A.** Loci are enriched for synaptic genes; none is sufficient alone
**B.** A single locus explains most of the genetic risk for schizophrenia
**C.** The loci show that the disorder is purely genetic in origin
**D.** The result now allows a genetic diagnosis of schizophrenia in clinic
**E.** All of the loci lie within the major histocompatibility complex

Answer Contents

**BQ-057**

**A medical student asks why DSM categories overlap so much genetically. Which finding best explains this?**

**A.** Each disorder has its own distinct set of risk genes and pathways
**B.** Genetic risk is confined to rare variants of large effect only
**C.** A cross-disorder analysis found loci shared by several disorders
**D.** Twin studies show no heritability for any psychiatric disorder
**E.** All of the disorders share a single causal gene of large effect

Answer Contents

**BQ-058**

**A 32-year-old woman asks whether a serotonin transporter variant combined with stress will cause her depression. Which answer reflects the best evidence?**

**A.** Yes; this gene-by-stress interaction is well replicated
**B.** Yes; testing for the variant is recommended in clinical practice
**C.** Only in women who were exposed to childhood abuse as children
**D.** Only when the variant is combined with a second candidate gene
**E.** No; large preregistered tests found no support for it

Answer Contents

**BQ-059**

**A review of childhood adversity and psychosis pooled case-control, prospective, and cross-sectional studies. Which statement about its findings is accurate?**

**A.** Adversity showed no association with psychosis in any of the designs

**B.** Odds ratios were similar across designs; risk share assumes causality

**C.** Only the prospective studies found an association with psychosis

**D.** Adversity was protective against psychosis in the pooled analysis

**E.** Only sexual abuse was examined as a childhood adversity in the review

Answer Contents

**BQ-060**

**A trainee summarizes a postmortem study in which people who died by suicide after childhood abuse showed lower glucocorticoid receptor expression and higher promoter methylation in the hippocampus. What is the best statement of its limits?**

**A.** It proves that childhood abuse is a direct cause of suicide

**B.** It proves methylation changes are reversible with therapy

**C.** It shows methylation is a validated diagnostic test for suicide risk

**D.** It shows association in postmortem tissue, not causation in humans

**E.** It was a randomized controlled trial with a blinded comparison group

Answer Contents

**BQ-061**

**A school psychiatrist asks when most mental disorders begin. According to a meta-analysis of 192 studies, which statement is accurate?**

**A.** Peak onset is in mid-adolescence; most cases begin before age 25

**B.** Most mental disorders begin after age 40, in later adult life

**C.** Peak onset is in early infancy, before the age of two years

**D.** Onset is spread evenly across the whole lifespan from infancy

**E.** Onset occurs almost exclusively in adulthood, after age 25

Answer Contents

**BQ-062**

**A 42-year-old patient with depression asks whether a hippocampal volume scan could confirm her diagnosis. The consortium analysis in depression found a hippocampal effect size of -0.14. What is the best response?**

**A.** The scan is diagnostic if the hippocampal volume is below average
**B.** The scan predicts which antidepressant treatment will work best
**C.** The difference is small and overlaps, so a scan cannot confirm it
**D.** The hippocampus is larger in depression than in healthy controls
**E.** Only the amygdala volume differs in depression in this analysis

Answer Contents

**BQ-063**

**A meta-analysis of 193 structural imaging studies across six diagnostic groups, including schizophrenia, bipolar disorder, depression, addiction, obsessive-compulsive disorder, and anxiety disorders, looked for shared findings. What did it report?**

**A.** Each diagnosis had its own unique pattern of gray matter change
**B.** Only the occipital cortex differed across the six diagnostic groups
**C.** Gray matter was increased everywhere in all six diagnostic groups
**D.** Findings were limited to the cerebellum in every diagnostic group
**E.** Shared loss converged in dorsal anterior cingulate and both insulae

Answer Contents

**BQ-064**

**Two postmortem studies reach opposite conclusions about adult human hippocampal neurogenesis. A resident asks how to teach it. Which answer is best?**

**A.** It is firmly established to occur throughout adult life in humans
**B.** It is firmly established to cease in early childhood in humans
**C.** It explains all of the antidepressant effects of SSRIs in humans
**D.** It is contested: one study found none in adults, another did
**E.** It occurs only in the cerebellum and not in the hippocampus

Answer Contents

**BQ-065**

**A 33-year-old man with cocaine use disorder reports that seeing his old drug-using location triggers intense craving two years after quitting. Which explanation best fits current understanding?**

**A.** Dopamine is simply a pleasure chemical, and craving is nostalgia

**B.** Dopamine is a learning signal; cues linked to drugs trigger craving

**C.** The prefrontal cortex matures first and is therefore unaffected

**D.** Craving reflects an acute withdrawal state that lasts for years

**E.** Craving is a learned belief that has no measurable brain correlates

Answer Contents

SUBJECT 4

# Principles of Psychopharmacology

Questions BQ-066 to BQ-087

**BQ-066**

**A 69-year-old man has taken lithium for 12 years with stable levels. He starts naproxen daily for knee osteoarthritis. A week later he has a coarse tremor, vomiting, and an unsteady gait, and his lithium level is 1.9 mEq/L. What is the best explanation and next step?**

**A.** NSAIDs induce renal clearance, so lithium has fallen and the dose should be raised

**B.** Serotonin syndrome from the NSAID, so cyproheptadine should be started

**C.** The level is therapeutic, so the symptoms are unrelated to lithium

**D.** The NSAID reduced lithium clearance, so lithium should be held and toxicity evaluated

**E.** Lithium withdrawal, so the dose should be restarted at a higher level

Answer Contents

**BQ-067**

**A 24-year-old woman with bipolar I disorder is stable on divalproex and uses no contraception. She says she hopes to conceive within a year. Which counseling best reflects the boxed warning?**

**A.** Continue divalproex, because folic acid alone removes the malformation risk

**B.** Plan a move to a non-valproate mood stabilizer before conception, with effective contraception until then

**C.** Double the divalproex dose to protect her through pregnancy

**D.** Stop all medication as soon as the pregnancy test is positive

**E.** Valproate is the preferred first-line agent in pregnancy

Answer Contents

**BQ-068**

**A 32-year-old man of Han Chinese ancestry is about to start carbamazepine for epilepsy. What is the best step before the first dose?**

**A.** Obtain a baseline serum carbamazepine level

**B.** Start lamotrigine instead without further testing

**C.** Screen for rash only after the first month

**D.** Obtain a baseline ECG for QT prolongation

**E.** Test for HLA-B*15:02 and avoid the drug if positive unless benefit outweighs risk

Answer Contents

**BQ-069**

**A 35-year-old woman taking divalproex for bipolar I disorder is about to start lamotrigine for maintenance. Which plan is correct?**

**A.** Begin 25 mg every other day and escalate slowly toward a lower target of 100 mg/day

**B.** Begin 25 mg daily and aim for 200 mg/day within six weeks

**C.** Begin 50 mg daily because valproate speeds lamotrigine clearance

**D.** Begin at the 100 mg target to protect against early relapse

**E.** Avoid lamotrigine, because valproate makes it ineffective

Answer Contents

**BQ-070**

**A 54-year-old woman taking tamoxifen develops a depressive episode. Which choice best fits the paroxetine label's interaction warning?**

**A.** Paroxetine, because tamoxifen needs strong CYP2D6 inhibition to work

**B.** Fluoxetine, because it is weaker than paroxetine at CYP2D6

**C.** An antidepressant with little or no CYP2D6 inhibition

**D.** Carbamazepine, to induce tamoxifen activation

**E.** No antidepressant, because every one blocks tamoxifen

Answer Contents

**BQ-071**

**A 31-year-old man stable on aripiprazole is started on fluoxetine for depressive symptoms. According to the aripiprazole label, what dose adjustment applies?**

**A.** No change

**B.** Double the aripiprazole dose

**C.** Reduce aripiprazole to one quarter of the dose

**D.** Reduce aripiprazole to half of the dose

**E.** Stop aripiprazole during fluoxetine treatment

Answer Contents

**BQ-072**

**A 50-year-old man stable on quetiapine for bipolar depression needs ketoconazole for a fungal infection. What is the best adjustment?**

**A.** Reduce quetiapine to one sixth of the original dose, and raise it again after ketoconazole is stopped

**B.** Double quetiapine for the duration of ketoconazole

**C.** Halve quetiapine and recheck in one year

**D.** Continue the same dose without change

**E.** Stop quetiapine permanently

Answer Contents

**BQ-073**

**A 62-year-old woman with cirrhosis and severe anxiety needs a benzodiazepine for a short period. Which statement about lorazepam is accurate?**

**A.** It is oxidized by CYP3A4, so cirrhosis has no effect

**B.** It is conjugated rather than oxidized, but severe liver disease calls for lower doses and it may worsen hepatic encephalopathy

**C.** It is contraindicated in any degree of liver disease

**D.** It needs a higher dose in cirrhosis because of enzyme induction

**E.** It is safe at any dose in cirrhosis

Answer Contents

**BQ-074**

**A 47-year-old man taking oxycodone for chronic pain is given lorazepam for insomnia by a second clinician. The pharmacist calls you. What is the best response?**

**A.** Continue both and review at the yearly visit

**B.** Increase the lorazepam to cover the opioid effect

**C.** Switch to alprazolam, which carries no opioid warning

**D.** Stop the opioid abruptly

**E.** Reconsider the combination, and if unavoidable use the lowest dose and shortest duration with warnings about sedation and breathing

Answer Contents

**BQ-075**

**A clinician counsels three patients starting an SSRI, aged 16, 30, and 72. Which statement best reflects the age pattern in the label's boxed warning data?**

**A.** Excess suicidal thoughts and behavior occurred at every age

**B.** Excess risk occurred only at 65 and older

**C.** Excess risk occurred under 25, with fewer events than placebo at 25 and older

**D.** Risk was identical at every age

**E.** The warning applies only to children under 12

Answer Contents

**BQ-076**

**An 84-year-old woman who fell last year asks for zolpidem for insomnia. Which response is best?**

**A.** Offer cognitive behavioral therapy for insomnia first

**B.** Start zolpidem 10 mg, because it is safer than a benzodiazepine

**C.** Switch to lorazepam at bedtime

**D.** Add diphenhydramine as a gentler option

**E.** Combine zolpidem with a second sedative at half doses

Answer Contents

**BQ-077**

**A 79-year-old man with Alzheimer disease has new agitation. Pain, infection, and constipation have been treated. His family asks for haloperidol. What is the best response?**

**A.** Start intramuscular haloperidol at once

**B.** Start lorazepam three times daily

**C.** Start diphenhydramine at night

**D.** Use nonpharmacologic measures first, and consider an antipsychotic only if they fail or harm is substantial, after discussing the mortality warning

**E.** Start valproate as a safer alternative

Answer Contents

**BQ-078**

**A 56-year-old woman reducing long-term alprazolam reports tremor, insomnia, and panic after her third dose reduction. What is the best next step?**

**A.** Stop the remaining dose at once to finish the taper

**B.** Pause, or return to the previous dose, then reduce more slowly

**C.** Add a second benzodiazepine as cover

**D.** Double the rate of reduction

**E.** Switch to zolpidem and stop the benzodiazepine

Answer Contents

**BQ-079**

**A 40-year-old woman taking paroxetine misses her tablets for three days while traveling. She reports dizziness, nausea, electric shock sensations, and irritability. What is the best interpretation?**

**A.** Relapse of major depression

**B.** Serotonin syndrome

**C.** Discontinuation symptoms, treated by resuming the drug

**D.** A drug allergy

**E.** Early mania

Answer Contents

**BQ-080**

**A 45-year-old man is stopping fluoxetine for poor response, and his psychiatrist plans to start an MAOI. What is the earliest appropriate start?**

**A.** The next day

**B.** After 3 days

**C.** After 7 days

**D.** After 14 days

**E.** After at least 5 weeks

Answer Contents

**BQ-081**

**A 43-year-old woman with treatment-resistant depression asks to take esketamine nasal spray at home. Which statement is correct?**

**A.** It must be taken in a certified setting under direct observation, for at least 2 hours afterward

**B.** A pharmacist may supervise home use

**C.** A monthly mail-order supply is allowed once she is stable

**D.** A trained caregiver may supervise at home

**E.** The REMS was removed in 2025, as it was for clozapine

Answer Contents

**BQ-082**

**A resident says that a patient must be enrolled in the clozapine REMS before the first dose. The attending physician corrects her. Which statement is accurate?**

**A.** Enrollment is still required for every patient

**B.** ANC monitoring is no longer needed

**C.** Pharmacies still verify the ANC before dispensing

**D.** The REMS ended effective 13 June 2025, but ANC monitoring still follows the label

**E.** The FDA now sets a single universal ANC schedule

Answer Contents

**BQ-083**

**A clinic plans to give olanzapine pamoate to a patient with schizophrenia. Which requirement applies?**

**A.** Observation for 30 minutes after each injection

**B.** Administration in a certified setting with at least 3 hours of observation, under a REMS

**C.** Self-injection at home after training

**D.** Observation only after the first injection

**E.** No special requirement, because oral olanzapine is well tolerated

Answer Contents

**BQ-084**

**A 29-year-old man with schizophrenia has relapsed twice after stopping oral risperidone. He says daily pills are hard to remember and he would prefer fewer doses. What is the best next step?**

**A.** Repeat education and continue the same oral regimen

**B.** Add a second oral antipsychotic

**C.** Offer a long-acting injectable antipsychotic

**D.** Stop the antipsychotic until symptoms return

**E.** Switch every patient with a relapse to clozapine

Answer Contents

**BQ-085**

**A 36-year-old woman known to be a CYP2D6 poor metabolizer needs an antidepressant, and her clinician is considering nortriptyline. What is the best approach?**

**A.** Use the standard dose with no monitoring

**B.** Double the dose to compensate

**C.** Choose amitriptyline, which avoids the problem

**D.** Add paroxetine to raise the levels

**E.** Avoid a tricyclic, or if one is used reduce the dose and monitor levels

Answer Contents

**BQ-086**

**A 52-year-old woman has congenital long QT syndrome and schizophrenia. Which antipsychotic is contraindicated by its label?**

**A.** Ziprasidone
**B.** Aripiprazole
**C.** Olanzapine
**D.** Quetiapine
**E.** Clozapine

Answer Contents

**BQ-087**

**A 33-year-old man with schizophrenia relapses repeatedly on clozapine at a stable prescribed dose. Pharmacy refills are inconsistent, and a trough clozapine level is far below the level expected for his dose. What is the best interpretation?**

**A.** Proven ultrarapid metabolism, so the dose should be doubled
**B.** The level is useless, so ignore it
**C.** A need to add carbamazepine to raise levels
**D.** Probable nonadherence or increased clearance such as smoking, to be explored before raising the dose
**E.** Treatment failure, so stop clozapine

Answer Contents

SUBJECT 5

# Antidepressants, Anxiolytics, and Hypnotics

Questions BQ-088 to BQ-112

**BQ-088**

**A 52-year-old man takes venlafaxine ER 225 mg daily for depression. At two consecutive visits his blood pressure is 168/102 mm Hg, previously normal. Which is the best next step?**

**A.** Continue the same dose and recheck blood pressure at the next annual visit

**B.** Add a benzodiazepine to treat the associated anxiety

**C.** Stop venlafaxine abruptly and observe

**D.** Reduce the dose or change the antidepressant, and treat the hypertension

**E.** Switch to tranylcypromine

Answer Contents

**BQ-089**

**A 60-year-old man has depression and painful diabetic neuropathy. He has cirrhosis from alcohol use. Which antidepressant is best avoided because of his liver disease?**

**A.** Duloxetine

**B.** Mirtazapine at a reduced dose

**C.** Venlafaxine ER at a reduced dose

**D.** Escitalopram 10 mg

**E.** Bupropion XL at a reduced frequency

Answer Contents

**BQ-090**

**A 34-year-old man with obsessive-compulsive disorder takes tizanidine for spasticity. Which SSRI is contraindicated with tizanidine?**

**A.** Sertraline

**B.** Escitalopram

**C.** Paroxetine

**D.** Fluoxetine

**E.** Fluvoxamine

Answer Contents

**BQ-091**

**A 48-year-old woman taking tamoxifen for breast cancer develops major depression. Which antidepressant is best avoided because it may reduce the activation of tamoxifen?**

**A.** Venlafaxine

**B.** Paroxetine

**C.** Mirtazapine

**D.** Escitalopram

**E.** Vortioxetine

Answer Contents

**BQ-092**

**A 35-year-old woman has taken mirtazapine 30 mg for eight weeks. She now has fever and a sore throat. Which is the best next step?**

**A.** Increase the mirtazapine dose

**B.** Start an antibiotic and continue mirtazapine

**C.** Obtain a complete blood count with differential and hold mirtazapine pending the result

**D.** Reassure her that this is a common adverse effect

**E.** Switch to phenelzine

Answer Contents

**BQ-093**

**A 56-year-old man takes trazodone 50 mg at night for sleep. He calls because of a painful erection lasting 5 hours. What should you advise?**

**A.** Stop trazodone and go to an emergency department now

**B.** Take a second dose to relax

**C.** Wait until morning and call the clinic

**D.** Apply a cold pack and wait 12 hours

**E.** Reduce the dose by half tomorrow

Answer Contents

**BQ-094**

**A 40-year-old man tells you vilazodone 20 mg is not helping. He takes it in the morning before breakfast. What is the best next step?**

**A.** Increase to 80 mg
**B.** Add buspirone
**C.** Switch to an MAOI
**D.** Have him take it with a meal and reassess
**E.** Stop treatment since it has failed

Answer Contents

**BQ-095**

**A 29-year-old woman takes bupropion XL 300 mg daily. A second clinician plans to add vortioxetine. How should the vortioxetine dose be handled?**

**A.** Double the usual dose because bupropion lowers levels
**B.** Use half the usual dose because bupropion is a strong CYP2D6 inhibitor
**C.** Make no change
**D.** Avoid the combination because bupropion is an MAOI
**E.** Give it on alternate days

Answer Contents

**BQ-096**

**A 44-year-old woman has a baseline QTc of 470 msec on ECG. A clinician considers starting gepirone. Which statement is correct?**

**A.** Start at the labeled initial dose and repeat the ECG in a year
**B.** Start at half the initial dose
**C.** Correct potassium and start at the labeled initial dose
**D.** Add a benzodiazepine to blunt the effect
**E.** Avoid gepirone because the baseline QTc exceeds the labeled threshold

Answer Contents

**BQ-097**

**A 27-year-old woman with postpartum depression is prescribed zuranolone for a 14-day course. Which counseling is correct?**

**A.** Take it in the morning on an empty stomach

**B.** Drive normally if you feel alert

**C.** Do not drive or do hazardous activities for at least 12 hours after each dose

**D.** Continue it for 6 months

**E.** No contraception is needed

Answer Contents

**BQ-098**

**A 41-year-old man with treatment-resistant depression is to receive esketamine nasal spray. Which statement is correct?**

**A.** He may take later doses at home after the first

**B.** He must self-administer under direct observation in a certified setting and be observed for at least 2 hours

**C.** No restricted program applies

**D.** He should take it after a large meal

**E.** He should start with once-weekly sessions

Answer Contents

**BQ-099**

**A 33-year-old woman is brought in after an amitriptyline overdose. Her ECG shows a widened QRS complex. Which treatment is most appropriate?**

**A.** Induce vomiting

**B.** Hemodialysis

**C.** Forced diuresis

**D.** Flumazenil

**E.** Sodium bicarbonate with cardiac monitoring

Answer Contents

**BQ-100**

**A 36-year-old man taking phenelzine develops a severe occipital headache, palpitations, and a blood pressure of 210/118 mm Hg after a meal with aged cheese. What is the best management?**

**A.** Stop phenelzine and lower the blood pressure urgently in an emergency setting

**B.** Add an SSRI

**C.** Observe at home for 24 hours

**D.** Give meperidine for the headache

**E.** Give an oral benzodiazepine and send him home

Answer Contents

**BQ-101**

**A 50-year-old woman taking tranylcypromine is scheduled for elective surgery under general anesthesia in two weeks. What is the best advice?**

**A.** Continue tranylcypromine through surgery

**B.** Stop it the evening before

**C.** Stop it 3 days before

**D.** Stop it at least 10 days before, if elective

**E.** Stop it 5 days before and start an SSRI

Answer Contents

**BQ-102**

**In the VAST-D randomized trial of patients who had not remitted on an antidepressant, which finding is correct?**

**A.** Switching to bupropion was superior to augmentation

**B.** Bupropion augmentation was significantly superior to aripiprazole augmentation

**C.** Aripiprazole augmentation had higher 12-week remission than switching to bupropion, with more akathisia and weight gain

**D.** The arms did not differ in adverse effects

**E.** Remission exceeded half of patients in every arm

Answer Contents

**BQ-103**

**According to the VA/DoD 2022 guideline, which drug class is suggested against for initial pharmacotherapy of major depressive disorder?**

**A.** Bupropion

**B.** Mirtazapine

**C.** Vortioxetine

**D.** Serotonin and norepinephrine reuptake inhibitors

**E.** Tricyclic antidepressants

Answer Contents

**BQ-104**

**A 35-year-old man reached remission on sertraline after two months and wants to stop. According to VA/DoD 2022, what is the best recommendation?**

**A.** Continue the therapeutic dose for at least 6 months

**B.** Stop immediately

**C.** Stop after 4 more weeks

**D.** Reduce to a quarter of the dose indefinitely

**E.** Switch to as-needed dosing

Answer Contents

**BQ-105**

**A 28-year-old pregnant woman has mild to moderate depression and no prior treatment. According to VA/DoD 2022, which is the best first-line treatment?**

**A.** Paroxetine

**B.** A benzodiazepine

**C.** Evidence-based psychotherapy

**D.** Electroconvulsive therapy

**E.** Vitamin D and omega-3 fatty acids

Answer Contents

**BQ-106**

**A 45-year-old man takes oxycodone for chronic pain and asks for alprazolam for anxiety. What is the best response?**

**A.** Prescribe alprazolam 0.5 mg three times daily

**B.** Decline the benzodiazepine, explain the boxed warning, and treat the anxiety with an SSRI or psychotherapy

**C.** Prescribe alprazolam with naloxone and no dose limit

**D.** Raise the alprazolam dose to cover the pain

**E.** Prescribe lorazepam instead

Answer Contents

**BQ-107**

**A 52-year-old woman has taken lorazepam 3 mg daily for years. She wants to start buspirone and stop lorazepam in one week. What is the best advice?**

**A.** Stop lorazepam on day one because buspirone prevents withdrawal

**B.** Buspirone provides immediate anxiolysis, so a taper is unnecessary

**C.** Switch to zolpidem first

**D.** Taper lorazepam gradually, because buspirone does not block benzodiazepine withdrawal

**E.** Add flumazenil to speed the taper

Answer Contents

**BQ-108**

**A 31-year-old woman with congenital long QT syndrome seeks treatment for anxiety. Which agent is contraindicated by its label in prolonged QT?**

**A.** Buspirone

**B.** Sertraline

**C.** Venlafaxine

**D.** Mirtazapine

**E.** Hydroxyzine

Answer Contents

**BQ-109**

**A veteran with PTSD asks about prazosin for nightmares. Which statement is most accurate?**

**A.** It is FDA labeled for PTSD

**B.** VA/DoD 2023 suggests it for nightmares but against it for overall PTSD symptoms, and the label covers hypertension only

**C.** It is strongly recommended for all PTSD symptoms

**D.** It is contraindicated in PTSD

**E.** It is equivalent to sertraline

Answer Contents

**BQ-110**

**A 33-year-old man with narcolepsy and cataplexy reports insomnia. A clinician considers lemborexant. Which statement is correct?**

**A.** Lemborexant is contraindicated in narcolepsy

**B.** Lemborexant 5 mg is appropriate

**C.** The dose should be halved

**D.** It is acceptable with strong CYP3A inhibitors

**E.** It is appropriate only above 10 mg

Answer Contents

**BQ-111**

**A 38-year-old woman stopped paroxetine 3 days ago and now has dizziness, electric-shock sensations, and irritability. Which is the best next step?**

**A.** Start lithium

**B.** Add a benzodiazepine and stay off paroxetine

**C.** Double the old dose

**D.** Restart paroxetine at the prior dose and taper more slowly

**E.** Switch to an MAOI

Answer Contents

**BQ-112**

**A 78-year-old woman taking hydrochlorothiazide started sertraline three weeks ago. She now has confusion, nausea, and a fall. What is the best next step?**

**A.** Increase sertraline

**B.** Add a benzodiazepine for agitation

**C.** Check the serum sodium

**D.** Start haloperidol

**E.** Reassure her that this is discontinuation syndrome

Answer Contents

SUBJECT 6

# Antipsychotics, Mood Stabilizers, and Other Agents

Questions BQ-113 to BQ-137

**BQ-113**

**A 22-year-old woman with schizophrenia gains weight quickly and is excessively sleepy after starting a new antipsychotic. Which receptor action best explains both effects?**

**A.** Muscarinic M1 agonism

**B.** Histamine H1 blockade

**C.** Alpha-1 adrenergic blockade

**D.** Dopamine D2 partial agonism

**E.** Serotonin 5-HT2A agonism

Answer Contents

**BQ-114**

**A 45-year-old man takes quetiapine 600 mg daily for schizophrenia. He needs oral ketoconazole for a fungal infection. What dose adjustment does the quetiapine label advise while ketoconazole is given?**

**A.** Increase to 800 mg daily

**B.** Make no change

**C.** Reduce by about one-third

**D.** Reduce to about one-sixth of the original dose

**E.** Stop quetiapine permanently

Answer Contents

**BQ-115**

**A 30-year-old man with schizophrenia takes ziprasidone 80 mg twice daily, swallowed on an empty stomach, and has had little benefit. What is the best explanation and response?**

**A.** Absorption is lower without food, so take each dose with a meal

**B.** The dose is too low for his illness, so raise it to 120 mg twice daily

**C.** Ziprasidone works best at bedtime, so move both doses to the night

**D.** Absorption is already maximal when fasting, so no change is needed

**E.** The capsules should be opened and sprinkled on food at each dose

Answer Contents

**BQ-116**

**A 26-year-old woman takes lurasidone 80 mg with breakfast for schizophrenia. Her physician plans to prescribe clarithromycin for sinusitis. What is the best advice?**

**A.** Continue both drugs unchanged

**B.** Halve the lurasidone dose

**C.** Double the lurasidone dose

**D.** Take lurasidone on an empty stomach during the antibiotic

**E.** Avoid the combination, because lurasidone is contraindicated with strong CYP3A4 inhibitors

Answer Contents

**BQ-117**

**A 33-year-old man with bipolar I disorder is prescribed sublingual asenapine. Which instruction is correct?**

**A.** Swallow the tablet whole with a full glass of water

**B.** Chew the tablet and eat a meal at once

**C.** Let it dissolve under the tongue and avoid eating or drinking for 10 minutes

**D.** Crush it and mix with juice

**E.** Split the tablet if the dose is too high

Answer Contents

**BQ-118**

**A 40-year-old woman with schizophrenia has cariprazine increased from 3 mg to 4.5 mg daily. Two weeks later she reports new restlessness. Why should she be monitored for several weeks after each change?**

**A.** Cariprazine and its active metabolites have long half-lives, so the full effect of a dose change appears over weeks
**B.** Cariprazine is a prodrug that takes weeks to be activated
**C.** Tolerance to the dose develops after 2 weeks
**D.** Restlessness always signals an unrelated medical illness
**E.** Active metabolites are cleared within hours

Answer Contents

**BQ-119**

**A 55-year-old man with schizophrenia has benign prostatic hyperplasia and a history of urinary retention. A clinician considers xanomeline-trospium. What is the best assessment?**

**A.** Appropriate at half the usual dose
**B.** Appropriate if taken with a high-fat meal
**C.** Appropriate if combined with an anticholinergic
**D.** Appropriate if liver tests are normal
**E.** Contraindicated because of urinary retention

Answer Contents

**BQ-120**

**An 86-year-old nursing home resident with Alzheimer dementia has persistent paranoid delusions. Her family asks about adding quetiapine. Which risk is most important to discuss?**

**A.** Tardive dyskinesia within weeks
**B.** Increased risk of death in elderly patients with dementia-related psychosis
**C.** Fatal liver failure
**D.** Agranulocytosis requiring weekly counts
**E.** Nephrogenic diabetes insipidus

Answer Contents

**BQ-121**

**A 38-year-old man with schizophrenia is stable on oral haloperidol 12 mg daily and will start haloperidol decanoate. Which first-dose plan follows the label?**

**A.** One 240 mg injection

**B.** 50 mg weekly for 4 weeks

**C.** 12 mg intramuscularly every day

**D.** 100 mg now, with the remainder of the calculated dose 3 to 7 days later

**E.** 400 mg every 4 weeks from the first injection

Answer Contents

**BQ-122**

**A 60-year-old man with schizophrenia is stable on oral paliperidone. His creatinine clearance by the Cockcroft-Gault method is 42 mL/min. Monthly paliperidone palmitate is considered. What is the best assessment?**

**A.** Use the standard day 1 and day 8 initiation doses

**B.** Use the reduced initiation doses designed for mild renal impairment

**C.** Use of the injection is not recommended at this clearance

**D.** Use the 6-month product

**E.** Double the initiation doses

Answer Contents

**BQ-123**

**A 35-year-old man begins risperidone long-acting microspheres 25 mg every 2 weeks after tolerating oral risperidone. For how long after the first injection should oral risperidone or another antipsychotic continue?**

**A.** It is not needed

**B.** 3 weeks

**C.** 1 week

**D.** 8 weeks

**E.** Until the second injection

Answer Contents

**BQ-124**

**A 33-year-old woman in the emergency department receives intramuscular olanzapine 10 mg for agitation. She remains agitated at 30 minutes, and a resident proposes intramuscular lorazepam. What is the best advice?**

**A.** Avoid combining it with parenteral lorazepam because of excessive sedation and cardiorespiratory depression, and reassess before more olanzapine

**B.** Mix the lorazepam in the same syringe as the olanzapine to save time

**C.** Use the lorazepam solution to reconstitute the olanzapine vial

**D.** Give lorazepam first and give olanzapine again only if she then sleeps

**E.** Add a second olanzapine dose immediately without any reassessment

Answer Contents

**BQ-125**

**A 36-year-old man who smokes heavily has been stable on clozapine 400 mg daily. He enters a smoke-free hospital and stops smoking. Ten days later he is very sedated, drooling, and has a generalized seizure. What is the best explanation?**

**A.** Severe neutropenia

**B.** Neuroleptic malignant syndrome

**C.** Tolerance to clozapine

**D.** Inhibition of CYP2D6

**E.** Loss of CYP1A2 induction, raising clozapine levels

Answer Contents

**BQ-126**

**A 29-year-old man started clozapine 16 days ago. He now has a temperature of 38.7 degrees C, a heart rate of 120, and chest discomfort. The ANC is 3,800 per microliter. What is the best next step?**

**A.** Reassure him that this is a viral illness and continue clozapine

**B.** Increase the clozapine dose

**C.** Stop clozapine and evaluate for myocarditis, including troponin and C-reactive protein

**D.** Repeat only the ANC in one week

**E.** Add lithium

Answer Contents

**BQ-127**

**A 38-year-old Black man has two baseline ANC values of 1,300 per microliter on separate days, with no infection and a suspected Duffy-null associated neutrophil count. May clozapine be started for treatment-resistant schizophrenia?**

**A.** No, because the ANC must be 1500 or higher in every patient before a start

**B.** Yes, but only after enrolling the prescriber and patient in a REMS program

**C.** Yes, but only with daily ANC tests for the entire first year of treatment

**D.** Yes, because benign ethnic neutropenia allows starting at an ANC of 1000 or higher, with monitoring on the label schedule

**E.** Yes, but only after a course of granulocyte colony-stimulating factor

Answer Contents

**BQ-128**

**Which statement describes the clozapine REMS and monitoring in 2026?**

**A.** The REMS was removed in June 2025, but the label still calls for ANC monitoring at set frequencies

**B.** The REMS continues, and pharmacists must verify each ANC before dispensing clozapine

**C.** The REMS was removed, so ANC monitoring is no longer needed in clinical practice

**D.** The boxed warning for severe neutropenia was removed together with the REMS

**E.** Prescribers must still report each ANC result to a central national registry

Answer Contents

**BQ-129**

**A 27-year-old man receives intramuscular haloperidol for agitation. Two days later his temperature is 40.1 degrees C, with generalized rigidity, confusion, a heart rate of 130, labile blood pressure, and an elevated creatine kinase. What is the best next step?**

**A.** Add benztropine and continue haloperidol

**B.** Give another dose of haloperidol

**C.** Stop the antipsychotic and begin intensive supportive care

**D.** Start an SSRI

**E.** Start lithium

Answer Contents

**BQ-130**

**A 40-year-old man began lithium 300 mg three times daily 3 days ago. A level drawn 2 hours after the morning dose is 1.9 mEq/L, and he feels well. What is the best next step?**

**A.** Stop lithium permanently

**B.** Reduce the dose by half immediately

**C.** Treat with hemodialysis

**D.** Give activated charcoal

**E.** Repeat the level 12 hours after the last dose before changing the dose

Answer Contents

**BQ-131**

**A 50-year-old woman who has taken lithium for 12 years reports polyuria and thirst. Her creatinine is normal and her lithium level is 0.7 mEq/L. What is the best next step?**

**A.** Add hydrochlorothiazide to reduce urine output

**B.** Evaluate for nephrogenic diabetes insipidus, monitor renal function, and consider amiloride

**C.** Restrict fluids to reduce the thirst and urine output

**D.** Increase the lithium dose to maintain a higher serum level

**E.** Stop lithium that day and discharge her without a follow-up plan

Answer Contents

**BQ-132**

**A 45-year-old man with bipolar I disorder has been stable on valproate with a trough of 85 mcg/mL. He receives meropenem for pneumonia. One week later he is manic, and the valproate level is 18 mcg/mL. What is the best explanation?**

**A.** Poor adherence

**B.** Induction of valproate metabolism by pneumonia

**C.** Displacement from albumin raising the free fraction

**D.** A carbapenem lowering valproate to subtherapeutic levels

**E.** Valproate-induced pancreatitis

Answer Contents

**BQ-133**

**A 29-year-old woman with bipolar I disorder is stable on lamotrigine 200 mg daily. She starts a combined estrogen-progestin contraceptive, and depressive symptoms recur 6 weeks later. What is the best explanation and response?**

**A.** Estrogen lowers lamotrigine levels, so check the response and consider raising the dose

**B.** Progestin raises lamotrigine levels, so reduce the dose

**C.** Lamotrigine induces estrogen metabolism, so stop the contraceptive

**D.** The combination is contraindicated, so stop lamotrigine at once

**E.** Add valproate at full dose

Answer Contents

**BQ-134**

**A 31-year-old man is stable on aripiprazole 15 mg daily. Carbamazepine is added for mood stabilization. What adjustment does the aripiprazole label advise?**

**A.** Halve the aripiprazole dose

**B.** Make no change

**C.** Double the aripiprazole dose over 1 to 2 weeks

**D.** Reduce the dose to one-quarter

**E.** Stop aripiprazole

Answer Contents

**BQ-135**

**A 58-year-old man with tardive dyskinesia takes paroxetine. Deutetrabenazine is started at 12 mg daily. What is the maximum daily dose while he takes paroxetine?**

**A.** 24 mg

**B.** 48 mg

**C.** 60 mg

**D.** 12 mg

**E.** 36 mg

Answer Contents

**BQ-136**

**Parents of a 9-year-old boy with ADHD ask about the safety of atomoxetine. Which boxed warning applies?**

**A.** Abuse and addiction

**B.** Increased risk of suicidal ideation in children and adolescents

**C.** Hepatic failure

**D.** Cardiomyopathy

**E.** Priapism

Answer Contents

**BQ-137**

**A 72-year-old man with mild Alzheimer dementia is a candidate for lecanemab. Which pre-treatment work-up does the label call for?**

**A.** Lumbar puncture alone, without genotype testing or brain imaging

**B.** Echocardiogram and Holter monitoring for conduction disease

**C.** Serum lithium level and thyroid function tests

**D.** Baseline brain MRI, ApoE e4 genotype testing, and confirmation of amyloid pathology

**E.** Electroencephalogram with sleep deprivation

Answer Contents

SUBJECT 7

# Psychotherapies

Questions BQ-138 to BQ-159

**BQ-138**

**A 31-year-old man with depression has stopped working out, stopped seeing friends, and stays in bed most of the day. He says he has no energy and cannot "think his way out of it." His clinician chooses behavioral activation. Which technique best matches this therapy?**

**A.** Identifying and relabeling cognitive distortions in writing

**B.** Tracking activity and mood, then scheduling small valued tasks

**C.** Interpreting the unconscious meaning of his withdrawal

**D.** Repeated exposure to feared social situations and cues

**E.** Mindfulness practice to prevent relapse of depression

Answer Contents

**BQ-139**

**A 29-year-old woman believes that if she speaks up in a meeting, her colleagues will think she is stupid. Her therapist asks her to speak once in the next meeting, record what happens, and compare it with her prediction. Which cognitive behavioral technique is this?**

**A.** Thought record

**B.** Chain analysis

**C.** Interpersonal inventory

**D.** Behavioral experiment

**E.** Defusion

Answer Contents

**BQ-140**

**A 67-year-old woman lost her husband four months ago. She is tearful, isolated, and no longer sees the people they used to spend time with. Her clinician proposes interpersonal therapy. Which focus is most appropriate?**

**A.** Grief

**B.** Role dispute

**C.** Role transition

**D.** Interpersonal deficits

**E.** Social rhythm stabilization

Answer Contents

**BQ-141**

**A 35-year-old man in psychodynamic therapy tells his therapist he is sure she is judging him harshly, as his father did, though she has said nothing critical. Which term best describes this?**

**A.** Countertransference

**B.** Reaction formation

**C.** Projective identification

**D.** Sublimation

**E.** Transference

Answer Contents

**BQ-142**

**A 33-year-old man with schizophrenia was discharged a week after a psychotic relapse. He is still disorganized and distressed, and he asks his psychiatrist for "therapy to understand why this keeps happening." Which approach is best at this stage?**

**A.** Long-term psychodynamic interpretation of early conflicts

**B.** Prolonged exposure to the memory of the relapse episode

**C.** Supportive therapy focused on stabilization and practical problems

**D.** An unstructured open-ended group for new patients

**E.** Intensive couples therapy with his partner

Answer Contents

**BQ-143**

**A 41-year-old woman in acceptance and commitment therapy says, "I can't go to my daughter's recital because I'll have a panic attack." Her therapist asks what matters to her about being at the recital and what small step toward it she could take with the anxiety present. Which process is the therapist using?**

**A.** Thought stopping
**B.** Values and committed action
**C.** Systematic desensitization
**D.** Cognitive restructuring
**E.** Distress tolerance by distraction

Answer Contents

**BQ-144**

**A 26-year-old man in a stimulant use disorder program receives a voucher for each urine sample that tests negative, and the voucher value rises with consecutive negative samples. After a positive sample, the value returns to the starting level. Which feature of contingency management is the reset meant to preserve?**

**A.** The tie between verified behavior and reward
**B.** The therapeutic alliance with the clinician
**C.** The patient's insight into his triggers
**D.** The use of an escalating reward schedule
**E.** Confidentiality of the test results

Answer Contents

**BQ-145**

**A 30-year-old man with schizophrenia lives with parents who are critical and often argue with him about his illness. He relapses after each discharge. Which intervention is most likely to reduce his relapse risk?**

**A.** Placing him in a separate residence without family contact

**B.** Switching to a lower antipsychotic dose without family involvement

**C.** Insight-oriented therapy for the parents alone

**D.** Family psychoeducation and communication training

**E.** Group therapy limited to other patients

Answer Contents

**BQ-146**

**In a weekly therapy group, one member dominates the discussion and another member is repeatedly criticized by the others. A third member says she is thinking of leaving the group. Which is the best action by the group leader?**

**A.** Allow the group to settle it on its own without interference

**B.** Remove the criticized member from the group for safety

**C.** Intervene in the process, restate ground rules, and meet individually

**D.** Ask the departing member to stay to avoid disrupting the group

**E.** Refer the dominant member to individual therapy at once

Answer Contents

**BQ-147**

**A 40-year-old veteran has PTSD and a current alcohol use disorder. He wants trauma-focused therapy but his program says he must be abstinent first. According to the VA/DoD PTSD guideline, which is the best statement?**

**A.** Trauma-focused therapy is contraindicated when alcohol use disorder is present

**B.** He should receive supportive counseling only until abstinent

**C.** Group therapy is recommended instead of individual treatment

**D.** He should receive a benzodiazepine for withdrawal-related hyperarousal

**E.** Co-occurring substance use does not by itself rule out the therapy

Answer Contents

**BQ-148**

**A pregnant 28-year-old woman has mild to moderate major depressive disorder and no history of severe illness. She asks about treatment options. According to the VA/DoD depression guideline, what is the recommended first-line approach?**

**A.** Psychotherapy
**B.** An SSRI at a low dose
**C.** Electroconvulsive therapy
**D.** Ketamine
**E.** Watchful waiting only

Answer Contents

**BQ-149**

**A 72-year-old man has mild to moderate depression after retirement and has no cognitive impairment. He is wary of adding to his medications. Which approach is most consistent with the VA/DoD depression guideline?**

**A.** Start two antidepressants at once for faster effect
**B.** Offer psychotherapy as first-line treatment
**C.** Defer treatment until his symptoms clearly worsen
**D.** Start a benzodiazepine for sleep and anxiety
**E.** Refer for electroconvulsive therapy as first treatment

Answer Contents

**BQ-150**

**A clinician wants to explain to a patient with recurrent, severe depression why she is recommending both an antidepressant and psychotherapy. Which statement is best supported by a meta-analysis of randomized trials?**

**A.** Combined treatment was no better than either treatment given alone
**B.** Psychotherapy alone was better than combined treatment in all trials
**C.** Combined treatment helped only in the anxiety disorders, not depression
**D.** Combined treatment beat medication alone in depression and anxiety
**E.** Medication alone was better than combined treatment in most trials

Answer Contents

**BQ-151**

**A 25-year-old woman with borderline personality disorder and chronic self-harm asks her psychiatrist for a medication that will "fix" her mood swings and impulsivity. Which response is most consistent with NICE guidance?**

**A.** Prescribe an antipsychotic for long-term treatment of emotional instability

**B.** Prescribe several medications, one for each symptom she has

**C.** Offer structured psychological therapy and avoid disorder-specific drugs

**D.** Defer all therapy until her mood is stable on medication

**E.** Offer a brief intervention of under 3 months by any clinician

Answer Contents

**BQ-152**

**A resident reads a meta-analysis showing that cognitive behavioral therapy for psychosis has a small overall effect on symptoms, and that the effect shrinks when only trials with masked assessors are pooled. Which conclusion is best?**

**A.** CBT for psychosis is ineffective and should be abandoned

**B.** The result proves that CBT replaces antipsychotic medication

**C.** The benefit is large and independent of the quality of trials

**D.** Effects were larger in the trials at low risk of bias in all domains

**E.** The benefit is modest and may be overestimated, so CBT is an adjunct

Answer Contents

**BQ-153**

**A 34-year-old rural veteran with PTSD lives far from a specialty clinic. A clinician considers providing prolonged exposure by secure video. According to the VA/DoD PTSD guideline, which statement is accurate?**

**A.** Recommended therapies may be delivered by secure video when validated for that mode or when other options are unavailable

**B.** Only supportive therapy can be delivered by video

**C.** Video delivery is acceptable only after in-person treatment fails

**D.** Video delivery is recommended only for group therapy

**E.** Video delivery is not recommended for exposure-based therapy

Answer Contents

**BQ-154**

**After the final session, a patient offers his therapist an expensive watch as a thank-you gift. Which response is best?**

**A.** Accept it to avoid hurting his feelings at the final session

**B.** Accept it and donate it to charity without telling him

**C.** Accept it but then remove him from the practice

**D.** Decline kindly, explore what the gesture means, and document it

**E.** Ignore the gift and say nothing about it at all

Answer Contents

**BQ-155**

**A trainee summarizes the alliance literature by saying, "A strong alliance causes good outcomes in every therapy." Which correction is most accurate?**

**A.** The alliance is unrelated to outcome in controlled studies

**B.** It is associated with outcome in many therapies, but is correlational

**C.** The alliance matters only in psychodynamic and supportive therapy

**D.** The association appears only in internet-delivered therapy trials

**E.** The association is largest when measured at the first session

Answer Contents

**BQ-156**

**A psychiatrist who prescribes for a patient learns from the patient that he has told his therapist about new suicidal thoughts. The therapist has not contacted the psychiatrist. Which action is best?**

**A.** Wait for the next scheduled visit and ask about it then

**B.** Change the medication without contacting the therapist first

**C.** Ask the patient to deliver messages between them himself

**D.** Discontinue prescribing until the therapist calls him

**E.** With consent, call the therapist and assess risk now

Answer Contents

**BQ-157**

**During a motivational interviewing session, a patient with cannabis use disorder says, "I should probably cut back, though it's not easy." Which category best describes the first part of this statement?**

**A.** Sustain talk

**B.** Righting reflex

**C.** Change talk

**D.** Resistance to the therapist

**E.** Commitment language of a completed action

Answer Contents

**BQ-158**

**The parents of a 15-year-old with obsessive-compulsive disorder reassure him every time he asks whether he washed his hands well. He is starting CBT with exposure and response prevention. Which family instruction is best?**

**A.** Respond with supportive, non-reassuring statements

**B.** Stop all contact with him during exposures

**C.** Remove all cleaning supplies from the home

**D.** Punish him whenever he asks for reassurance

**E.** Increase reassurance to reduce his anxiety

Answer Contents

**BQ-159**

**A 38-year-old woman in prolonged exposure becomes very distressed during imaginal exposure and says she wants to stop. Her therapist has seen that she can tolerate this level of distress. Which response is best?**

**A.** Stop the session and skip imaginal exposure from now on

**B.** Give a benzodiazepine before the next exposure session

**C.** Switch immediately to present-centered therapy instead

**D.** Validate distress, use grounding, and go on at a manageable pace

**E.** Tell her that she must continue regardless of her wishes

Answer Contents

SUBJECT 8

# Neuromodulation and Interventional Psychiatry

Questions BQ-160 to BQ-175

**BQ-160**

**A 31-year-old man with bipolar I disorder is hospitalized with severe mania that has not responded to lithium and an antipsychotic. The team considers ECT. Which statement about the FDA classification of ECT devices for this indication is correct?**

**A.** Mania is a class II indication, and the device is cleared for it

**B.** Mania remains a class III use, yet a physician may use a cleared device off-label

**C.** FDA prohibits the use of ECT devices for mania

**D.** Mania is exempt from device regulation

**E.** Mania is a class II indication only under age 13

Answer Contents

**BQ-161**

**A 74-year-old woman with severe depression is starting ECT. Her family worries about memory, and baseline testing shows mild impairment. Which approach best fits guideline reasoning for limiting cognitive effects while preserving efficacy?**

**A.** Bitemporal brief pulse at 1.5 to 2.0 times seizure threshold

**B.** Bilateral ultrabrief pulse at a low multiple of threshold

**C.** Bitemporal placement at the highest available energy

**D.** Right unilateral ultrabrief pulse

**E.** Bifrontal placement at the highest available energy

Answer Contents

**BQ-162**

**A 45-year-old woman has received five right unilateral ECT treatments with minimal improvement. Which is the best next step according to CANMAT 2016?**

**A.** Switch to bilateral placement

**B.** Stop ECT and declare her resistant to it

**C.** Increase to five treatments each week

**D.** Add lithium and continue unchanged placement

**E.** Wait three months before reassessing

Answer Contents

**BQ-163**

**Three weeks after finishing an ECT course, a 67-year-old man asks whether his slowed thinking will last. Which statement is best supported by meta-analytic data on objective cognitive testing?**

**A.** Most test deficits worsen over the following months

**B.** Deficits remain at their lowest point after 15 days

**C.** Deficits are permanent in most older adults

**D.** Only anterograde memory is ever affected

**E.** Deficits mostly resolve within two weeks, and some functions exceed baseline

Answer Contents

**BQ-164**

**A 58-year-old woman has remitted with ECT for psychotic depression. Which relapse-prevention plan is best supported by CANMAT 2016?**

**A.** Stop all treatment because remission is complete

**B.** Resume the same antidepressant that failed before ECT

**C.** Start lithium with nortriptyline or venlafaxine, with continuation ECT if needed

**D.** Use a benzodiazepine alone

**E.** Use bright light therapy alone

Answer Contents

**BQ-165**

**A busy clinic wants to treat more patients per day with rTMS. Which finding best supports replacing 10 Hz stimulation with intermittent theta burst?**

**A.** Theta burst was superior to 10 Hz in remission

**B.** Theta burst was noninferior and each session lasted about 3 minutes rather than 37.5

**C.** Theta burst caused no headaches

**D.** Theta burst needs no motor threshold

**E.** Theta burst requires no seizure screening

Answer Contents

**BQ-166**

**A 36-year-old woman has recurrent winter depression of moderate severity and prefers not to start medication. She asks about light therapy. Which counseling is most accurate?**

**A.** Evidence is strong enough to replace all treatment

**B.** A dim lamp at bedtime is the standard regimen

**C.** Light is contraindicated in depression

**D.** Use is limited to hospital settings

**E.** A morning light box is a reasonable low-risk option after screening

Answer Contents

**BQ-167**

**A 44-year-old man with treatment-resistant depression arrives for an esketamine session. His pre-dose blood pressure is 152/96 mmHg and he feels well. Which action is most consistent with the label?**

**A.** Weigh the risk of a pressure rise, and withhold the dose if it is serious

**B.** Dose as planned and skip blood pressure checks

**C.** Give a nasal steroid after the dose

**D.** Shorten observation to 30 minutes

**E.** Discontinue esketamine permanently

Answer Contents

**BQ-168**

**A 50-year-old woman with treatment-resistant depression had an intracerebral hemorrhage three years ago and asks about esketamine. What is the best response?**

**A.** Proceed at half the usual dose

**B.** Proceed with longer monitoring

**C.** Proceed at home with a family member present

**D.** Do not give it, because prior hemorrhage is a contraindication

**E.** Give the drug intravenously instead

Answer Contents

**BQ-169**

**A patient with treatment-resistant depression has completed four weeks of twice-weekly esketamine with a good response. What is the label schedule for weeks 5 to 8?**

**A.** Three times weekly

**B.** Twice weekly, unchanged

**C.** Once weekly

**D.** Every four weeks

**E.** Daily at home

Answer Contents

**BQ-170**

**A psychiatrist plans IV ketamine for a 39-year-old with treatment-resistant depression, no psychosis, and controlled blood pressure. Which plan matches the APA consensus statement and the CANMAT task force?**

**A.** 0.5 mg/kg over 40 minutes with cardiac and oxygen monitoring and an escort

**B.** Compounded oral lozenges taken daily at home

**C.** Rapid bolus in an unmonitored room with no escort

**D.** Compounded nasal spray used at home

**E.** Weekly infusions indefinitely without reassessment

Answer Contents

**BQ-171**

**A patient shows you compounded oral ketamine mailed each month by a telehealth company for anxiety and PTSD. Which statement reflects the FDA warning of October 2023?**

**A.** It is approved when prescribed by a licensed clinician

**B.** It is not approved, has not been evaluated, and has no REMS

**C.** It is equivalent to esketamine nasal spray

**D.** It is approved for PTSD but not for anxiety

**E.** It is safer than esketamine because doses are lower

Answer Contents

**BQ-172**

**A 48-year-old man has chronic depression that has failed five antidepressant treatments. Which describes the FDA-approved indication for vagus nerve stimulation?**

**A.** First-line therapy for a first depressive episode

**B.** Treatment after one failed antidepressant

**C.** Acute treatment of manic episodes

**D.** Epilepsy only, with no mood indication

**E.** Adjunctive treatment after four or more failed antidepressant treatments

Answer Contents

**BQ-173**

**A 35-year-old woman with severe OCD has failed three SSRIs and cognitive-behavioral therapy. Which statement about deep brain stimulation is accurate?**

**A.** It is approved for treatment-resistant depression

**B.** It has no FDA status for any condition

**C.** It requires a prior failed course of ECT

**D.** It has a humanitarian device exemption for severe, treatment-resistant OCD

**E.** It is approved only for adolescents with OCD

Answer Contents

**BQ-174**

**A 40-year-old man with moderate major depression that is not medication refractory asks about a home brain stimulation headset. Which statement is accurate?**

**A.** No tDCS device has ever received FDA authorization

**B.** FDA approval covers only treatment-resistant patients

**C.** One home-use headset was approved in December 2025 for non-refractory depression

**D.** VA/DoD recommends tDCS as first-line therapy

**E.** FDA approved it for acute bipolar mania

Answer Contents

**BQ-175**

**A 29-year-old woman with treatment-resistant depression asks to take psilocybin at a retreat. Which counseling is best?**

**A.** It is not FDA approved, and VA/DoD advises against it outside research

**B.** It is approved and covered if a physician refers her

**C.** It is approved only for PTSD

**D.** It is safer than any approved treatment

**E.** It is a first-line option in VA/DoD 2022

Answer Contents

SUBJECT 9

# Depressive Disorders

Questions BQ-176 to BQ-202

**BQ-176**

**A 45-year-old man begins treatment for major depression in a primary care clinic. The clinic asks how to track his progress. Which approach is most consistent with the 2022 VA/DoD guideline?**

**A.** Rely on the clinician's impression at each visit

**B.** Measure severity only at baseline

**C.** Repeat a measure only if the patient reports worsening

**D.** Use a quantitative severity measure at planning and at regular intervals

**E.** Track serum biomarkers of inflammation

Answer Contents

**BQ-177**

**The director of a primary care network asks which organizational model for treating major depression carries a strong VA/DoD recommendation. Which should you name?**

**A.** Collaborative or integrated care

**B.** Antidepressant monotherapy in every case

**C.** Referral of every patient to a psychiatrist

**D.** Telehealth-only visits

**E.** Peer support without clinicians

Answer Contents

**BQ-178**

**A 16-year-old has had depressed or irritable mood on most days for 14 months, with poor appetite, low self-esteem, and poor concentration, and no clear break. He has never had elevated mood. Outbursts are not prominent. Which diagnosis fits best?**

**A.** Single-episode major depressive disorder

**B.** Adjustment disorder with depressed mood

**C.** Persistent depressive disorder

**D.** Disruptive mood dysregulation disorder

**E.** Cyclothymic disorder

Answer Contents

**BQ-179**

**A 33-year-old woman has had major depressive episodes every November through March for four years, each remitting in spring. She wants prevention. Which plan matches the bupropion XL label for seasonal affective disorder?**

**A.** Start in March and continue through summer

**B.** Start when symptoms begin and stop at remission

**C.** Take it all year at the maximum dose

**D.** Use only on days with short daylight

**E.** Start in autumn before symptoms and taper in early spring

Answer Contents

**BQ-180**

**A 57-year-old man with severe depression cannot enjoy anything, does not brighten at good news, wakes at 4 a.m. feeling worst in the morning, is markedly slowed, has lost 12 pounds, and feels guilty. Which specifier applies?**

**A.** Atypical features

**B.** Melancholic features

**C.** Seasonal pattern

**D.** Anxious distress

**E.** Peripartum onset

Answer Contents

**BQ-181**

**A 50-year-old woman lost her husband three months ago. She has waves of yearning and tearfulness triggered by reminders, mixed with warm memories. She sleeps adequately, keeps her self-respect, and denies thoughts of suicide. What is the best approach?**

**A.** Start an antidepressant for major depression

**B.** Admit to hospital

**C.** Start a benzodiazepine

**D.** Offer support and monitor for a full depressive syndrome

**E.** Refer for electroconvulsive therapy

Answer Contents

**BQ-182**

**A 31-year-old woman has a depressive episode with racing thoughts, increased energy, and a reduced need for sleep that do not reach full mania. Which specifier fits, and what does it signal?**

**A.** Mixed features, raising concern for bipolar disorder

**B.** Atypical features, confirming unipolar illness

**C.** Melancholic features, predicting a good response to SSRIs

**D.** Anxious distress, ruling out bipolar disorder

**E.** Psychotic features, requiring an antipsychotic

Answer Contents

**BQ-183**

**A 52-year-old woman reports fatigue, low mood, weight gain, constipation, and cold intolerance for three months. Which test is the best initial step in evaluating a medical contribution?**

**A.** Brain MRI

**B.** Thyroid-stimulating hormone

**C.** Electroencephalogram

**D.** Polysomnography

**E.** Lumbar puncture

Answer Contents

**BQ-184**

**A 31-year-old woman with confirmed premenstrual dysphoric disorder prefers intermittent sertraline. Which dosing plan matches the US label?**

**A.** Start on the first day of menses, to a maximum of 150 mg daily

**B.** Start on cycle day 1, to a maximum of 200 mg daily

**C.** Start 7 days before expected menses, to a maximum of 50 mg daily

**D.** Start at ovulation and stop after 3 days

**E.** Start 14 days before expected menses, to a maximum of 100 mg daily

Answer Contents

**BQ-185**

**A 27-year-old woman with confirmed premenstrual dysphoric disorder wants treatment that also provides contraception. Which statement about oral contraceptives reflects the evidence summarized in a recent review?**

**A.** Any 21-day combined pill relieves symptoms

**B.** A GnRH agonist is the first-line contraceptive option

**C.** A 24-day active drospirenone pill reduced symptoms versus placebo

**D.** Progestin-only pills are the best-studied option

**E.** Oral contraceptives are contraindicated

Answer Contents

**BQ-186**

**A 5-year-old has had severe outbursts five times a week for 14 months at home and in daycare, with irritable mood between outbursts. A colleague suggests disruptive mood dysregulation disorder. What is the best response?**

**A.** Make the diagnosis, because duration and frequency are met

**B.** The diagnosis is not made before age 6, so another formulation is needed

**C.** Diagnose bipolar I disorder

**D.** Diagnose major depressive disorder

**E.** Diagnose persistent depressive disorder

Answer Contents

**BQ-187**

**A 21-year-old is about to start an antidepressant. Which statement matches the age-stratified boxed warning data in the US label?**

**A.** There are 14 fewer suicidal events per 1,000 than with placebo

**B.** There are 5 fewer events per 1,000 than with placebo

**C.** There is no difference from placebo

**D.** There are 5 more events per 1,000 than with placebo

**E.** There are 14 more events per 1,000 than with placebo

Answer Contents

**BQ-188**

**A 45-year-old man with major depression and insomnia starts mirtazapine. At six weeks he reports sleeping well but is hungrier and has gained weight. Which adverse effect profile is most characteristic of this drug?**

**A.** Weight gain and somnolence

**B.** Ejaculation failure

**C.** Sustained hypertension

**D.** Seizures

**E.** Insomnia

Answer Contents

**BQ-189**

**A 24-year-old woman with major depression has a history of bulimia nervosa. Which antidepressant is contraindicated by the label?**

**A.** Sertraline

**B.** Escitalopram

**C.** Bupropion

**D.** Mirtazapine

**E.** Venlafaxine

Answer Contents

**BQ-190**

**A 55-year-old man with controlled hypertension is starting venlafaxine XR. Which monitoring plan matches the label?**

**A.** No blood pressure monitoring is needed with SNRIs
**B.** Check blood pressure only if headache occurs
**C.** Check blood pressure only after the dose exceeds 300 mg
**D.** Check blood pressure once, at baseline
**E.** Measure it before starting and regularly during treatment

Answer Contents

**BQ-191**

**A patient on sertraline receives linezolid for an infection. Two days later she has agitation, tremor, clonus, sweating, and a temperature of 38.9 degrees C. What is the best next step?**

**A.** Continue both drugs and add haloperidol
**B.** Stop the serotonergic drugs and give supportive care
**C.** Increase the sertraline dose
**D.** Give dantrolene alone
**E.** Start lithium

Answer Contents

**BQ-192**

**A woman who has taken fluoxetine for six months will switch to an MAOI because of treatment failure. According to the fluoxetine label, how long should she wait after stopping fluoxetine?**

**A.** 3 days
**B.** 7 days
**C.** 14 days
**D.** At least 5 weeks
**E.** 12 weeks

Answer Contents

**BQ-193**

**A 49-year-old man with partial response to escitalopram starts adjunctive brexpiprazole. Two weeks later he cannot sit still, paces, and feels inner restlessness. What is the most likely cause?**

**A.** Akathisia

**B.** Serotonin syndrome

**C.** Tardive dyskinesia

**D.** Acute dystonia

**E.** Relapse of anxiety

Answer Contents

**BQ-194**

**A 51-year-old woman with partial response to venlafaxine will start adjunctive quetiapine XR. Which baseline plan is consistent with the label?**

**A.** No laboratory tests are needed

**B.** An electroencephalogram

**C.** Fasting glucose and lipids, with regular weight checks

**D.** A urine drug screen only

**E.** A chest radiograph

Answer Contents

**BQ-195**

**A 63-year-old man will start lithium augmentation for depression. According to NICE, which checks come before starting?**

**A.** Liver enzymes and a blood count

**B.** Lipid panel and fasting glucose

**C.** Head CT and an EEG

**D.** ECG and serum potassium only

**E.** Weight, renal and thyroid function, and calcium

Answer Contents

**BQ-196**

**A 39-year-old woman with major depression has had no response to two adequate antidepressant trials. She declines ECT for now. According to the 2022 VA/DoD guideline, which option is supported?**

**A.** Deep brain stimulation in routine practice

**B.** Repetitive transcranial magnetic stimulation (weak for)

**C.** Vagus nerve stimulation as a routine step

**D.** Psilocybin as a routine step

**E.** Omega-3 fatty acids as treatment

Answer Contents

**BQ-197**

**A clinic wants to give esketamine at home to a reliable patient to save travel. Which statement is accurate?**

**A.** It may be self-administered after four weeks

**B.** It may be dispensed to clinicians′ relatives

**C.** It may be taken at home if the patient has a monitor

**D.** It requires a REMS-certified setting and 2 hours of observation

**E.** It may be given at home on the weekly schedule

Answer Contents

**BQ-198**

**In the STAR*D sequenced-treatment study, how did remission rates change across the four treatment steps?**

**A.** They fell from 36.8% at step 1 to 13.0% at step 4

**B.** They rose from 13.0% at step 1 to 36.8% at step 4

**C.** They were about 30% at every step

**D.** They were highest at step 3

**E.** They were not reported

Answer Contents

**BQ-199**

**A 35-year-old woman with a first depressive episode remitted on sertraline two months ago. She wants to stop. What is the best advice under VA/DoD guidance?**

**A.** Stop now, because she is well

**B.** Taper over two weeks and stop

**C.** Continue the therapeutic dose for at least 6 months after remission

**D.** Switch to a herbal product

**E.** Stop and begin omega-3 fatty acids

Answer Contents

**BQ-200**

**A 44-year-old man with three previous episodes is in remission on an antidepressant. Which addition does VA/DoD weakly support to reduce relapse?**

**A.** Vitamin D supplementation

**B.** A continuation course of CBT, IPT, or MBCT

**C.** Omega-3 fatty acid supplements

**D.** Switching to psychotherapy alone

**E.** Doubling the antidepressant dose

Answer Contents

**BQ-201**

**A 28-year-old woman at 10 weeks of pregnancy has mild to moderate major depression and takes no medication. Which first-line approach carries a strong VA/DoD recommendation?**

**A.** Start sertraline at once

**B.** Esketamine

**C.** ECT

**D.** Bupropion

**E.** An evidence-based psychotherapy

Answer Contents

**BQ-202**

**A 38-year-old woman who is not pregnant has mild depression and prefers an herbal remedy. How do VA/DoD (2022) and NICE (2022) differ on St. John′s wort?**

**A.** Both strongly recommend it

**B.** Both advise against it

**C.** VA/DoD strongly recommends against it and NICE recommends it

**D.** VA/DoD weakly suggests it for mild depression; NICE advises against it

**E.** Neither guideline mentions it

Answer Contents

SUBJECT 10

# Bipolar and Related Disorders

Questions BQ-203 to BQ-224

**BQ-203**

**A 24-year-old man is brought in by police after not sleeping for three nights. For ten days he has been elated, talking rapidly, and starting several projects. He believes he is a chosen messenger and hears voices that confirm it. He is admitted to the hospital. He has never had a mood episode before and has no substance use or medical illness. Which diagnosis is most appropriate?**

**A.** Brief psychotic disorder

**B.** Bipolar II disorder

**C.** Bipolar I disorder

**D.** Schizophreniform disorder

**E.** Cyclothymic disorder

Answer Contents

**BQ-204**

**A 26-year-old woman has had, for three years, many periods of mildly elevated mood and energy and many periods of low mood and fatigue. None has reached the level of a manic or major depressive episode, and she has never been symptom-free for more than a few weeks. Which diagnosis fits best?**

**A.** Cyclothymic disorder

**B.** Bipolar II disorder

**C.** Bipolar I disorder

**D.** Persistent depressive disorder

**E.** Other specified depressive disorder

Answer Contents

**BQ-205**

**A 52-year-old man with vasculitis started high-dose prednisone five days ago. He now sleeps two hours a night, is euphoric, and speaks rapidly. He has no psychiatric history. Which diagnosis is most likely?**

**A.** Bipolar I disorder

**B.** Bipolar II disorder

**C.** Bipolar disorder due to hyperthyroidism

**D.** Substance/medication-induced bipolar and related disorder

**E.** Cyclothymic disorder

Answer Contents

**BQ-206**

**A 33-year-old woman became euphoric, grandiose, and sleepless six weeks into treatment with sertraline for depression. The sertraline was stopped. Six weeks later the full manic syndrome continues. Which diagnosis is most appropriate?**

**A.** Substance/medication-induced bipolar and related disorder

**B.** Bipolar I disorder

**C.** Bipolar II disorder

**D.** Major depressive disorder with mixed features

**E.** Adjustment disorder with elevated mood

Answer Contents

**BQ-207**

**A 45-year-old woman with depression reports racing thoughts and four hours of sleep a night. She paces, is irritable, and says she would be better off dead. She has never taken a mood stabilizer. What is the best approach?**

**A.** Raise the antidepressant dose and review in a month

**B.** Add a benzodiazepine and discharge with follow-up

**C.** Add a stimulant to treat the low energy

**D.** Start a hypnotic and treat as unipolar depression

**E.** Stop antidepressants, start an antipsychotic, and assess safety

Answer Contents

**BQ-208**

**A 31-year-old man has taken lithium for three years. He has had five distinct mood episodes in the past year and now reports fatigue, cold intolerance, and weight gain. Which test is the best next step?**

**A.** Serum ammonia
**B.** HLA-B*1502 genotype
**C.** Thyroid-stimulating hormone
**D.** Lamotrigine level
**E.** Fasting lipids

Answer Contents

**BQ-209**

**A 28-year-old woman had a manic episode with delusions. After her mood symptoms resolved, the delusions and hallucinations continued for another five weeks. Which diagnosis is most likely?**

**A.** Bipolar I disorder with psychotic features
**B.** Schizoaffective disorder
**C.** Brief psychotic disorder
**D.** Delusional disorder
**E.** Substance-induced psychotic disorder

Answer Contents

**BQ-210**

**A 24-year-old man has been distractible, impulsive, and restless since age 7 and has poor concentration at all times. He sleeps seven hours a night, has never had a stretch of elevated mood, and talks a lot but not under pressure. Which diagnosis is most likely?**

**A.** Attention-deficit/hyperactivity disorder
**B.** Bipolar I disorder
**C.** Bipolar II disorder
**D.** Cyclothymic disorder
**E.** Mania due to hyperthyroidism

Answer Contents

**BQ-211**

**A 35-year-old woman with bipolar I disorder will start lithium. Which pre-treatment evaluation is most appropriate?**

**A.** Serum ammonia, lipase, and a pregnancy test

**B.** HLA-B*1502 genotype and a complete blood count

**C.** Liver tests and platelet count only

**D.** Creatinine, electrolytes, thyroid-stimulating hormone, and pregnancy test

**E.** Fasting lipids and prolactin only

Answer Contents

**BQ-212**

**A patient started lithium 300 mg three times daily three days ago. When should the first serum lithium level be drawn?**

**A.** Two hours after the morning dose

**B.** Just before bedtime

**C.** Immediately after the third daily dose

**D.** At any time on day 3

**E.** About 12 hours after the last dose, around day 3

Answer Contents

**BQ-213**

**A 29-year-old man with bipolar I disorder was discharged yesterday after a suicide attempt. He takes lithium. Which prescribing step is most appropriate?**

**A.** Prescribe a 90-day supply to improve adherence

**B.** Limit the quantity dispensed and arrange close follow-up with levels

**C.** Stop lithium because it is dangerous in overdose

**D.** Stop all medication until he is stable

**E.** Skip suicide reassessment because lithium protects against suicide

Answer Contents

**BQ-214**

**A 59-year-old woman taking lithium arrives confused and ataxic, then has a seizure. Her lithium level is 3.4 mEq/L and her creatinine is elevated. Which treatment is most appropriate?**

**A.** Activated charcoal alone

**B.** A specific lithium antidote

**C.** Hemodialysis

**D.** A thiazide diuretic

**E.** Strict sodium restriction

Answer Contents

**BQ-215**

**A 50-year-old man has taken lithium for 15 years. He now drinks large volumes of water and urinates frequently, and his urine is dilute. Which cause is most likely?**

**A.** Lithium-related hyperthyroidism

**B.** Psychogenic polydipsia only

**C.** New-onset type 2 diabetes mellitus

**D.** Lithium-related nephrogenic diabetes insipidus

**E.** Lithium-related hyperparathyroidism

Answer Contents

**BQ-216**

**A 37-year-old woman has taken divalproex for five weeks. She becomes lethargic and vomits. Her liver tests are normal. Which test is the best next step?**

**A.** Serum ammonia

**B.** Serum lithium

**C.** Thyroid-stimulating hormone

**D.** HLA-B*1502 genotype

**E.** Urine toxicology

Answer Contents

**BQ-217**

**A 28-year-old man who started divalproex three weeks ago develops severe epigastric pain radiating to the back, with vomiting. What is the best next step?**

**A.** Halve the dose and continue

**B.** Give an antiemetic and observe

**C.** Give an NSAID for pain

**D.** Check a valproate level and continue

**E.** Stop divalproex and evaluate for pancreatitis

Answer Contents

**BQ-218**

**A 33-year-old woman is in week 3 of lamotrigine titration. She develops a rash, fever, and painful mouth sores. What is the best next step?**

**A.** Slow the titration and continue

**B.** Stop lamotrigine and arrange urgent evaluation

**C.** Add an antihistamine and continue

**D.** Hold one dose, then restart

**E.** Return to the previous lower dose

Answer Contents

**BQ-219**

**A 27-year-old woman has been stable on lamotrigine 200 mg daily. She starts an estrogen-containing contraceptive. What is the expected effect?**

**A.** Lamotrigine levels rise and toxicity appears

**B.** No change in lamotrigine levels

**C.** The contraceptive loses effectiveness

**D.** Lamotrigine levels fall, so the dose may need to increase

**E.** The risk of rash disappears

Answer Contents

**BQ-220**

**A 40-year-old man of Asian ancestry will start extended-release carbamazepine for mania. Which test should be completed before the first dose?**

**A.** Serum lithium

**B.** Thiopurine methyltransferase activity

**C.** HLA-B*1502 genotype

**D.** CYP2D6 genotype

**E.** Echocardiogram

Answer Contents

**BQ-221**

**A 30-year-old woman with bipolar I depression takes lurasidone 60 mg each morning on an empty stomach and has had little benefit. Which change is best?**

**A.** Take it with a meal of at least 350 calories

**B.** Take it with grapefruit juice

**C.** Switch to a once-weekly schedule

**D.** Add an antidepressant

**E.** Take the dose at bedtime without food

Answer Contents

**BQ-222**

**A 30-year-old woman with bipolar I disorder has been stable on lithium for four years. She learns she is seven weeks pregnant. What is the best approach?**

**A.** Stop lithium abruptly to avoid fetal exposure

**B.** Switch to valproate before the second trimester

**C.** Continue the dose without further monitoring

**D.** Continue the lowest effective dose with shared decision-making

**E.** Switch to carbamazepine before the second trimester

Answer Contents

**BQ-223**

**A 15-year-old with bipolar I disorder is in a depressive episode. Which single agent does CANMAT/ISBD list as first-line for pediatric bipolar depression?**

**A.** Fluoxetine

**B.** Bupropion

**C.** Valproate

**D.** Lamotrigine

**E.** Lurasidone

Answer Contents

**BQ-224**

**A 72-year-old man with bipolar I disorder has had stable lithium levels for years. His primary care clinician starts lisinopril for hypertension. What is the best response?**

**A.** No change is needed because the dose is stable

**B.** Check a lithium level and kidney function soon

**C.** Halve the lithium dose in advance

**D.** Switch lithium to valproate

**E.** Add a diuretic to offset the effect

Answer Contents

SUBJECT 11

# Anxiety and Obsessive-Compulsive Disorders

Questions BQ-225 to BQ-251

**BQ-225**

**A 9-year-old boy has had six months of excessive worry about school, his family's safety, and his health, with stomachaches and poor sleep. CBT is under way. His parents ask which of the following has an FDA indication for generalized anxiety disorder at his age. Which drug fits?**

**A.** Paroxetine

**B.** Venlafaxine extended-release

**C.** Duloxetine

**D.** Buspirone

**E.** Sertraline

Answer Contents

**BQ-226**

**A 24-year-old graduate student has avoided seminar presentations and group meals for more than a year because she fears being judged. She prefers psychotherapy to medication. Which is the best psychological option?**

**A.** Group CBT, chosen by default for cost

**B.** Weekly supportive therapy

**C.** Individual CBT developed for social anxiety disorder

**D.** Alprazolam before each seminar

**E.** Daily propranolol

Answer Contents

**BQ-227**

**A 33-year-old man with panic disorder has had no improvement after a 12-week course of sertraline at an adequate dose, and he declines CBT. According to NICE, which option may be considered next?**

**A.** Long-term alprazolam

**B.** An antipsychotic add-on

**C.** Stopping all drug treatment

**D.** Buspirone alone

**E.** Imipramine or clomipramine

Answer Contents

**BQ-228**

**A 35-year-old woman has a lifelong, intense fear of flying and wants lorazepam before each flight. She declines therapy. Which advice is best?**

**A.** Take lorazepam before every flight

**B.** Try exposure-based therapy, ideally with in vivo exposure

**C.** Start a daily SSRI

**D.** Use relaxation recordings alone

**E.** Avoid flying permanently

Answer Contents

**BQ-229**

**A 6-year-old girl speaks freely at home but has not spoken at school for four months since she started kindergarten. Hearing and language are normal for her age, and she is shy and clingy. Which is the best first step?**

**A.** Start an antipsychotic

**B.** Apply consequences for silence

**C.** Prescribe alprazolam

**D.** Begin a graded behavioral program coordinated with the school

**E.** Wait until puberty

Answer Contents

**BQ-230**

**A pediatrician asks how the antidepressant boxed warning differs by age in the label. Which pattern is correct?**

**A.** Fewer events than placebo under age 18

**B.** The same excess in every age group

**C.** Excess suicidal events under 25, with fewer events in adults 65 and older

**D.** Excess events only after age 65

**E.** No age data are given

Answer Contents

**BQ-231**

**A 19-year-old believes with great conviction that if he does not tap a door frame seven times his mother will die, and he taps for hours. He has no hallucinations, disorganization, or other delusions. Which is the most likely diagnosis?**

**A.** Obsessive-compulsive disorder with poor insight

**B.** Schizophrenia

**C.** Delusional disorder

**D.** Obsessive-compulsive personality disorder

**E.** Illness anxiety disorder

Answer Contents

**BQ-232**

**A 38-year-old accountant is rigidly perfectionistic, works excessively, and cannot delegate. He believes his standards are correct, has no intrusive thoughts, and performs no rituals to ward off harm. Which is the most likely diagnosis?**

**A.** Obsessive-compulsive disorder

**B.** Generalized anxiety disorder

**C.** Autism spectrum disorder

**D.** Obsessive-compulsive personality disorder

**E.** Hoarding disorder

Answer Contents

**BQ-233**

**Which patient meets the FDA labeling for deep brain stimulation for obsessive-compulsive disorder?**

**A.** A 35-year-old whose symptoms improved on sertraline

**B.** A 52-year-old with two SSRI failures who has never had CBT

**C.** A 17-year-old with severe OCD

**D.** A 40-year-old with active psychosis and mild OCD

**E.** A 41-year-old with chronic, severe OCD who has failed at least three SSRIs

Answer Contents

**BQ-234**

**A 38-year-old with refractory OCD on a stable SSRI dose is considering deep transcranial magnetic stimulation. Which finding is a contraindication?**

**A.** Prior sertraline use

**B.** A cochlear implant

**C.** Comorbid depression

**D.** Female sex

**E.** A stable medication dose

Answer Contents

**BQ-235**

**A 31-year-old man with OCD will start clomipramine and asks about seizures. Which statement matches the label?**

**A.** Risk is negligible and does not depend on dose

**B.** Seizures occur only in people with known epilepsy

**C.** Risk disappears after the first month of treatment

**D.** Risk arises only in overdose, not at therapeutic doses

**E.** Cumulative risk rises with time, about 1.45 percent at one year at doses up to 300 mg daily

Answer Contents

**BQ-236**

**A 44-year-old man with OCD takes fluvoxamine 200 mg daily and develops spasticity after a spinal injury. Which muscle relaxant is contraindicated with fluvoxamine?**

**A.** Baclofen

**B.** Gabapentin

**C.** Lorazepam

**D.** Tizanidine

**E.** Dantrolene

Answer Contents

**BQ-237**

**A 36-year-old man with panic disorder has taken fluoxetine 40 mg daily for a year without remission, and a psychiatrist plans phenelzine. How long should he wait after stopping fluoxetine?**

**A.** 24 hours

**B.** At least 5 weeks

**C.** 7 days

**D.** 14 days

**E.** No wait if the dose is halved first

Answer Contents

**BQ-238**

**Which statement about the Pediatric OCD Treatment Study (112 patients aged 7 to 17) is correct?**

**A.** Combined treatment beat CBT alone, sertraline alone, and placebo on remission

**B.** Sertraline alone outperformed CBT alone

**C.** The combination was no better than placebo

**D.** CBT alone was no better than placebo

**E.** More than 20 percent of placebo patients reached remission

Answer Contents

**BQ-239**

**In a placebo-controlled trial of 122 adults with OCD, which conclusion is supported at week 12?**

**A.** Clomipramine alone beat exposure and ritual prevention alone

**B.** Clomipramine was no better than placebo

**C.** Exposure and ritual prevention alone matched the combination; both beat clomipramine

**D.** The combination beat exposure and ritual prevention alone

**E.** Placebo matched every active arm

Answer Contents

**BQ-240**

**A 33-year-old woman has a partial response after 12 weeks of an SSRI at the highest tolerated dose plus well-delivered ERP. Which add-on has support from pooled placebo-controlled trials?**

**A.** Alprazolam

**B.** Risperidone

**C.** Propranolol

**D.** Zolpidem

**E.** Gabapentin

Answer Contents

**BQ-241**

**A 30-year-old man is completely convinced that his skin is scarred and ugly, though no scars are visible. He spends hours camouflaging and checking mirrors. He has no other psychotic symptoms. Which is the most likely diagnosis?**

**A.** Delusional disorder, somatic type

**B.** Schizophrenia

**C.** Social anxiety disorder

**D.** Illness anxiety disorder

**E.** Body dysmorphic disorder with absent insight

Answer Contents

**BQ-242**

**A 29-year-old woman pulls hair from her scalp, mostly while reading, leaving a thin patch. She has tried repeatedly to stop. Skin examination is normal. Which is the best first-line treatment?**

**A.** An antipsychotic

**B.** A benzodiazepine

**C.** Habit reversal training

**D.** Shaving her scalp

**E.** A topical steroid

Answer Contents

**BQ-243**

**A 20-year-old has recurrent severe anxiety episodes that began within an hour of cannabis use, serious enough to bring him to the emergency department twice. He has had no attacks in three months of abstinence. Which is the most likely diagnosis?**

**A.** Panic disorder

**B.** Generalized anxiety disorder

**C.** Specific phobia

**D.** Substance/medication-induced anxiety disorder

**E.** Anxiety disorder due to another medical condition

Answer Contents

**BQ-244**

**A 27-year-old man started an antipsychotic one week ago. He now reports intense inner restlessness, cannot sit through meals, and believes his anxiety has worsened. Which is the best next step?**

**A.** Assess for akathisia and review the antipsychotic dose

**B.** Add alprazolam for anxiety

**C.** Start sertraline

**D.** Increase the antipsychotic dose

**E.** Refer for CBT

Answer Contents

**BQ-245**

**A 70-year-old woman with generalized anxiety disorder has a QTc of 520 ms on ECG. She asks for an as-needed medication. Which drug is contraindicated by its label?**

**A.** Buspirone
**B.** Sertraline
**C.** Duloxetine
**D.** Hydroxyzine
**E.** Pregabalin

Answer Contents

**BQ-246**

**A 30-year-old woman planning pregnancy takes paroxetine 40 mg daily for well-controlled panic disorder. Which counseling reflects the paroxetine label?**

**A.** Paroxetine has no pregnancy-related warning
**B.** The label reports under a 2-fold rise in cardiovascular malformations, so weigh alternatives against relapse risk
**C.** Stop paroxetine at once and offer no alternative
**D.** Switch to alprazolam for the entire pregnancy
**E.** Pregnancy contraindicates every SSRI

Answer Contents

**BQ-247**

**A resident asks what the September 2020 FDA update to the benzodiazepine boxed warning added. Which is correct?**

**A.** Added warnings of abuse, misuse, addiction, dependence, and withdrawal beside the opioid warning
**B.** Withdrawal of the class from the market
**C.** A seven-day limit on every prescription
**D.** A hepatotoxicity warning
**E.** A REMS enrollment requirement

Answer Contents

**BQ-248**

**A 50-year-old woman has taken alprazolam 2 mg daily for five years. She asks to switch to buspirone and stop alprazolam within days because "buspirone treats anxiety." Which response is best?**

**A.** Agree, since buspirone prevents withdrawal

**B.** Add flumazenil to speed the switch

**C.** Explain that buspirone does not block benzodiazepine withdrawal, so taper alprazolam gradually

**D.** Switch to zolpidem

**E.** Stop alprazolam abruptly and monitor

Answer Contents

**BQ-249**

**A 38-year-old man with generalized anxiety disorder is titrated to venlafaxine extended-release 225 mg daily. Which monitoring is most relevant at each visit?**

**A.** Serum lithium

**B.** Absolute neutrophil count

**C.** Fasting lipids

**D.** Platelet count

**E.** Blood pressure

Answer Contents

**BQ-250**

**A 24-year-old woman starts escitalopram for generalized anxiety disorder. According to NICE, which follow-up is best?**

**A.** See her within a week and check suicidal thinking weekly for the first month

**B.** Review at 6 months

**C.** No scheduled follow-up because she is an adult

**D.** Review at 12 weeks

**E.** Monthly blood tests

Answer Contents

**BQ-251**

**An 11-year-old boy has moderate OCD with contamination fears and daily rituals. Which is the best first-line treatment?**

**A.** Sertraline at the maximum dose alone

**B.** Clomipramine

**C.** Alprazolam

**D.** Family-involved CBT with ERP adapted to his age

**E.** Risperidone

Answer Contents

SUBJECT 12

# Trauma, Stressor, and Dissociative Disorders

Questions BQ-252 to BQ-273

**BQ-252**

**A primary care clinic wants a brief PTSD screen that opens with a lifetime trauma exposure question and then asks five yes or no items about the past month, scored 0 to 5. Which tool is this?**

**A.** PTSD Checklist for DSM-5

**B.** Primary Care PTSD Screen for DSM-5

**C.** Clinician-Administered PTSD Scale for DSM-5

**D.** Patient Health Questionnaire-9

**E.** Generalized Anxiety Disorder-7

Answer Contents

**BQ-253**

**A veteran's PTSD Checklist for DSM-5 score falls from 54 to 41 after 10 weeks of cognitive processing therapy. How should you interpret the change?**

**A.** Remission, because the score fell by more than 10 points

**B.** No meaningful change, because the score remains above 40

**C.** Clinically significant change, because the score is below 50

**D.** A meaningful response, though the score remains above the probable-PTSD range

**E.** Treatment failure, because the score is not below 28

Answer Contents

**BQ-254**

**A 34-year-old woman was assaulted 10 days ago and meets criteria for acute stress disorder. She asks what can lower her chance of developing PTSD. Which option has VA/DoD support?**

**A.** Individual trauma-focused cognitive behavioral therapy

**B.** Sertraline started now

**C.** Lorazepam for the first month

**D.** A single-session psychological debriefing

**E.** Prazosin at bedtime

Answer Contents

**BQ-255**

**A 38-year-old man develops anxiety, tearfulness, and irritability 5 weeks after being laid off. He meets criteria for no other disorder, but job-search tasks have become impossible. Which statement about his expected course is most accurate?**

**A.** The diagnosis requires symptoms for 12 months

**B.** Symptoms usually become chronic after the stressor ends

**C.** The diagnosis requires a life-threatening event

**D.** He needs an antidepressant for at least 6 months

**E.** Symptoms usually resolve within about 6 months of the stressor ending

Answer Contents

**BQ-256**

**A 50-year-old paramedic had a few intrusive memories after a mass casualty event but worked normally. At 8 months he develops nightmares, avoidance, hypervigilance, and impairment. Which diagnosis fits best?**

**A.** Acute stress disorder

**B.** Adjustment disorder with anxiety

**C.** PTSD with delayed expression

**D.** PTSD with dissociative symptoms

**E.** Panic disorder

Answer Contents

**BQ-257**

**A 40-year-old man is starting sertraline for PTSD. Which plan matches the label?**

**A.** Start 50 mg and raise by 50 mg every 3 days to 300 mg

**B.** Start 25 mg daily, raise by 25 to 50 mg a day no faster than weekly, maximum 200 mg

**C.** Start 100 mg and set a maximum of 400 mg

**D.** Start 25 mg and raise daily to a maximum of 100 mg

**E.** Start 10 mg and keep a maximum of 50 mg

Answer Contents

**BQ-258**

**A 28-year-old woman with PTSD takes paroxetine and plans a pregnancy. Which counseling statement best matches the label?**

**A.** Paroxetine is contraindicated in all women who may conceive

**B.** Paroxetine carries no warning about pregnancy exposure

**C.** First-trimester exposure is linked to over a 5-fold rise in malformations

**D.** The cardiac signal applies only to third-trimester exposure

**E.** First-trimester exposure is linked to under a 2-fold rise in cardiac malformations

Answer Contents

**BQ-259**

**A 46-year-old woman taking tamoxifen develops PTSD after her cancer diagnosis and requests an antidepressant. What is the best approach?**

**A.** Choose an antidepressant with little CYP2D6 inhibition

**B.** Choose paroxetine because it is labeled for PTSD

**C.** Use paroxetine at a reduced dose

**D.** Add prazosin to prevent interaction

**E.** Avoid every antidepressant

Answer Contents

**BQ-260**

**A 55-year-old man with controlled hypertension prefers medication for PTSD. Which point about venlafaxine matters most?**

**A.** It is FDA labeled for PTSD

**B.** It lowers blood pressure

**C.** It requires prazosin at the same time

**D.** It can raise blood pressure, so monitor it

**E.** It is contraindicated with every antihypertensive

Answer Contents

**BQ-261**

**A 78-year-old woman with PTSD starts sertraline and 2 weeks later is confused and has fallen. What is the best next step?**

**A.** Increase the dose

**B.** Add a benzodiazepine

**C.** Check the serum sodium

**D.** Start an antipsychotic

**E.** Reassure and wait

Answer Contents

**BQ-262**

**A 19-year-old begins sertraline for PTSD. Which statement about the boxed warning is accurate?**

**A.** Risk is greatest after age 65

**B.** Ages 18 to 24 show 5 more cases per 1,000 than placebo

**C.** The warning applies only below age 12

**D.** Ages 25 to 64 show 14 more cases per 1,000

**E.** The warning has been removed from the label

Answer Contents

**BQ-263**

**A 44-year-old man with PTSD nightmares will start prazosin. He takes tadalafil and hydrochlorothiazide. Which counseling is best?**

**A.** Take the first dose in the morning

**B.** Start 5 mg to ensure benefit

**C.** Stop tadalafil permanently

**D.** No interaction is expected

**E.** Start 1 mg at bedtime and warn about fainting and low blood pressure

Answer Contents

**BQ-264**

**A patient asks for MDMA-assisted therapy for PTSD, believing it is FDA approved. What is the most accurate reply?**

**A.** It is not approved; FDA issued a complete response letter in 2024

**B.** It was approved in 2024 and VA covers it

**C.** VA/DoD strongly recommends it for PTSD

**D.** It is approved for veterans only

**E.** It is approved but limited to inpatient use

Answer Contents

**BQ-265**

**A woman with a history of sexual assault becomes distressed during a routine inpatient examination. Which action best reflects trauma-informed care?**

**A.** Proceed quickly to finish the examination

**B.** Require her to describe the assault first

**C.** Have staff hold her still

**D.** Explain each step and let her control the pace

**E.** Postpone all care indefinitely

Answer Contents

**BQ-266**

**A 47-year-old man with PTSD and alcohol use disorder is told he must be abstinent for 6 months before trauma-focused therapy. What is the best response?**

**A.** Agree, since substance use makes therapy unsafe

**B.** Offer supportive therapy only

**C.** Offer trauma-focused therapy alongside treatment for alcohol use

**D.** Prescribe a benzodiazepine for sleep

**E.** Refer him for MDMA-assisted therapy

Answer Contents

**BQ-267**

**A 9-year-old girl, 3 months after sexual abuse by a relative, has nightmares and avoids school. Her mother is supportive. What is the best first-line treatment?**

**A.** Sertraline

**B.** Trauma-focused CBT with caregiver involvement

**C.** Nondirective play therapy

**D.** Prazosin

**E.** Observation until age 12

Answer Contents

**BQ-268**

**A 4-year-old adopted from an orphanage walks up to strangers, climbs into laps, and left a park with a stranger without looking back. Which diagnosis fits best?**

**A.** Autism spectrum disorder

**B.** Reactive attachment disorder

**C.** Attention-deficit/hyperactivity disorder

**D.** Selective mutism

**E.** Disinhibited social engagement disorder

Answer Contents

**BQ-269**

**A 26-year-old woman with childhood trauma has frantic efforts to avoid abandonment, rapid shifts in self-image, unstable relationships, and recurrent self-harm. Which diagnosis best explains the picture?**

**A.** Borderline personality disorder

**B.** Complex PTSD

**C.** Dissociative identity disorder

**D.** Bipolar II disorder

**E.** Adjustment disorder

Answer Contents

**BQ-270**

**A 35-year-old woman cannot recall the 2 days after an assault but remembers the rest of her life. Examination is normal and toxicology is negative. Which diagnosis fits best?**

**A.** Generalized dissociative amnesia

**B.** Transient global amnesia

**C.** Selective dissociative amnesia

**D.** Localized dissociative amnesia

**E.** Anterograde amnesia from head injury

Answer Contents

**BQ-271**

**A 19-year-old has felt for a year as if he were watching himself from outside his body and the world looks like a film. He knows this is not real. Toxicology is negative and there are no hallucinations. Which feature best separates his condition from psychosis?**

**A.** Onset in adolescence

**B.** Absence of anxiety

**C.** Intact reality testing

**D.** Presence of memory gaps

**E.** Response to antipsychotics

Answer Contents

**BQ-272**

**A patient receiving esketamine nasal spray for treatment-resistant depression feels detached from his body 30 minutes after dosing. Which statement is most accurate?**

**A.** Dissociation is a boxed warning, so observation for 2 hours is required

**B.** It signals a relapse of PTSD

**C.** It signals dissociative identity disorder

**D.** It signals an allergic reaction

**E.** No monitoring is needed after dosing

Answer Contents

**BQ-273**

**A 16-year-old whose mother died 8 months ago has persistent yearning, disbelief, avoidance of reminders, and a sense that life is meaningless, with impaired schooling. Which statement is accurate?**

**A.** The diagnosis cannot be made before 12 months

**B.** The time since death meets the adolescent minimum, so the diagnosis can be considered

**C.** The diagnosis must be major depression

**D.** An antidepressant is required first

**E.** Adjustment disorder must be diagnosed first

Answer Contents

SUBJECT 13

# Schizophrenia Spectrum and Other Psychotic Disorders

Questions BQ-274 to BQ-300

**BQ-274**

**A program director reviews the NIMH RAISE Early Treatment Program, a cluster-randomized trial of team-based first-episode care (NAVIGATE) against community care. Which finding is reported?**

**A.** Fewer hospitalizations with NAVIGATE

**B.** Longer retention in treatment and greater gains in quality of life with NAVIGATE

**C.** No difference in work or school involvement

**D.** Benefit only in participants with the longest untreated psychosis

**E.** Lower antipsychotic doses with NAVIGATE

Answer Contents

**BQ-275**

**A 52-year-old woman is certain her neighbor is transmitting signals through the wall. She has held the belief for 8 months, works full time, has no hallucinations or mood symptoms, and refuses medication. Which approach to the first visits is best?**

**A.** Present evidence that the signals do not exist

**B.** Admit her involuntarily

**C.** Agree that the signals are real to build trust

**D.** Build an alliance, explore her distress and sleep, and treat what she will accept

**E.** End care until she accepts an antipsychotic

Answer Contents

**BQ-276**

**A 28-year-old man develops delusions and disorganized speech after his father's death. The symptoms last 3 weeks and resolve fully, and he returns to work. Substance and medical evaluations are negative. Which diagnosis fits best?**

**A.** Brief psychotic disorder
**B.** Schizophreniform disorder
**C.** Schizophrenia
**D.** Delusional disorder
**E.** Schizoaffective disorder

Answer Contents

**BQ-277**

**Which finding best separates schizoaffective disorder from major depressive disorder with psychotic features?**

**A.** Delusions that are mood-congruent
**B.** Depressed mood lasting longer than 2 weeks
**C.** Hallucinations in the auditory modality
**D.** A family history of psychosis
**E.** At least 2 weeks of delusions or hallucinations occurring without a mood episode

Answer Contents

**BQ-278**

**In a meta-analysis of progression to schizophrenia after a first substance-induced psychosis, which substance carried the highest pooled transition rate?**

**A.** Alcohol
**B.** Sedatives
**C.** Cannabis
**D.** Opioids
**E.** Amphetamines

Answer Contents

**BQ-279**

**A 27-year-old woman, 3 days after her aripiprazole dose was increased, says she cannot sit still and paces the hallway. Her hallucinations have not changed. What is the best next step?**

**A.** Examine for akathisia and consider lowering the dose

**B.** Raise the dose for worsening psychosis

**C.** Start an anticholinergic and raise the dose

**D.** Diagnose catatonia

**E.** Stop all treatment permanently

Answer Contents

**BQ-280**

**A 35-year-old man stable on risperidone 6 mg daily needs paroxetine for depression. According to the risperidone label, what is the maximum daily dose of risperidone for an adult taking paroxetine?**

**A.** 4 mg

**B.** 6 mg

**C.** 8 mg

**D.** 12 mg

**E.** 16 mg

Answer Contents

**BQ-281**

**A 30-year-old woman is about to start olanzapine. Which metabolic monitoring plan matches the VA/DoD guideline for glucose or HbA1c and lipids?**

**A.** Baseline only

**B.** Baseline and every 5 years

**C.** Annually, beginning after the first year

**D.** At 1 month and then every 6 months

**E.** Baseline, at 3 months for olanzapine, and at least yearly

Answer Contents

**BQ-282**

**According to the clozapine label, what total daily dose is the titration target by the end of 2 weeks for a patient who tolerates the drug?**

**A.** 25 to 50 mg

**B.** 300 to 450 mg

**C.** 500 to 600 mg

**D.** 700 to 800 mg

**E.** 900 to 1,000 mg

Answer Contents

**BQ-283**

**A man with benign ethnic neutropenia (Duffy-null associated neutrophil count) has two baseline ANC values of 1,100 and 1,050 per microliter. What does the clozapine label say about starting?**

**A.** Do not start below 500

**B.** Do not start below 750

**C.** Do not start below 900

**D.** Do not start below 1000

**E.** Do not start below 1500

Answer Contents

**BQ-284**

**A patient without benign ethnic neutropenia has an ANC of 800 per microliter during month 2 of clozapine, with no fever. Which action matches the label?**

**A.** Continue and recheck in a month

**B.** Continue with weekly testing

**C.** Interrupt clozapine, obtain a hematology consultation, and test daily

**D.** Increase the clozapine dose

**E.** Discontinue permanently and never rechallenge

Answer Contents

**BQ-285**

**A resident says that because the clozapine REMS ended in 2025, neutrophil monitoring is optional. Which statement is accurate?**

**A.** Prescribers should still monitor ANC at the frequencies in the prescribing information, although REMS enrollment and reporting ended

**B.** Monitoring is optional after the first month

**C.** The boxed warning was removed with the REMS

**D.** Pharmacists must still verify the ANC at every fill

**E.** The label now requires monitoring only every 6 months

Answer Contents

**BQ-286**

**In week 3 of clozapine titration, a patient has fever, chest pain, and tachycardia. Which is the best response?**

**A.** Treat as a viral illness

**B.** Slow the titration and continue

**C.** Add an anticholinergic

**D.** Stop clozapine and obtain urgent cardiac evaluation, including troponin

**E.** Switch to a depot antipsychotic the same day

Answer Contents

**BQ-287**

**A patient stable on clozapine is prescribed fluvoxamine for obsessive symptoms. What does the label advise about the clozapine dose?**

**A.** No change

**B.** Reduce to about one-third of the dose

**C.** Double the dose

**D.** Reduce by 10%

**E.** Stop clozapine

Answer Contents

**BQ-288**

**A psychiatrist considers carbamazepine for irritability in a patient taking clozapine. Which statement is most accurate?**

**A.** Carbamazepine raises clozapine levels

**B.** The combination is required for resistant illness

**C.** Carbamazepine has no effect on clozapine

**D.** The combination lowers the ANC threshold

**E.** The clozapine label advises against use with strong CYP3A4 inducers such as carbamazepine

Answer Contents

**BQ-289**

**A man on clozapine for 5 weeks reports "my bowels are fine" but has abdominal distension and vomiting. Which is the best interpretation?**

**A.** Anxiety

**B.** Normal anticholinergic effect

**C.** Possible hypomotility with ileus; the label notes that subjective constipation can understate the problem, so assess urgently

**D.** Clozapine withdrawal

**E.** Tardive dyskinesia of the gut

Answer Contents

**BQ-290**

**A 40-year-old stable on oral haloperidol 8 mg daily is to start haloperidol decanoate. The calculated first dose, at 15 times the oral dose, is 120 mg. What does the label advise?**

**A.** Give 120 mg intravenously

**B.** Give 120 mg intramuscularly in one injection

**C.** Give 50 mg every 2 weeks

**D.** Give 25 mg monthly

**E.** Give no more than 100 mg initially and the remainder 3 to 7 days later

Answer Contents

**BQ-291**

**A patient receives the first injection of risperidone long-acting microspheres. For how long does the label advise continuing oral risperidone or another antipsychotic?**

**A.** 3 weeks
**B.** 1 day
**C.** 1 week
**D.** 8 weeks
**E.** Oral cover is not needed

Answer Contents

**BQ-292**

**Which condition is a contraindication to xanomeline-trospium?**

**A.** Controlled hypertension
**B.** Type 2 diabetes
**C.** Well-controlled epilepsy
**D.** Urinary retention
**E.** Mild asthma

Answer Contents

**BQ-293**

**A patient with tardive dyskinesia takes paroxetine, a strong CYP2D6 inhibitor, and is started on deutetrabenazine. What is the maximum total daily dose per the label?**

**A.** 12 mg
**B.** 36 mg
**C.** 48 mg
**D.** 60 mg
**E.** 72 mg

Answer Contents

**BQ-294**

**A 19-year-old man given haloperidol 2 days ago develops a sustained twisting spasm of the neck and eyes. What is the best treatment?**

**A.** Increase haloperidol
**B.** Start lithium
**C.** Give an anticholinergic
**D.** Give an antibiotic
**E.** Observe without treatment

Answer Contents

**BQ-295**

**A clinician screens for catatonia with the first 14 items of the Bush-Francis scale. What result is positive?**

**A.** Two or more items
**B.** Five or more items
**C.** Eight or more items
**D.** All 14 items
**E.** Any score with fever

Answer Contents

**BQ-296**

**A catatonic patient receives a lorazepam challenge. Which description matches the 2014 clinical review?**

**A.** 10 mg orally with reassessment at 24 hours
**B.** 0.5 mg orally with reassessment at 1 week
**C.** 5 mg intramuscularly and discharge
**D.** 1 mg subcutaneously with reassessment at 2 hours
**E.** 1 or 2 mg intravenously with reexamination after 5 minutes

Answer Contents

**BQ-297**

**A patient with catatonia develops fever, tachycardia, and fluctuating blood pressure. Which is the best management?**

**A.** Increase antipsychotic dosing

**B.** Urgent medical care and ECT

**C.** Outpatient lorazepam

**D.** Watchful waiting

**E.** Discharge with follow-up

Answer Contents

**BQ-298**

**A 78-year-old with dementia and agitation is offered an antipsychotic. Which labeled warning matters most to the discussion?**

**A.** Increased risk of mania

**B.** Hypoglycemia

**C.** Risk of addiction

**D.** Increased mortality in elderly patients with dementia-related psychosis

**E.** Loss of taste

Answer Contents

**BQ-299**

**A woman took olanzapine through her third trimester. What do labels advise the pediatric team to watch for after delivery?**

**A.** Extrapyramidal and withdrawal symptoms in the neonate

**B.** Neural tube defects

**C.** Neonatal diabetes insipidus

**D.** Cleft palate

**E.** No monitoring is needed

Answer Contents

**BQ-300**

**A 33-year-old man with schizophrenia has had five admissions in 2 years, disengages from clinic visits, and is homeless. Which intervention has a 1B APA recommendation for exactly this pattern?**

**A.** Cognitive remediation alone

**B.** Assertive community treatment

**C.** Yoga

**D.** Weekly office psychotherapy alone

**E.** Acupuncture

Answer Contents

SUBJECT 14

# Substance-Related and Addictive Disorders

Questions BQ-301 to BQ-327

**BQ-301**

**A 55-year-old man drank heavily for decades and stopped 3 days ago. He is now disoriented, sees insects on the wall, and has a heart rate of 126 and a temperature of 38.4 C. Which is the best setting and treatment?**

**A.** Intensive care with IV access and benzodiazepines titrated to light somnolence

**B.** Home care with propranolol

**C.** Observation only until symptoms clear

**D.** A regular ward with haloperidol alone

**E.** Outpatient care with oral gabapentin

Answer Contents

**BQ-302**

**A 62-year-old malnourished man in alcohol withdrawal has confusion, an unsteady gait, and abnormal eye movements. What is the most important immediate step?**

**A.** Oral thiamine in 3 days

**B.** Glucose infusion only

**C.** Discharge with follow-up

**D.** Parenteral thiamine without waiting for test results

**E.** Haloperidol

Answer Contents

**BQ-303**

**A 30-year-old man with mild alcohol withdrawal (CIWA-Ar score 6), no prior seizures, no intoxication, and a reliable partner asks to be treated at home. Which is most consistent with the ASAM guideline?**

**A.** Hospital admission for all patients with any withdrawal

**B.** Ambulatory management with supportive care or medication and daily follow-up

**C.** Mandatory ICU admission

**D.** Phenobarbital infusion

**E.** No follow-up is needed

Answer Contents

**BQ-304**

**A 45-year-old man with moderate alcohol use disorder takes oxycodone daily for chronic pain. He asks for a medicine to reduce drinking. Which is contraindicated?**

**A.** Acamprosate

**B.** Gabapentin

**C.** Topiramate

**D.** Brief counseling

**E.** Oral or extended-release naltrexone

Answer Contents

**BQ-305**

**A 50-year-old man with alcohol use disorder has cirrhosis with elevated liver enzymes and normal kidney function. He takes no opioids. Which labeled option avoids hepatic concerns most directly?**

**A.** Extended-release naltrexone without liver tests

**B.** Disulfiram

**C.** Acamprosate

**D.** Oral naltrexone at any dose without monitoring

**E.** Metronidazole

Answer Contents

**BQ-306**

**A clinician wants to start disulfiram for a patient who drank 6 hours ago. What does the label say?**

**A.** Start at 1000 mg

**B.** Do not start until the patient has abstained from alcohol for at least 12 hours

**C.** Start only if the patient is unaware

**D.** Start immediately to prevent relapse

**E.** Wait 3 days

Answer Contents

**BQ-307**

**A 40-year-old woman receives extended-release naltrexone for alcohol use disorder. Two weeks later she has a hard, painful, necrotic area at the gluteal injection site. What is the best interpretation?**

**A.** An expected finding that needs no evaluation

**B.** A disulfiram reaction

**C.** A sign of acamprosate toxicity

**D.** Precipitated opioid withdrawal

**E.** A serious injection-site reaction described in the label, needing examination and possible surgical care

Answer Contents

**BQ-308**

**A 33-year-old man with opioid use disorder is being considered for extended-release naltrexone. He last used short-acting opioids 3 days ago. Which statement is correct?**

**A.** He should receive naloxone daily

**B.** He needs 28 days opioid-free

**C.** He should wait at least 7 to 10 days opioid-free, because earlier injection risks precipitated withdrawal

**D.** He can be injected today with no risk

**E.** He should first take a benzodiazepine

Answer Contents

**BQ-309**

**A 36-year-old man on buprenorphine/naloxone wants to switch to extended-release naltrexone. According to ASAM, how long should he typically wait after the last buprenorphine dose?**

**A.** 7 to 14 days

**B.** 28 days

**C.** 3 days

**D.** 1 hour

**E.** 24 hours

Answer Contents

**BQ-310**

**A 42-year-old man stable on methadone and an alprazolam prescription presents to a new office-based clinic requesting buprenorphine instead. Which statement best reflects ASAM guidance?**

**A.** He must stay on methadone forever

**B.** Buprenorphine can be started the same hour as the last methadone dose

**C.** The benzodiazepine should be stopped abruptly first

**D.** Benzodiazepine use should not by itself prevent treatment but calls for a risk-benefit assessment and careful management

**E.** Buprenorphine is contraindicated in anyone taking a benzodiazepine

Answer Contents

**BQ-311**

**A 28-year-old man on methadone 90 mg wants a buprenorphine-based injection. Which prerequisite appears in the monthly extended-release injection label?**

**A.** Opioid-free for 28 days

**B.** Three weeks of naltrexone

**C.** A single 4 mg transmucosal dose with observation, or established treatment with transmucosal buprenorphine

**D.** A negative urine screen for all drugs

**E.** A prescription for home self-injection

Answer Contents

**BQ-312**

**A 46-year-old man was released from jail after 3 months of abstinence from opioids. He used his previous dose of heroin and was found unresponsive. Which concept best explains this?**

**A.** Loss of opioid tolerance
**B.** Precipitated withdrawal
**C.** Naltrexone blockade
**D.** Opioid-induced hyperalgesia
**E.** Serotonin syndrome

Answer Contents

**BQ-313**

**A 24-year-old woman received naloxone for fentanyl overdose in the field and now is shivering, vomiting, and wants to leave. What is the best emergency department action?**

**A.** Admit involuntarily for detoxification only
**B.** Discharge without a plan
**C.** Prescribe a benzodiazepine
**D.** Offer buprenorphine, provide take-home naloxone, and arrange follow-up
**E.** Give naltrexone immediately

Answer Contents

**BQ-314**

**Which statement about methadone for opioid use disorder is correct?**

**A.** It cannot be used in pregnancy
**B.** It must be dispensed by a certified opioid treatment program, except in limited circumstances
**C.** It has no QT effect
**D.** The first dose should be 60 mg
**E.** It can be prescribed in any pharmacy for this indication

Answer Contents

**BQ-315**

**Under the 2024 revision of 42 CFR Part 8, which statement is correct?**

**A.** A one-year history of opioid use is required

**B.** Telehealth evaluation is prohibited

**C.** No take-home doses are ever allowed

**D.** Take-home doses are always 28 days from day 1

**E.** Take-home supplies may be up to 28 days from day 31 at the practitioner's discretion

Answer Contents

**BQ-316**

**A 35-year-old man tells you he has been on buprenorphine for 18 months and wants to stop. Which is most consistent with ASAM guidance?**

**A.** Stop abruptly

**B.** Add naltrexone on the same day

**C.** Medication must always be indefinite regardless of preference

**D.** Slow tapering over months with close monitoring and counseling about overdose risk

**E.** Taper over 2 days

Answer Contents

**BQ-317**

**A 39-year-old man with a COWS of 28 asks what the score means. How is it classified?**

**A.** Mild

**B.** Moderately severe withdrawal

**C.** Severe

**D.** No withdrawal

**E.** Moderate

Answer Contents

**BQ-318**

**A 52-year-old woman with opioid withdrawal cannot take buprenorphine or methadone and has a history of hypotension. Which statement about lofexidine is correct?**

**A.** It has no cardiovascular effects

**B.** It is an opioid agonist

**C.** It treats opioid use disorder long-term

**D.** It is a benzodiazepine

**E.** It mitigates withdrawal symptoms but is not a treatment for opioid use disorder, and it can cause hypotension and bradycardia

Answer Contents

**BQ-319**

**A 31-year-old man with cocaine use disorder has chest pain and agitation after use. Which is the best first approach?**

**A.** Attribute the pain to withdrawal

**B.** Start naltrexone

**C.** Evaluate for cardiac causes and treat agitation with a calm setting and a benzodiazepine

**D.** Give a beta-blocker alone

**E.** Assume anxiety and discharge

Answer Contents

**BQ-320**

**Which finding best distinguishes the effect of contingency management in stimulant use disorder in the ASAM/AAAP evidence tables?**

**A.** Strong benefit for abstinence during 12 weeks that was no longer present against CBT at last follow-up

**B.** Benefit only after one year

**C.** Superior to all medications proven

**D.** Harmful in adolescents

**E.** No benefit over placebo

Answer Contents

**BQ-321**

**A 19-year-old man with daily cannabis use for four years has repeated episodes of nausea, vomiting, and abdominal pain relieved by hot showers. What is the most effective definitive treatment?**

**A.** Proton pump inhibitor
**B.** Antipsychotic
**C.** Ondansetron indefinitely
**D.** Opioid analgesia
**E.** Complete cannabis cessation

Answer Contents

**BQ-322**

**Which statement about medications for cannabis use disorder is most accurate?**

**A.** Brief interventions of 60 minutes or less are recommended as treatment
**B.** Naltrexone is FDA-approved
**C.** No medication is FDA-approved, and CBT or motivational enhancement therapy are initial treatments
**D.** Varenicline is first-line
**E.** Benzodiazepines are first-line

Answer Contents

**BQ-323**

**A 23-year-old man brought in after using phencyclidine is agitated and hypertensive. Which complication is most characteristic of this drug?**

**A.** Psychosis that can last days to weeks with dangerous behavior
**B.** Opioid-like pinpoint pupils and respiratory depression
**C.** Cannabinoid hyperemesis
**D.** Seizure from sudden alcohol cessation
**E.** Delayed myelopathy

Answer Contents

**BQ-324**

**A 15-year-old boy is found unresponsive after inhaling a spray. What is the most likely mechanism of sudden death?**

**A.** Hepatic failure
**B.** Alcohol withdrawal
**C.** Opioid receptor agonism
**D.** Cardiac arrhythmia, which can occur even after one use
**E.** Serotonin toxicity

Answer Contents

**BQ-325**

**A 49-year-old man with schizophrenia who smokes tobacco is started on varenicline. Which statement best reflects the label?**

**A.** It is contraindicated in psychiatric illness
**B.** No boxed warning is present, and postmarketing neuropsychiatric events warrant monitoring of mood
**C.** There is a boxed warning for suicidality
**D.** It requires a dose of 2 mg twice daily
**E.** It is only for pregnant women

Answer Contents

**BQ-326**

**A 57-year-old man smokes 25 cigarettes a day and wants to use a nicotine patch. Which regimen matches the Drug Facts schedule?**

**A.** 21 mg daily for 6 weeks, 14 mg for 2 weeks, then 7 mg for 2 weeks
**B.** 7 mg for 6 weeks, then 21 mg
**C.** 42 mg for 2 weeks
**D.** 21 mg for 6 months
**E.** 14 mg for 10 weeks only

Answer Contents

**BQ-327**

**A woman with gambling disorder reports hopelessness after losing her savings. What is the most important additional assessment?**

**A.** Begin an FDA-approved gambling medication

**B.** Order a urine toxicology panel only

**C.** Recommend a casino-based exposure plan

**D.** Direct inquiry about suicidal thoughts

**E.** Assume the problem is only financial

Answer Contents

SUBJECT 15

# Personality, Impulse-Control, and Conduct Disorders

Questions BQ-328 to BQ-349

**BQ-328**

**A 22-year-old woman reports intense fear of being left, relationships that swing from idealizing to despising partners, mood shifts lasting hours after rejection, chronic emptiness, and repeated self-cutting. She has never had a period of reduced need for sleep. Which diagnosis fits best?**

**A.** Bipolar II disorder

**B.** Histrionic personality disorder

**C.** Borderline personality disorder

**D.** Dependent personality disorder

**E.** Narcissistic personality disorder

Answer Contents

**BQ-329**

**A 26-year-old man carries a diagnosis of borderline personality disorder. He now describes four-day periods of elevated mood, markedly reduced need for sleep, and racing thoughts, unrelated to relationship events. What is the best next step?**

**A.** Evaluate for a bipolar spectrum disorder before attributing these episodes to the personality disorder

**B.** Attribute the episodes to interpersonal stress and continue current care unchanged

**C.** Add a second antipsychotic for emotional instability

**D.** Stop all psychotherapy until mood stabilizes

**E.** Reassure him that mood shifts are expected in borderline personality disorder

Answer Contents

**BQ-330**

**A community service offers an eight-week, general supportive group for adults with borderline personality disorder, with no supervision structure or integrated model. Which statement is most consistent with NICE CG78?**

**A.** Brief groups are preferred for all patients

**B.** Any therapy is acceptable if it is short

**C.** Group formats are discouraged in all circumstances

**D.** Therapy should be delivered only by physicians

**E.** Interventions under 3 months should not be offered unless the service has an integrated approach, structured care, and supervision

Answer Contents

**BQ-331**

**A 30-year-old woman with borderline personality disorder is in acute crisis and the team considers a sedating medication. Which plan is most consistent with NICE CG78?**

**A.** Several sedatives together to ensure effect

**B.** One drug with low overdose risk at the minimum effective dose, with a plan to stop it, usually within 1 week

**C.** A long-term antipsychotic for self-harm

**D.** An open-ended benzodiazepine prescription

**E.** A sedating antihistamine without discussion because it is unlicensed

Answer Contents

**BQ-332**

**A clinician describes a patient using ICD-11 and mentions a "borderline pattern." How does ICD-11 treat this?**

**A.** As one of ten separate personality disorder categories

**B.** As a childhood-onset conduct specifier

**C.** As a psychotic disorder specifier

**D.** As an optional qualifier attached to a personality disorder severity diagnosis

**E.** As a diagnosis that excludes trait domains

Answer Contents

**BQ-333**

**A 30-year-old man prefers solitude, shows little interest in relationships, has a flat affect, and has had restricted interests and difficulty reading social cues since early childhood. Which diagnosis should be considered first?**

**A.** Schizophrenia
**B.** Autism spectrum disorder
**C.** Schizotypal personality disorder
**D.** Avoidant personality disorder
**E.** Major depressive disorder

Answer Contents

**BQ-334**

**A 25-year-old man has had repeated explosive outbursts of rage in response to minor provocation, including smashing objects, followed by remorse. The outbursts are not planned, and he has no mood episodes or intoxication. Which diagnosis fits best?**

**A.** Antisocial personality disorder
**B.** Conduct disorder
**C.** Bipolar I disorder
**D.** Intermittent explosive disorder
**E.** Oppositional defiant disorder

Answer Contents

**BQ-335**

**A 40-year-old woman repeatedly steals small items she neither needs nor can use, feels mounting tension beforehand and relief afterward, and discards or returns the items. Which diagnosis fits best?**

**A.** Kleptomania
**B.** Antisocial personality disorder
**C.** Obsessive-compulsive disorder
**D.** Shoplifting for gain
**E.** Bipolar I disorder, manic episode

Answer Contents

**BQ-336**

**A 15-year-old boy has set several fires to hurt peers he is angry with. He also skips school, steals, and has been cruel to animals. Which diagnosis best explains the fire setting?**

**A.** Pyromania

**B.** Oppositional defiant disorder

**C.** Intermittent explosive disorder

**D.** Adjustment disorder

**E.** Conduct disorder

Answer Contents

**BQ-337**

**During therapy with a patient who has borderline personality disorder, a clinician finds herself extending sessions and offering her personal number after the patient calls her uniquely understanding. Which response is best?**

**A.** Continue, because it strengthens the alliance

**B.** Transfer the patient to avoid dependence

**C.** Discuss the pull in supervision and restate the treatment frame warmly

**D.** Confront the patient about manipulating her

**E.** Reduce all contact to prevent splitting

Answer Contents

**BQ-338**

**A 35-year-old woman with bipolar I disorder started lamotrigine three weeks ago and develops a spreading rash with mouth sores and fever. What is the best action?**

**A.** Continue titration and add an antihistamine

**B.** Reduce the dose by half and monitor

**C.** Switch to a second antiepileptic that day without evaluation

**D.** Wait one week to see if the rash resolves

**E.** Stop lamotrigine and arrange urgent medical evaluation

Answer Contents

**BQ-339**

**A 33-year-old man with antisocial personality disorder has persistent low mood, anhedonia, and poor sleep for two months. His clinician considers the antisocial diagnosis a reason not to treat. What is the best response?**

**A.** Defer treatment until his behavior improves

**B.** Refuse care because of the diagnosis

**C.** Assess and treat the depression with standard evidence-based care

**D.** Treat only with a mood stabilizer

**E.** Refer to law enforcement only

Answer Contents

**BQ-340**

**A woman with borderline personality disorder has had two psychiatric admissions in the past 6 months. Which NICE CG78 step applies?**

**A.** Discharge from all services

**B.** A formal Care Programme Approach review

**C.** Compulsory long-term residential care

**D.** Stopping psychological therapy

**E.** Starting an antipsychotic

Answer Contents

**BQ-341**

**A 28-year-old woman recently diagnosed with borderline personality disorder asks about her future. Which statement best reflects long-term cohort data?**

**A.** Most patients reach remission over years, but recovery of function is slower and relapse can occur

**B.** The diagnosis is lifelong and fixed

**C.** Remission is rare

**D.** Recovery is as fast as in other personality disorders

**E.** Relapse never occurs once remission is reached

Answer Contents

**BQ-342**

**A 29-year-old man wants close friendships but avoids parties and work meetings for fear of criticism and feels inferior to others. This has been present since adolescence across settings. Which treatment approach is best?**

**A.** Long-term antipsychotic

**B.** Benzodiazepine as needed

**C.** Hospital admission

**D.** Graded exposure with cognitive therapy and skills training

**E.** Avoidance of all social demands

Answer Contents

**BQ-343**

**A 13-year-old girl has conduct problems at home and school. According to the CDC, which approach is often used and effective for school-age children and teens?**

**A.** Training and therapy that involves the child, the family, and the school

**B.** Medication alone

**C.** Detention alone

**D.** Individual play therapy alone

**E.** Watchful waiting only

Answer Contents

**BQ-344**

**A systematic review pooled randomized trials of psychotherapy versus treatment as usual for borderline personality disorder. Which outcome met the review's threshold for clinically meaningful improvement?**

**A.** Depression

**B.** Self-harm

**C.** Psychosocial functioning

**D.** Suicide-related outcomes

**E.** Borderline symptom severity

Answer Contents

**BQ-345**

**NICE NG225 addresses a 34-year-old woman with a history of self-harm. Which intervention is advised for adults who self-harm?**

**A.** A drug specifically to reduce self-harm

**B.** A risk scale to predict repetition

**C.** Punitive measures to deter self-harm

**D.** A structured, person-centered, CBT-informed intervention, typically 4 to 10 sessions

**E.** No psychological care until substance use stops

Answer Contents

**BQ-346**

**A resident states that the 2024 VA/DoD suicide guideline recommends against dialectical behavior therapy because it graded it "neither for nor against." Which reply is most accurate?**

**A.** The grade is a strong recommendation against DBT

**B.** The guideline recommended DBT strongly

**C.** The grade reflects insufficient trial evidence for suicide attempts, not a recommendation against DBT

**D.** The guideline does not mention DBT

**E.** The grade shows DBT is harmful

Answer Contents

**BQ-347**

**A 38-year-old woman is dramatically emotional, seeks attention, is easily influenced by others, and has shifting, shallow emotion. She has no self-injury, no abandonment panic, and no somatic complaints. Which diagnosis fits best?**

**A.** Borderline personality disorder

**B.** Histrionic personality disorder

**C.** Somatic symptom disorder

**D.** Bipolar II disorder

**E.** Dependent personality disorder

Answer Contents

**BQ-348**

**A 45-year-old man with grandiose self-presentation loses his job and becomes ashamed, enraged, and tells you life is not worth living. What is the first priority?**

**A.** Interpret his grandiosity

**B.** Confront his entitlement

**C.** Refer for vocational counseling

**D.** Assess suicide risk directly

**E.** Document narcissistic personality disorder and end the visit

Answer Contents

**BQ-349**

**A nurse calls a patient with borderline personality disorder "manipulative." Which reframing is most useful?**

**A.** She is dangerous and should be avoided

**B.** Her indirect requests express unmet needs and skills deficits that treatment can address

**C.** She should be refused further contact

**D.** Her behavior means the diagnosis is wrong

**E.** Her behavior is always intentional deception

Answer Contents

SUBJECT 16

# Neurocognitive Disorders and Delirium

Questions BQ-350 to BQ-374

**BQ-350**

**An 84-year-old man two days after pneumonia is quiet, sleeping most of the day, and eats little. The team calls him "depressed." He cannot name the months backward, and his wife says he was sharp last week. What is the most likely diagnosis?**

**A.** Hypoactive delirium

**B.** Major depressive disorder

**C.** Alzheimer disease progression

**D.** Normal aging

**E.** Catatonia due to schizophrenia

Answer Contents

**BQ-351**

**A resident applies the Confusion Assessment Method to a hospitalized woman. Acute onset with fluctuation is present, inattention is present, her thinking is disorganized, and she is alert. What is the result?**

**A.** Negative, because the level of consciousness must also be altered

**B.** Negative, because hallucinations are required

**C.** Positive, because the first two features plus disorganized thinking meet the rule

**D.** Indeterminate, because the patient is not agitated

**E.** Positive only if the patient is agitated

Answer Contents

**BQ-352**

**A 68-year-old man with Parkinson disease is delirious after a urinary infection and is shouting. Which measure should be avoided because it can severely worsen his motor function?**

**A.** Treating the infection

**B.** Reorientation by family

**C.** Hydration

**D.** Pain control

**E.** Haloperidol

Answer Contents

**BQ-353**

**In the 1999 multicomponent prevention trial of 852 hospitalized patients aged 70 or older, which set of risk factors was targeted?**

**A.** Anemia, hypotension, hyperglycemia, fever, hypoxia, anxiety

**B.** Cognitive impairment, sleep deprivation, immobility, visual impairment, hearing impairment, dehydration

**C.** Age, sex, race, education, income, marital status

**D.** Hypertension, diabetes, smoking, obesity, alcohol, inactivity

**E.** Pain, constipation, polypharmacy, falls, malnutrition, incontinence

Answer Contents

**BQ-354**

**A randomized trial in ICU patients with delirium compared haloperidol, ziprasidone, and placebo. What was the main result for days alive without delirium or coma?**

**A.** Haloperidol was best

**B.** Ziprasidone was best

**C.** Placebo was worst

**D.** No significant difference among the three groups

**E.** Both drugs doubled the days

Answer Contents

**BQ-355**

**A 66-year-old woman shows decline in executive function and learning, struggles with bills and medication management, and needs help daily. Which DSM-5-TR level applies?**

**A.** Delirium

**B.** Mild neurocognitive disorder

**C.** Major neurocognitive disorder

**D.** Subjective cognitive concern

**E.** Adjustment disorder

Answer Contents

**BQ-356**

**A 75-year-old woman with moderate Alzheimer dementia has taken donepezil 10 mg for four months and tolerates it. Per the label, when may the 23 mg dose be given?**

**A.** Only after at least 3 months on 10 mg daily, for moderate to severe disease

**B.** Immediately

**C.** After 1 week at 10 mg

**D.** Never, because 10 mg is the maximum

**E.** Only in mild disease

Answer Contents

**BQ-357**

**A patient stopped rivastigmine patches for five days because of travel. How should treatment resume per the label?**

**A.** At 13.3 mg/24 h

**B.** At the last used strength

**C.** At 9.5 mg/24 h

**D.** Two patches at once to catch up

**E.** At 4.6 mg/24 h with retitration

Answer Contents

**BQ-358**

**A 79-year-old man with Alzheimer disease has creatinine clearance of 40 mL/min. Which galantamine ceiling applies per the label?**

**A.** 24 mg/day

**B.** Generally not above 16 mg/day

**C.** 32 mg/day

**D.** 8 mg/day

**E.** Not recommended

Answer Contents

**BQ-359**

**A patient with moderate Alzheimer dementia has creatinine clearance of 20 mL/min. What memantine target dose does the label give for severe renal impairment?**

**A.** 20 mg twice daily

**B.** 20 mg once daily

**C.** 10 mg three times daily

**D.** 5 mg twice daily

**E.** 40 mg daily

Answer Contents

**BQ-360**

**A woman with Alzheimer dementia on donepezil has new falls and a pulse of 44. What is the best explanation and action?**

**A.** Normal aging; continue

**B.** Vagotonic effect causing bradycardia; check ECG and reduce or stop the drug

**C.** Memantine effect; double donepezil

**D.** Hypothyroidism only; start levothyroxine

**E.** Dehydration only; stop all fluids

Answer Contents

**BQ-361**

**A man begins donanemab. According to the label, before which infusions should follow-up MRI be obtained, after baseline?**

**A.** Before every infusion
**B.** Only at month 12
**C.** Before the 5th and 10th infusions
**D.** Only if symptoms occur
**E.** Before the 2nd, 3rd, 4th, and 7th infusions

Answer Contents

**BQ-362**

**A 73-year-old man on lecanemab develops sudden aphasia and right-sided weakness. What should the team consider before giving thrombolytic therapy?**

**A.** Normal aging
**B.** ARIA-E mimicking stroke
**C.** Medication nonadherence
**D.** Hyponatremia only
**E.** Drug-induced parkinsonism

Answer Contents

**BQ-363**

**Which statement about ApoE e4 homozygotes and anti-amyloid antibodies is correct?**

**A.** They are about 15 percent of Alzheimer patients and have the highest ARIA incidence
**B.** They are the majority of Alzheimer patients and have the lowest ARIA incidence
**C.** Testing is prohibited
**D.** They never develop symptomatic ARIA
**E.** They must be excluded from all treatment by law

Answer Contents

**BQ-364**

**The FDA-cleared plasma pTau217/amyloid beta 42 ratio test is intended for which use?**

**A.** Population screening of healthy adults

**B.** Stand-alone diagnosis in primary care

**C.** Predicting future dementia in cognitively normal people

**D.** Adults 55 and older with signs and symptoms of Alzheimer disease in specialized care, interpreted with other clinical information

**E.** Replacement for all PET imaging

Answer Contents

**BQ-365**

**Under the 2024 Alzheimer's Association revised criteria, which biomarker group is sufficient to diagnose Alzheimer disease?**

**A.** Core 1 (such as amyloid PET, CSF amyloid beta 42, or p-tau217)

**B.** Core 2 (tau PET) alone

**C.** Structural MRI atrophy alone

**D.** APOE genotype alone

**E.** EEG slowing

Answer Contents

**BQ-366**

**A 70-year-old has dementia with visual hallucinations and parkinsonism. DAT SPECT shows reduced uptake. Which classification applies under the 2017 consortium criteria?**

**A.** Possible Alzheimer disease

**B.** Frontotemporal dementia

**C.** Vascular dementia

**D.** Normal pressure hydrocephalus

**E.** Probable dementia with Lewy bodies

Answer Contents

**BQ-367**

**A 60-year-old man shows disinhibition, apathy, and perseverative behavior, with a dysexecutive neuropsychological profile. How many of the six clinical features define possible behavioral variant frontotemporal dementia?**

**A.** One
**B.** Two
**C.** Three
**D.** Five
**E.** All six

Answer Contents

**BQ-368**

**A 62-year-old develops dementia over eight weeks, with myoclonus and cerebellar ataxia. Which tests support probable Creutzfeldt-Jakob disease?**

**A.** Serum B12 only
**B.** MRI diffusion changes, periodic EEG complexes, or positive CSF RT-QuIC
**C.** Routine thyroid function
**D.** Plain skull radiograph
**E.** Urine toxicology

Answer Contents

**BQ-369**

**A 74-year-old man with Parkinson disease has distressing hallucinations and delusions. Which drug is labeled for this indication?**

**A.** Haloperidol
**B.** Risperidone
**C.** Olanzapine
**D.** Pimavanserin
**E.** Lorazepam

Answer Contents

**BQ-370**

**A 79-year-old woman with Alzheimer disease has sleep difficulty. According to NICE NG97, which statement is correct?**

**A.** Melatonin is first line

**B.** Valproate is recommended for agitation

**C.** Antidepressants are routine for mild depression

**D.** Benzodiazepines are first line

**E.** Use a personalized multicomponent approach, and do not use melatonin for insomnia in Alzheimer disease

Answer Contents

**BQ-371**

**A 77-year-old man with moderate to severe Alzheimer disease has taken donepezil for years, and the team considers stopping it because the disease is progressing. What does the randomized discontinuation trial suggest?**

**A.** Continuing donepezil gave better cognition and function than stopping it over 52 weeks

**B.** Stopping is always better

**C.** Memantine plus donepezil was clearly superior to donepezil alone

**D.** Neither drug helped

**E.** Donepezil should be doubled

Answer Contents

**BQ-372**

**A 52-year-old with Huntington disease starts deutetrabenazine, and 6 weeks later shows worsening mood. Which action is best?**

**A.** Increase the dose

**B.** Add a second VMAT2 inhibitor

**C.** Assess suicide risk, treat depression, and reduce or stop the drug if needed

**D.** Ignore it

**E.** Begin haloperidol

Answer Contents

**BQ-373**

**A 83-year-old man with mixed dementia takes oxybutynin and diphenhydramine nightly and is more confused. What is the best step?**

**A.** Add a cholinesterase inhibitor

**B.** Increase both drugs

**C.** Review and deprescribe the strongly anticholinergic drugs

**D.** Add a benzodiazepine

**E.** Start an antipsychotic

Answer Contents

**BQ-374**

**A family asks whether a feeding tube should be placed in their mother with severe dementia who has stopped eating. According to NICE NG97, what is the guidance?**

**A.** Place one routinely

**B.** Place one for all hospital admissions

**C.** Place one for every patient with weight loss

**D.** Do not use enteral feeding routinely in severe dementia unless treating a potentially reversible comorbidity

**E.** Never discuss it

Answer Contents

SUBJECT 17

# Neurologic Disorders in Psychiatric Practice

Questions BQ-375 to BQ-396

**BQ-375**

**An 11-year-old boy has motor and vocal tics for two years that now embarrass him at school and disrupt his concentration. He takes no medication, and a clinician trained in behavioral therapy for tics is available. Which is the best first treatment?**

**A.** Risperidone

**B.** Pimozide

**C.** Haloperidol

**D.** Comprehensive behavioral intervention for tics

**E.** Topiramate

Answer Contents

**BQ-376**

**A 38-year-old woman with temporal lobe epilepsy had a cluster of seizures two days ago. She now has delusions of being watched and is agitated, with no fever and a normal metabolic panel. Which is the best management?**

**A.** Treat the cluster and give a benzodiazepine or low-dose antipsychotic

**B.** Diagnose schizophrenia and start a long-acting injectable antipsychotic

**C.** Start bupropion for mood and psychosis

**D.** Stop her antiseizure drugs to unmask the focus

**E.** Arrange electroconvulsive therapy at once

Answer Contents

**BQ-377**

**A 34-year-old woman started topiramate for migraine prevention and now says she cannot find words and loses her train of thought at work. Which is the best explanation and response?**

**A.** A primary psychotic disorder that needs an antipsychotic

**B.** Expected anxiety that will resolve without change

**C.** A dose-related cognitive adverse reaction; lower the dose or switch

**D.** Depression from migraine that needs an immediate antidepressant

**E.** A sign of anti-NMDA receptor encephalitis

Answer Contents

**BQ-378**

**A 29-year-old man with bipolar I disorder takes valproate and will start lamotrigine for maintenance. Which plan matches the label?**

**A.** Start 25 mg daily and aim for 200 mg daily

**B.** Start 50 mg daily and aim for 400 mg daily

**C.** Start 100 mg daily and aim for 100 mg daily

**D.** Use the same schedule as for a patient on no interacting drugs

**E.** Start 25 mg every other day and aim for 100 mg daily

Answer Contents

**BQ-379**

**A 40-year-old man of Southeast Asian ancestry is about to start carbamazepine for epilepsy. Which pretreatment step is most important?**

**A.** Serum lithium level

**B.** HLA-B*1502 testing

**C.** Fasting lipid panel

**D.** Brain MRI with contrast

**E.** Serum prolactin

Answer Contents

**BQ-380**

**A 24-year-old woman with epilepsy and bipolar I disorder is not using contraception and may become pregnant. A clinician proposes valproate. What is the best advice?**

**A.** Avoid valproate unless other drugs have failed or are unacceptable

**B.** Use valproate; fetal risks match other antiseizure drugs

**C.** Use valproate with a low dose of vitamin A

**D.** Use valproate only after the first trimester

**E.** Stop all antiseizure medication before conception

Answer Contents

**BQ-381**

**A 30-year-old man with epilepsy and a history of angry outbursts is prescribed perampanel. Which counseling is most accurate?**

**A.** Aggression is unrelated to the dose

**B.** The risk is greatest after one year of treatment

**C.** Serious behavioral reactions are dose related and need monitoring during titration

**D.** Hormonal contraceptives become more effective

**E.** Only patients with prior psychiatric diagnoses are affected

Answer Contents

**BQ-382**

**A 66-year-old man three weeks after an ischemic stroke asks whether fluoxetine will speed his recovery. What is the best response?**

**A.** It reliably improves motor recovery

**B.** In a large trial it did not improve function and raised fractures

**C.** It prevents recurrent stroke

**D.** It should be started in every patient for at least 3 years

**E.** It is contraindicated in all stroke survivors

Answer Contents

**BQ-383**

**A 58-year-old man with ALS has sudden, uncontrollable crying that does not match his mood, and he reports a normal mood between episodes. Which treatment is labeled for this problem?**

**A.** Haloperidol

**B.** Lorazepam

**C.** Methylphenidate

**D.** Sertraline

**E.** Dextromethorphan with quinidine

Answer Contents

**BQ-384**

**A 31-year-old man is hospitalized with a moderate traumatic brain injury. Which factor most increases his risk of major depression in the following year?**

**A.** Age over 60

**B.** Absence of anxiety symptoms

**C.** Male sex

**D.** Depression before the injury

**E.** Abstinence from alcohol

Answer Contents

**BQ-385**

**A 17-year-old football player still has headaches, poor sleep, anxiety, and slowed thinking five weeks after a concussion. What is the best next step?**

**A.** Strict dark-room rest until symptoms resolve

**B.** Immediate return to contact practice

**C.** Start a stimulant and an opioid

**D.** Reassurance without evaluation

**E.** Multimodal evaluation of sleep, mood, and symptoms

Answer Contents

**BQ-386**

**A 34-year-old woman with multiple sclerosis who recently began interferon beta reports new low mood and hopelessness. Which statement is correct?**

**A.** Interferon beta cannot affect mood

**B.** Mood symptoms in multiple sclerosis never need treatment

**C.** Interferon beta can cause depression, so screen and treat

**D.** Her symptoms prove a relapse of multiple sclerosis

**E.** Antidepressants are contraindicated in multiple sclerosis

Answer Contents

**BQ-387**

**A 45-year-old man given a course of prednisone for an inflammatory flare develops insomnia, elation, and then suspiciousness within days. What is the best step?**

**A.** Diagnose bipolar I disorder and start lifelong lithium

**B.** Lower or stop the corticosteroid if the medical condition allows, and treat the symptoms

**C.** Add an antidepressant

**D.** Continue unchanged because psychiatric effects are never drug related

**E.** Start a stimulant

Answer Contents

**BQ-388**

**A 52-year-old woman has taken metoclopramide for diabetic gastroparesis for 14 months. She now has lip smacking and tongue movements. What is the best action?**

**A.** Double the dose to suppress the movements

**B.** Add haloperidol

**C.** Continue and recheck in a year

**D.** Add an anticholinergic and continue

**E.** Stop metoclopramide and assess for tardive dyskinesia

Answer Contents

**BQ-389**

**A 29-year-old man with schizophrenia has a generalized seizure during clozapine titration. Which statement about clozapine and seizures is correct?**

**A.** Seizures are independent of dose

**B.** Clozapine is contraindicated in anyone who has had a seizure

**C.** Seizures occur only above 900 mg daily

**D.** Seizure risk is dose related and is reduced by slow titration in divided doses

**E.** Seizures indicate agranulocytosis

Answer Contents

**BQ-390**

**A 60-year-old man taking lithium has diarrhea, begins an NSAID for pain, and then becomes confused and has a seizure. His lithium level is 3.4 mEq/L. What is the most likely explanation?**

**A.** Primary epilepsy unmasked by stress

**B.** Serotonin syndrome

**C.** Lithium toxicity, made worse by an interacting drug and volume loss

**D.** Neuroleptic malignant syndrome

**E.** Hypoglycemia

Answer Contents

**BQ-391**

**A 32-year-old woman who is using contraception has frequent migraine and moderate depression. Which preventive is supported for migraine and also treats depression?**

**A.** Amitriptyline

**B.** Lamotrigine

**C.** Clomipramine

**D.** Gabapentin

**E.** Fluoxetine

Answer Contents

**BQ-392**

**A 40-year-old man has unilateral leg weakness. When he flexes the opposite hip against resistance, the weak hip extends with normal force, while it is weak on direct testing. What does this finding support?**

**A.** A lacunar stroke

**B.** Functional leg weakness (a positive Hoover sign)

**C.** Multiple sclerosis

**D.** Malingering

**E.** A peripheral nerve injury

Answer Contents

**BQ-393**

**A 34-year-old woman with dissociative seizures asks what cognitive behavioral therapy will do. Which answer is best supported by the largest randomized trial?**

**A.** It will end her seizures within a month

**B.** It is harmful and should be avoided

**C.** It works only if she has no other psychiatric disorder

**D.** It may improve daily functioning even if seizure counts do not fall

**E.** It is the same as antiseizure medication

Answer Contents

**BQ-394**

**A 20-year-old woman diagnosed with anti-NMDA receptor encephalitis six months ago is improving slowly, and her family fears the recovery has stalled. Which statement is most accurate?**

**A.** Recovery is complete by 3 months

**B.** Relapse never occurs

**C.** Further improvement is not possible

**D.** Immunotherapy has no effect on outcome

**E.** Improvement can continue for up to 18 months

Answer Contents

**BQ-395**

**A 45-year-old woman has encephalopathy with seizures and hallucinations, thyroid antibodies, a normal MRI, and no well-characterized neuronal antibody. How do the Graus criteria classify this?**

**A.** Possible autoimmune encephalitis only

**B.** Definite anti-NMDA receptor encephalitis

**C.** Probable autoimmune encephalitis (Hashimoto encephalopathy)

**D.** Definite limbic encephalitis

**E.** Primary psychotic disorder

Answer Contents

**BQ-396**

**A 62-year-old man with tardive dyskinesia is a CYP2D6 poor metabolizer. Which valbenazine dosing is correct?**

**A.** Limit the dose to 40 mg once daily

**B.** Use 80 mg once daily from the start

**C.** Use 120 mg once daily

**D.** Avoid any dose adjustment

**E.** Double the dose weekly

Answer Contents

SUBJECT 18

# Somatic Symptom and Eating Disorders

Questions BQ-397 to BQ-418

**BQ-397**

**A 52-year-old man avoids doctors for 8 months because he is convinced he has a hidden heart condition after a friend died suddenly. He has only occasional, brief palpitations, and an ECG and echocardiogram were normal on his one prior visit. He spends hours checking his pulse and reading about heart disease. Which is the most likely diagnosis?**

**A.** Somatic symptom disorder

**B.** Illness anxiety disorder, care-avoidant type

**C.** Panic disorder

**D.** Delusional disorder, somatic type

**E.** Specific phobia

Answer Contents

**BQ-398**

**A primary care clinic uses the Patient Health Questionnaire-15 (PHQ-15) to track a 45-year-old woman with multiple symptoms. Her score is 12. Which interpretation is most accurate?**

**A.** Minimal somatic symptoms

**B.** Low somatic symptom severity

**C.** Diagnostic of somatic symptom disorder

**D.** High somatic symptom severity

**E.** Medium somatic symptom severity, which describes burden and does not make a diagnosis

Answer Contents

**BQ-399**

**A primary care physician asks a psychiatrist how to help a patient with many medically unexplained symptoms when the patient declines referral. Among approaches other than CBT, which has shown the most consistent benefit in randomized trials?**

**A.** A written consultation letter to the primary care physician with a management plan

**B.** Repeating the diagnostic workup every 6 months

**C.** A short course of benzodiazepines

**D.** Long-term opioid therapy

**E.** Discharge from the practice

Answer Contents

**BQ-400**

**A 33-year-old woman with dissociative seizures is offered CBT. She asks what the largest randomized trial found at 12 months. Which statement is most accurate?**

**A.** CBT eliminated seizures in most participants

**B.** CBT markedly reduced monthly seizure frequency

**C.** CBT had no benefit on any outcome

**D.** Seizure frequency did not differ, but psychosocial functioning was better with CBT

**E.** CBT increased serious adverse events

Answer Contents

**BQ-401**

**A 22-year-old man has a new, severe gait disturbance with variable leg weakness that improves when he is distracted. Neurologic examination shows positive signs of inconsistency, and MRI is normal. Which is the best next step?**

**A.** Start an SSRI for the gait disorder itself

**B.** Repeat MRI monthly

**C.** Explain that the diagnosis is real and based on positive signs, then refer for physiotherapy and CBT

**D.** Tell him the symptoms are imaginary

**E.** Start an antipsychotic

Answer Contents

**BQ-402**

**A nurse reports that a 3-year-old child's unexplained symptoms occur only when the mother is alone with the child, and laboratory results do not match the mother's reports. The treating team suspects factitious disorder imposed on another. Which is the best next step?**

**A.** Confront the mother alone and ask her to stop

**B.** Ensure the child's safety, document the facts, and notify child protective services as the law requires

**C.** Refer the mother to outpatient psychotherapy only

**D.** Order further invasive tests on the child before acting

**E.** Discharge the child to the mother with a follow-up visit

Answer Contents

**BQ-403**

**A 40-year-old man involved in a disability lawsuit reports severe memory loss and pain. Testing shows effort that varies with the examiner's attention, and findings contradict surveillance of his daily activity. Which is the best description?**

**A.** Factitious disorder imposed on self

**B.** Somatic symptom disorder

**C.** Functional neurological symptom disorder

**D.** Illness anxiety disorder

**E.** Malingering, which is not a mental disorder

Answer Contents

**BQ-404**

**A 26-year-old woman with anorexia nervosa is medically stable and asks which talk therapies are recommended for adults. According to NICE, which is appropriate?**

**A.** Individual CBT for eating disorders, the Maudsley Anorexia Nervosa Treatment for Adults, or specialist supportive clinical management, chosen by the patient after explanation

**B.** Family therapy only

**C.** Drug therapy as the sole treatment

**D.** Weight-loss counseling

**E.** Group exposure therapy only

Answer Contents

**BQ-405**

**A 19-year-old with anorexia nervosa is above the weight a colleague calls the "admission cutoff" but is losing weight quickly and has a prolonged QT interval on ECG. Which principle best guides the level of care?**

**A.** Admit only when weight falls below a published cutoff

**B.** Treat as an outpatient until weight falls further

**C.** Rate of weight loss, vital signs, laboratory, and ECG findings matter, because NICE advises against an absolute weight or BMI threshold

**D.** Discharge as soon as a healthy weight is reached

**E.** Use inpatient care only for psychological treatment

Answer Contents

**BQ-406**

**A 17-year-old with a restrictive eating disorder is seen in clinic. According to Society for Adolescent Health and Medicine criteria, which finding by itself would justify hospitalization?**

**A.** Blood pressure 98/60 mm Hg

**B.** Temperature 36.2 degrees C

**C.** An orthostatic heart rate rise of 25 beats per minute

**D.** A daytime heart rate of 48 beats per minute

**E.** A nighttime heart rate of 52 beats per minute

Answer Contents

**BQ-407**

**A 28-year-old outpatient with anorexia nervosa asks whether olanzapine will help. Which statement is most accurate?**

**A.** It is FDA-approved for anorexia nervosa

**B.** In one placebo-controlled trial it produced a modest BMI gain without improving obsessionality

**C.** It can replace psychotherapy

**D.** It reliably improves body image

**E.** It has been shown to lower mortality

Answer Contents

**BQ-408**

**A high-risk 20-year-old with severe malnutrition is about to start nutritional rehabilitation on a medical unit. Which is the best step before the first feeding?**

**A.** Check potassium, magnesium, and phosphate, replete low values, and give thiamine

**B.** Delay thiamine until the third day

**C.** Start dextrose-containing fluids before any laboratory tests

**D.** Check phosphate only if symptoms appear

**E.** Skip monitoring because refeeding syndrome only occurs in children

Answer Contents

**BQ-409**

**A 22-year-old woman of normal weight binges and induces vomiting daily. Which laboratory pattern is most likely?**

**A.** Hyperkalemia with metabolic acidosis

**B.** Hypernatremia with respiratory alkalosis

**C.** Hypercalcemia

**D.** Hypermagnesemia with normal pH

**E.** Hypokalemia with metabolic alkalosis

Answer Contents

**BQ-410**

**A 30-year-old woman with depression and a past history of bulimia nervosa, in remission for 2 years, asks about bupropion for smoking cessation. Which response is correct?**

**A.** Prescribe at the standard dose

**B.** Prescribe after a normal ECG

**C.** Choose another agent, because bupropion is contraindicated with a current or past bulimia nervosa diagnosis

**D.** Prescribe with lithium for protection

**E.** Prescribe, because the disorder is in remission

Answer Contents

**BQ-411**

**A 16-year-old with bulimia nervosa lives with engaged parents. Which treatment has guideline support for this age group?**

**A.** Fluoxetine alone

**B.** Bupropion

**C.** Inpatient behavioral weight management

**D.** Family-based treatment adapted for bulimia nervosa

**E.** Lisdexamfetamine

Answer Contents

**BQ-412**

**A 41-year-old woman with binge-eating disorder and obesity asks to begin a strict weight-loss diet. Which approach is most consistent with guidelines?**

**A.** A strict weight-loss diet first

**B.** Guided self-help or CBT for eating disorders, noting that weight loss is not the treatment target

**C.** Lisdexamfetamine as monotherapy

**D.** Fluoxetine 60 mg alone

**E.** Bariatric surgery referral before any psychological treatment

Answer Contents

**BQ-413**

**A 35-year-old man with moderate to severe binge-eating disorder starts lisdexamfetamine 30 mg every morning. Which titration plan matches the label?**

**A.** Increase by 20 mg at about weekly intervals toward 50 to 70 mg daily

**B.** Start at 70 mg

**C.** Increase by 10 mg daily to 100 mg

**D.** Keep 30 mg as the target dose

**E.** Increase by 50 mg monthly

Answer Contents

**BQ-414**

**A 34-year-old man eats large amounts with loss of control two to three times a week for 6 months and feels ashamed afterward. He does not vomit, use laxatives, fast, or exercise to compensate. Which is the most likely diagnosis?**

**A.** Bulimia nervosa

**B.** Anorexia nervosa, binge-eating/purging type

**C.** A purging pattern without binge eating

**D.** Night eating without loss of control

**E.** Binge-eating disorder

Answer Contents

**BQ-415**

**A 12-year-old boy refuses solid food after a choking episode and has lost weight. He worries about choking, not about weight or shape. Which is the most likely diagnosis?**

**A.** Anorexia nervosa, restricting type

**B.** Normal picky eating

**C.** Avoidant/restrictive food intake disorder

**D.** Bulimia nervosa

**E.** Rumination disorder

Answer Contents

**BQ-416**

**The parents of a 13-year-old with avoidant/restrictive food intake disorder ask what treatment is supported. Which statement is most accurate?**

**A.** Randomized trials establish family-based treatment as superior to all others

**B.** Fluoxetine is FDA-approved for the disorder

**C.** Bupropion is first-line

**D.** CBT or family-based approaches are commonly used, but controlled trial data are limited

**E.** Long-term inpatient admission is required

Answer Contents

**BQ-417**

**A 26-year-old pregnant woman eats clay daily and is fatigued. Which is the best next step?**

**A.** Evaluate for iron deficiency and other nutritional deficiency, and consider blood lead testing

**B.** Start an antipsychotic

**C.** Diagnose bulimia nervosa

**D.** Reassure her, since pica is always harmless

**E.** Order a weight-loss program

Answer Contents

**BQ-418**

**A 19-year-old effortlessly regurgitates and rechews food after most meals, with no nausea and no concern about weight, for 3 months. Endoscopy is normal. Which is the best initial treatment?**

**A.** High-dose proton pump inhibitor alone

**B.** Diaphragmatic breathing training with behavioral support

**C.** Fluoxetine 60 mg daily

**D.** A prokinetic agent alone

**E.** Fundoplication

Answer Contents

SUBJECT 19

# Sleep-Wake, Sexual, Gender, and Paraphilic Disorders

Questions BQ-419 to BQ-437

**BQ-419**

**A 61-year-old man with a body mass index of 41 has loud snoring and witnessed apneas. Home testing shows 46 events per hour, which is severe. Using the VA/DoD algorithm, which is the first-line treatment?**

**A.** Mandibular advancement device alone

**B.** Oxygen therapy alone

**C.** Positive airway pressure

**D.** Atomoxetine with oxybutynin

**E.** Modafinil alone

Answer Contents

**BQ-420**

**A 30-year-old man with narcolepsy takes phenelzine for treatment-resistant depression and needs a wake-promoting drug. Which drug is contraindicated with his current antidepressant?**

**A.** Solriamfetol

**B.** Pitolisant

**C.** Sodium oxybate

**D.** Modafinil

**E.** Armodafinil

Answer Contents

**BQ-421**

**A 40-year-old woman with narcolepsy starts pitolisant. She also takes an antipsychotic that prolongs the QT interval and a combined oral contraceptive. Which counseling is most accurate?**

**A.** Pitolisant is a controlled drug that requires a restricted program

**B.** Hormonal contraception stays fully effective with pitolisant

**C.** Pitolisant is dosed at bedtime to reduce insomnia

**D.** QT prolongation is not a concern with this drug

**E.** Avoid QT-prolonging combinations and use a non-hormonal contraceptive method

Answer Contents

**BQ-422**

**A 45-year-old man with narcolepsy type 1 takes a stable dose of sodium oxybate. His psychiatrist plans to start divalproex for a mood disorder. What adjustment does the oxybate label call for?**

**A.** Raise the oxybate dose by at least 20 percent

**B.** Lower the oxybate dose by at least 20 percent

**C.** Make no change

**D.** Stop oxybate permanently

**E.** Give both nightly doses together at bedtime

Answer Contents

**BQ-423**

**A 72-year-old woman with sleep maintenance insomnia has partly responded to CBT-I and wants a medication. She has no glaucoma or urinary retention and takes no MAO inhibitor. Which regimen matches the low-dose doxepin label?**

**A.** 25 mg with the evening meal

**B.** 6 mg after dinner, regardless of food

**C.** 10 mg at bedtime

**D.** 3 mg within 30 minutes of bedtime, not within 3 hours of a meal

**E.** 6 mg three times daily

Answer Contents

**BQ-424**

**A 34-year-old man has trouble falling asleep but sleeps through the night. He wants a non-controlled medicine and takes fluvoxamine for obsessive-compulsive disorder. Which statement about ramelteon is correct?**

**A.** It is contraindicated with fluvoxamine

**B.** It is a Schedule IV drug

**C.** It is indicated for sleep maintenance

**D.** It carries a boxed warning for complex sleep behaviors

**E.** VA/DoD names it as a preferred hypnotic

Answer Contents

**BQ-425**

**A 59-year-old woman with sleep maintenance insomnia takes diltiazem, a moderate CYP3A inhibitor. If suvorexant is chosen, which dosing fits the label?**

**A.** 20 mg at bedtime

**B.** 15 mg at bedtime

**C.** 5 mg, generally not exceeding 10 mg

**D.** 10 mg at bedtime with no adjustment

**E.** Avoid the drug because it is contraindicated

Answer Contents

**BQ-426**

**A 55-year-old woman has taken temazepam nightly for 6 years and asks about its risks. Which risk is covered by the current benzodiazepine class boxed warning?**

**A.** Respiratory depression only when combined with alcohol

**B.** Hepatotoxicity requiring liver tests every month

**C.** Complex sleep behaviors as the only listed risk

**D.** Torsades de pointes from QT prolongation

**E.** Abuse, misuse, addiction, physical dependence, and withdrawal reactions

Answer Contents

**BQ-427**

**A 48-year-old man with chronic insomnia has improved partly with CBT-I and asks for trazodone because a friend takes it. Which response best reflects the AASM and VA/DoD guidelines?**

**A.** Trazodone is strongly recommended for chronic insomnia

**B.** Trazodone is weakly advised against, and other drugs have more support

**C.** Trazodone is weakly recommended for sleep maintenance

**D.** Trazodone is the guideline-preferred drug for older adults

**E.** Trazodone is the first-line drug for sleep onset insomnia

Answer Contents

**BQ-428**

**A 57-year-old woman took pramipexole for restless legs for 2 years. Symptoms now start in the afternoon and have spread to her arms. Iron studies were never checked. What is the best next step?**

**A.** Raise the pramipexole dose

**B.** Add a second dopamine agonist

**C.** Add clonazepam at bedtime

**D.** Check iron studies and move toward a gabapentinoid while tapering the dopamine agonist

**E.** Start bupropion

Answer Contents

**BQ-429**

**A 16-year-old cannot fall asleep before 3 AM, sleeps 7 hours when allowed to wake at 10 AM, and misses first period. He has no mood symptoms and snores rarely. Which approach is best supported?**

**A.** Zolpidem nightly

**B.** A daytime stimulant

**C.** Timed melatonin with a fixed wake time and morning light

**D.** A 5 AM forced wake time alone

**E.** An atypical antipsychotic at bedtime

Answer Contents

**BQ-430**

**A 44-year-old blind woman with a non-24-hour sleep-wake disorder also takes fluvoxamine for OCD. Which statement about tasimelteon is most appropriate?**

**A.** Avoid combining it with fluvoxamine, which raises tasimelteon exposure about 7-fold

**B.** Fluvoxamine lowers tasimelteon exposure, so raise the dose

**C.** Take it in the morning with a heavy meal

**D.** It is a Schedule IV drug

**E.** It is labeled only for children

Answer Contents

**BQ-431**

**A 38-year-old veteran with PTSD reports nightly trauma nightmares for a year. He wants to avoid medication. According to the AASM 2018 position paper, which treatment is recommended?**

**A.** Venlafaxine

**B.** Clonazepam

**C.** Prazosin

**D.** Quetiapine

**E.** Image rehearsal therapy

Answer Contents

**BQ-432**

**A 41-year-old woman with acquired generalized hypoactive sexual desire disorder takes flibanserin at bedtime. She plans a dinner at which she expects to drink three or more alcoholic drinks. What should she do about that evening's dose?**

**A.** Take it as usual at bedtime

**B.** Take double the dose in the morning

**C.** Take it before the first drink

**D.** Skip that evening's dose

**E.** Take it with a large meal

Answer Contents

**BQ-433**

**A 35-year-old premenopausal woman with hypoactive sexual desire disorder takes oral naltrexone for alcohol use disorder. Which statement about bremelanotide is correct?**

**A.** It is safe with oral naltrexone and no change is needed

**B.** Avoid using it with oral naltrexone because naltrexone exposure may fall

**C.** It raises naltrexone levels and causes toxicity

**D.** It is given daily by mouth

**E.** It is approved for use in men

Answer Contents

**BQ-434**

**A 29-year-old man has had lifelong premature ejaculation. Erections are normal. Which treatment is first-line in the AUA/SMSNA guideline?**

**A.** A topical penile anesthetic

**B.** Daily tramadol

**C.** Sildenafil alone

**D.** Testosterone gel

**E.** An alpha-blocker alone

Answer Contents

**BQ-435**

**A 33-year-old woman stopped escitalopram 4 months ago and still has reduced sexual response. She asks whether this can be a medication effect. Which statement about regulators is accurate?**

**A.** Regulators have shown that it never occurs

**B.** Its frequency is well established

**C.** European regulators asked that persistent sexual dysfunction after withdrawal be added to product information, and its frequency is unknown

**D.** It occurs only with paroxetine

**E.** It is limited to women

Answer Contents

**BQ-436**

**A 28-year-old transgender man asks whether hormone therapy will improve his depression. His psychiatrist cites a 2021 systematic review. Which statement about that review is best supported?**

**A.** It showed fewer suicide deaths with treatment

**B.** It showed strong certainty of benefit

**C.** It showed harm to mental health

**D.** It studied only adolescents

**E.** It linked treatment to less depression and anxiety, with limited-certainty evidence and no conclusion on suicide deaths

Answer Contents

**BQ-437**

**A 27-year-old man has intrusive images of sexually harming a child. The images horrify him and cause no arousal. He avoids children and repeatedly checks that he has not harmed anyone. Which diagnosis is most likely?**

**A.** Pedophilic disorder

**B.** Obsessive-compulsive disorder

**C.** Exhibitionistic disorder

**D.** Compulsive sexual behavior disorder

**E.** Delusional disorder

Answer Contents

SUBJECT 20

# Child and Adolescent Psychiatry

Questions BQ-438 to BQ-461

**BQ-438**

**An 11-year-old girl has had 6 months of excessive worry, stomachaches, and reluctance to go to school. Her anxiety is moderate and impairs her at school and with friends. Which treatment approach has the strongest trial support?**

**A.** A benzodiazepine taken as needed

**B.** Cognitive behavioral therapy combined with sertraline

**C.** Hydroxyzine alone

**D.** Watchful waiting for 6 months

**E.** Quetiapine at bedtime

Answer Contents

**BQ-439**

**A 9-year-old boy is about to start atomoxetine for ADHD. His father asks about the boxed warning. Which statement is most accurate?**

**A.** The warning applies only to adults

**B.** Pooled pediatric trials found no suicidal ideation

**C.** The warning concerns liver failure only

**D.** In short-term pediatric trials, suicidal ideation occurred in 0.4 percent of treated children versus 0 percent on placebo, with no suicides

**E.** The warning was removed once long-acting forms were approved

Answer Contents

**BQ-440**

**A 10-year-old girl has taken clonidine extended-release for ADHD for 6 months. Her family ran out of the medication, and she stopped it for 3 days. She now has headache, restlessness, and an elevated blood pressure. What is the most likely explanation?**

**A.** Rebound hypertension after abrupt discontinuation

**B.** Serotonin syndrome

**C.** Neuroleptic malignant syndrome

**D.** Stimulant withdrawal

**E.** Water intoxication

Answer Contents

**BQ-441**

**A 30-month-old boy uses about 10 words and no two-word combinations. He points to show things, plays pretend games, and seeks out his parents for comfort. He has never had a hearing test. What is the best next step?**

**A.** Begin intervention for autism without further testing

**B.** Reassure the family and recheck at 4 years

**C.** Obtain audiology and refer for speech-language evaluation

**D.** Order a genetic panel before any other assessment

**E.** Start a trial of an alpha-2 agonist

Answer Contents

**BQ-442**

**A 4-year-old boy has stuttered for 5 months, with visible facial tension and effort when he speaks. His father stuttered as a child. What is the best next step?**

**A.** Ask the parents to have him slow down and repeat each sentence

**B.** Start a low-dose antipsychotic

**C.** Wait until he enters school

**D.** Prescribe a medication approved for stuttering

**E.** Refer for speech-language evaluation

Answer Contents

**BQ-443**

**An 11-year-old boy has Tourette syndrome. His tics cause teasing at school and neck pain. He has no ADHD or obsessive-compulsive symptoms. His parents ask for treatment. Which option should be offered first?**

**A.** Comprehensive Behavioral Intervention for Tics (CBIT)
**B.** Haloperidol
**C.** Risperidone
**D.** Deep brain stimulation
**E.** Botulinum toxin injections

Answer Contents

**BQ-444**

**A 3-month-old girl is brought to the emergency department for a bruise on her ear. The caregiver says she rolled off a couch. Applying the TEN-4-FACESp bruising rule, what is the correct interpretation?**

**A.** Negative, because the bruise is small
**B.** Negative, because the bruise is not patterned
**C.** Positive, so further evaluation for physical abuse is warranted
**D.** Diagnostic of abuse, so no further evaluation is needed
**E.** Not applicable to children under 1 year

Answer Contents

**BQ-445**

**A 4-year-old boy has autism, intellectual disability, and dysmorphic features. Chromosomal microarray and fragile X testing were normal. What is the best next genetic step?**

**A.** Repeat the karyotype
**B.** Lead level only
**C.** EEG
**D.** Stop testing, since common causes are excluded
**E.** Whole exome sequencing

Answer Contents

**BQ-446**

**An 8-year-old girl has struggled to read despite adequate instruction. Testing shows a specific learning disorder. The school has offered extra seating accommodations only, and her parents ask about specialized instruction. What is the best advice?**

**A.** Start a stimulant medication to improve her reading

**B.** Request an evaluation for special education services through an individualized education program under IDEA

**C.** Wait until she is 10 to see whether she catches up on her own

**D.** Apply for early intervention services under IDEA Part C

**E.** Seek private speech therapy alone and decline school services

Answer Contents

**BQ-447**

**A 14-year-old boy weighing 60 kg has severe tics from Tourette syndrome. CBIT was only partly helpful, and clonidine caused daytime sedation. His physician plans aripiprazole. Which dosing plan matches the label?**

**A.** Start 10 mg daily

**B.** Start 2 mg daily, with a maximum of 10 mg daily

**C.** Start 5 mg daily, with a target of 15 mg daily

**D.** Start 2 mg daily, target 10 mg daily, maximum 20 mg daily

**E.** Start 2 mg daily, with a maximum of 30 mg daily

Answer Contents

**BQ-448**

**A 16-year-old girl has taken fluoxetine for 3 weeks for depression. She now sleeps 3 hours a night without fatigue, speaks rapidly, and has spent her savings on impulsive purchases. Her mother has bipolar I disorder. What is the best next step?**

**A.** Increase the fluoxetine dose

**B.** Add a stimulant

**C.** Continue fluoxetine and add clonazepam

**D.** Stop fluoxetine and evaluate for mania or hypomania

**E.** Switch to venlafaxine

Answer Contents

**BQ-449**

**A 7-year-old boy has inattention and restlessness at school. His parents report loud snoring, restless sleep, and daytime sleepiness. His tonsils are large. What should come first?**

**A.** Evaluate for sleep-disordered breathing

**B.** Start methylphenidate

**C.** Start atomoxetine

**D.** Start risperidone

**E.** Treat for oppositional defiant disorder

Answer Contents

**BQ-450**

**A healthy 9-year-old boy with ADHD is about to start methylphenidate. He has no cardiac symptoms, fainting, or family history of sudden death. Which baseline assessment is most consistent with NICE and AACAP guidance?**

**A.** ECG and echocardiogram for every child before the first dose

**B.** Liver panel and complete blood count before the first dose

**C.** EEG to exclude a seizure disorder before the first dose

**D.** Cardiology consultation for every child before the first dose

**E.** Height, weight, pulse, and blood pressure, with an ECG only if cardiac features are present

Answer Contents

**BQ-451**

**A 12-year-old boy is starting lithium for bipolar I disorder. Which monitoring plan is most appropriate?**

**A.** Weekly complete blood counts throughout treatment

**B.** Fasting lipids and glucose only

**C.** Renal and thyroid function before and during treatment, with a serum level 3 days after starting, drawn 12 hours after the dose

**D.** Prolactin and fasting glucose only

**E.** Monthly ECG throughout treatment

Answer Contents

**BQ-452**

**A 5-year-old girl has severe tantrums, irritability, and high energy for a year, with no discrete episodes of changed mood. Her father has bipolar disorder, and her parents ask whether she has it. What is the best response?**

**A.** Start lithium now to protect against future mania given the family history

**B.** Explain that the bipolar diagnosis is not established in children under 6, and assess for ADHD, anxiety, trauma, and chronic irritability while starting parent training

**C.** Start risperidone for irritability and tantrums

**D.** Diagnose bipolar II disorder on the basis of the family history

**E.** Start an antidepressant for chronic irritability

Answer Contents

**BQ-453**

**A 16-year-old boy used cannabis several times in the past year. On the CRAFFT 2.1, he answers yes to three of the six questions. How should his risk be classified?**

**A.** Low risk, so give praise and information only

**B.** High risk, so give brief advice, schedule follow-up, and consider referral

**C.** Negative, because only cannabis was used

**D.** Medium risk, so provide information with no follow-up

**E.** Indeterminate until a urine toxicology result returns

Answer Contents

**BQ-454**

**A 12-year-old girl is seen in a pediatric emergency department for abdominal pain. She answers yes to one of the four Ask Suicide-Screening Questions. She has no behavioral complaints. What is the best next step?**

**A.** Discharge with reassurance because her complaint is physical

**B.** Repeat the screen in a year

**C.** Discharge on an antidepressant

**D.** Admit involuntarily

**E.** Complete a brief suicide safety assessment before disposition

Answer Contents

**BQ-455**

**A 15-year-old boy is stable on extended-release methylphenidate. His parents ask about safety at home. Which counseling is most consistent with the boxed warning?**

**A.** Keep the medication locked, count the doses, and ask at every visit about misuse and sharing

**B.** Misuse is not a concern with extended-release products at all

**C.** The boxed warning applies only to adults

**D.** Let him carry a month's supply to school for convenience

**E.** Switch to immediate-release tablets for greater safety

Answer Contents

**BQ-456**

**An 8-year-old boy has taken extended-release methylphenidate for 6 months. His appetite is poor, his weight has fallen from the 50th to the 25th percentile, and his height has plateaued. What is the best step?**

**A.** Continue unchanged, because growth effects of stimulants are always temporary

**B.** Stop all ADHD treatment permanently

**C.** Plot growth, review dose and timing, and consider a break or a change of drug if growth does not recover

**D.** Raise the dose to improve appetite and weight

**E.** Add an SSRI to improve appetite

Answer Contents

**BQ-457**

**The parents of a newly diagnosed 2-year-old with autism ask whether therapy can cure her. Which response is most accurate?**

**A.** Therapy started before age 3 cures most children with autism

**B.** No intervention has any supporting evidence, so services should be deferred

**C.** Medication should be started first, with therapy added only if it fails

**D.** Early intervention can improve skills in trials, but benefits are mostly specific, and the certainty of the evidence is limited, so start early without promising a cure

**E.** Wait for school-age testing before beginning any services

Answer Contents

**BQ-458**

**A 9-year-old boy with autism and severe aggression has started risperidone after behavioral interventions failed. Which review plan is consistent with NICE guidance?**

**A.** Continue indefinitely without any scheduled review

**B.** Double the dose weekly until the aggression stops

**C.** Review at 3 to 4 weeks, and stop if there is no clinically important response by 6 weeks

**D.** Switch to valproate after 2 weeks of treatment

**E.** Add a stimulant to reduce the aggression

Answer Contents

**BQ-459**

**An 8-year-old boy with primary nocturnal enuresis is prescribed desmopressin tablets at bedtime. Which counseling is correct?**

**A.** Limit fluids from 1 hour before the dose until morning, and hold the drug during vomiting, diarrhea, or fever

**B.** Encourage extra fluids at bedtime to prevent dehydration overnight

**C.** Take the dose with a large glass of water to aid absorption

**D.** An SSRI is the preferred add-on if desmopressin fails

**E.** Increase the dose to 1.2 mg if there is no response

Answer Contents

**BQ-460**

**A pediatric practice asks what the US Preventive Services Task Force recommended in 2022. Which statement is accurate?**

**A.** Screen all children from age 6 for suicide risk with a validated tool

**B.** Evidence is insufficient for screening adolescents for depression

**C.** Use specific PHQ-9 cutoffs for all adolescents

**D.** Screen adolescents aged 12 to 18 for major depressive disorder, with insufficient evidence for suicide risk screening

**E.** Recommend against anxiety screening at ages 8 to 18

Answer Contents

**BQ-461**

**A 13-year-old girl reached remission of generalized anxiety on escitalopram plus CBT after 4 months. Her parents ask when to stop the medication. Which advice is best?**

**A.** Stop the medication now, since she is well

**B.** Continue the effective dose for about 12 months after remission, then taper slowly with monitoring

**C.** Continue the medication for life

**D.** Stop abruptly at the start of summer break

**E.** Stop after another 4 weeks of remission

Answer Contents

SUBJECT 21

# Perinatal, Reproductive, and Geriatric Psychiatry

Questions BQ-462 to BQ-477

**BQ-462**

**A 26-year-old woman is seen 6 weeks after delivery. Her Edinburgh Postnatal Depression Scale total is 8, below the usual cutoff, but she marked "sometimes" on the item about thoughts of harming herself. She says she is tired but coping. What is the best next step?**

**A.** Record the negative screen and repeat the scale in one year

**B.** Start an SSRI on the basis of the single item

**C.** Refer her to a lactation consultant for sleep advice

**D.** Ask directly about suicidal thoughts, plan, and intent, and assess risk now

**E.** Reassure her that a score below the cutoff excludes risk

Answer Contents

**BQ-463**

**You direct a prenatal clinic that screens for depression only at the six-week postpartum visit. A new colleague asks which change best matches ACOG guidance. What do you recommend?**

**A.** Screen at the first prenatal visit, later in pregnancy, and at postpartum visits, with follow-up systems for positive screens

**B.** Screen only patients with a prior psychiatric history, at the first prenatal visit

**C.** Screen only when patients raise mood concerns during visits

**D.** Screen once, at 12 weeks postpartum, with an anxiety scale alone

**E.** Screen on the delivery admission as the single time point

Answer Contents

**BQ-464**

**A 30-year-old woman at 10 weeks of pregnancy has had low mood for a month and asks for sertraline. She describes three past episodes of about 4 days with little need for sleep, racing thoughts, and heavy spending, and her mother has bipolar disorder. What is the best next step?**

**A.** Start sertraline at a low dose and review in 4 weeks

**B.** Begin psychotherapy only and skip any further history

**C.** Take a structured history and screen for bipolar disorder before choosing treatment

**D.** Start valproate for presumed bipolar disorder

**E.** Defer any assessment until after delivery

Answer Contents

**BQ-465**

**A 33-year-old woman with bipolar I disorder takes lithium and is at 37 weeks of pregnancy with a stable level. She asks what to do with the lithium around delivery. Which plan is most consistent with the label?**

**A.** Stop lithium permanently when labor begins, with no restart planned

**B.** Double the dose in the week before delivery to prevent relapse

**C.** Continue the same dose unchanged with no further level checks

**D.** Switch to valproate for the final month of pregnancy

**E.** Reduce or stop it 2 to 3 days before delivery, then restart at the preconception dose with level checks

Answer Contents

**BQ-466**

**A breastfeeding woman, 8 weeks postpartum, is stable on sertraline for depression and fears that her milk will harm the infant. The infant is feeding and gaining weight normally. What is the best counseling?**

**A.** Stop breastfeeding because sertraline is unsafe in milk

**B.** Continue breastfeeding; milk levels are low, so watch the infant for sedation or poor feeding

**C.** Pump and discard all milk for a full day after each dose

**D.** Stop sertraline and switch to lithium to protect the infant

**E.** Add a benzodiazepine so that she sleeps while the infant feeds

Answer Contents

**BQ-467**

**A 2-day-old term newborn is jittery, feeds poorly, and has mild tachypnea. The mother took sertraline throughout pregnancy. Sepsis and hypoglycemia have been excluded. Which explanation and expectation are best?**

**A.** Neonatal adaptation after late SSRI exposure, which usually resolves within about 2 weeks

**B.** Fetal valproate syndrome, which persists for life

**C.** Lithium-related floppy baby syndrome

**D.** Congenital heart disease caused by first-trimester SSRI exposure

**E.** Neonatal opioid withdrawal that requires methadone

Answer Contents

**BQ-468**

**A 28-year-old woman with bipolar II disorder is stable on carbamazepine and uses an oral contraceptive. She asks whether she is fully protected against pregnancy. What is the best counseling?**

**A.** Her pill is unaffected because carbamazepine does not change hormone metabolism

**B.** Carbamazepine raises estrogen levels and mainly increases clotting risk

**C.** Carbamazepine affects only barrier methods such as condoms

**D.** Carbamazepine induces CYP3A4, so the pill may fail; add or choose a more reliable method

**E.** Folic acid fully reverses the interaction and protects against pregnancy

Answer Contents

**BQ-469**

**A 27-year-old woman at 16 weeks of pregnancy has generalized anxiety with panic attacks and asks for alprazolam, which helped her in the past. She has had no SSRI trial. What is the best plan?**

**A.** Prescribe alprazolam as long-term first-line therapy

**B.** Withhold all treatment until after delivery

**C.** Offer CBT and an SSRI such as sertraline or escitalopram, and avoid or limit benzodiazepines

**D.** Start valproate for anxiety

**E.** Prescribe diphenhydramine every night

Answer Contents

**BQ-470**

**A 27-year-old woman, 3 weeks postpartum, is distressed by recurrent images of dropping her baby on the stairs. She knows the thoughts are senseless, avoids carrying the infant on stairs, and has no hallucinations or delusions. What is the best next step?**

**A.** Admit her to hospital for presumed postpartum psychosis

**B.** Start olanzapine and lorazepam for presumed psychosis

**C.** Report the thoughts as evidence of intent to harm the infant

**D.** Prescribe a hypnotic and discharge her without further assessment

**E.** Explain that such intrusive thoughts occur in perinatal OCD and offer an SSRI and CBT

Answer Contents

**BQ-471**

**A 31-year-old woman with bipolar II disorder took lamotrigine at about double her pre-pregnancy dose through the third trimester. Eight days after delivery she reports dizziness, double vision, and unsteadiness. What is the most likely cause and best step?**

**A.** Postpartum psychosis, so admit her and start olanzapine now

**B.** Lamotrigine toxicity as clearance falls after delivery; check a level and reduce toward the earlier dose

**C.** Normal postpartum fatigue, so reassure her and continue the same dose

**D.** Hypoglycemia from breastfeeding, so advise her to eat extra meals

**E.** Serotonin syndrome, so stop lamotrigine and give cyproheptadine

Answer Contents

**BQ-472**

**A 49-year-old woman with irregular periods and nightly hot flashes has her first moderate major depressive episode. She asks whether estrogen alone can treat her depression. Which statement is most accurate?**

**A.** Estradiol is FDA-approved to treat depression in perimenopausal women

**B.** Antidepressants do not work during the menopausal transition because of hormone changes

**C.** Psychotherapy is ineffective for depression that begins in midlife

**D.** Antidepressants and psychotherapy are first-line; estradiol has limited evidence and no FDA approval for mood

**E.** Hormone therapy is the only proven treatment for perimenopausal depression

Answer Contents

**BQ-473**

**A 52-year-old woman with obsessive-compulsive disorder takes fluvoxamine and asks for treatment of severe hot flashes. Which option is contraindicated with her current regimen?**

**A.** Fezolinetant

**B.** Cognitive behavioral therapy for vasomotor symptoms

**C.** Clinical hypnosis

**D.** Gabapentin

**E.** Oxybutynin

Answer Contents

**BQ-474**

**A 72-year-old woman is started on citalopram 30 mg daily for depression. She also takes a diuretic and has a low potassium level. Which action is most appropriate?**

**A.** Increase citalopram to 40 mg daily for faster response

**B.** Continue 30 mg daily and add an antiarrhythmic drug

**C.** Withhold potassium replacement to avoid interactions

**D.** Switch to amitriptyline for better tolerability

**E.** Limit citalopram to 20 mg daily, correct potassium, and review QT-prolonging drugs

Answer Contents

**BQ-475**

**An 81-year-old man has chronic insomnia and a recent fall. He takes zolpidem and diphenhydramine every night and asks for a stronger sleeping pill. What is the best plan?**

**A.** Add temazepam to the current regimen

**B.** Replace diphenhydramine with hydroxyzine

**C.** Taper the sedative-hypnotics gradually and start cognitive behavioral therapy for insomnia

**D.** Increase zolpidem to the maximum dose

**E.** Add quetiapine at bedtime

Answer Contents

**BQ-476**

**A 71-year-old woman has had 4 months of fixed beliefs that neighbors enter her home, with voices and tactile hallucinations of insects on her skin. She has no formal thought disorder, her cognitive screen is normal, and she has mild hearing loss. What is the best next step?**

**A.** Diagnose schizophrenia and start a standard adult-dose antipsychotic today

**B.** Evaluate medical, medication, and cognitive causes first, then consider a very low antipsychotic dose

**C.** Start donepezil for presumed Alzheimer disease and review in one month

**D.** Prescribe a benzodiazepine to settle her fear and help her sleep

**E.** Arrange electroconvulsive therapy as first-line treatment for the psychosis

Answer Contents

**BQ-477**

**A 66-year-old widow, 14 months after her husband's death, describes constant yearning, disbelief that he is gone, avoidance of reminders, and intense loneliness. Her work function has declined. She does not have pervasive low mood or worthlessness. What is the best treatment?**

**A.** Citalopram alone

**B.** Alprazolam for loneliness

**C.** Watchful waiting because grief always resolves

**D.** Quetiapine at bedtime

**E.** Grief-focused, CBT-based therapy

Answer Contents

# Suicide, Violence, and Psychiatric Emergencies

Questions BQ-478 to BQ-499

**BQ-478**

**A 15-year-old girl is seen after a cutting episode. A clinician uses the Columbia Suicide Severity Rating Scale. Which element is rated specifically for actual attempts, based on the medical outcome of the attempt?**

**A.** Ideation severity, from wish to be dead to intent with a plan

**B.** Lethality of the attempt

**C.** Ideation intensity, including frequency and controllability

**D.** Preparatory behavior

**E.** Aborted attempts

Answer Contents

**BQ-479**

**In a meta-analysis of 37 studies, 5.5 percent of patients categorized as high risk died by suicide over a mean follow-up of 63 months, compared with 0.9 percent of lower-risk patients. A colleague proposes admitting every patient who receives a high-risk label. What is the best interpretation of these data?**

**A.** Risk categories are accurate enough to set disposition without further assessment

**B.** Nearly all patients labeled high risk will die by suicide without admission

**C.** A lower-risk label means follow-up is unnecessary

**D.** Most patients labeled high risk will not die by suicide, so the label should not set disposition alone

**E.** A sensitivity of 56 percent means that 56 percent of high-risk patients die by suicide

Answer Contents

**BQ-480**

**A 32-year-old woman is in the emergency department after self-poisoning. The on-call manager asks you to use a low, medium, or high rating to decide whether she can be discharged. Which response is most consistent with NICE guidance on self-harm?**

**A.** Complete a psychosocial assessment with a risk formulation and base disposition on her needs, safety, and a plan

**B.** Use the global rating, because it predicts repeat self-harm

**C.** Defer the psychosocial assessment until all medical treatment is complete

**D.** Use a validated scale score alone to decide who is offered treatment

**E.** Discharge her if she denies suicidal intent

Answer Contents

**BQ-481**

**A 50-year-old man with depression and passive suicidal ideation says he owns a hunting rifle and "would never use it." He agrees to talk about storage. Which approach is best?**

**A.** Accept his assurance and document that he has no access

**B.** Ask him to bring the rifle to the clinic so you can hold it

**C.** Tell him to give the rifle away permanently

**D.** Document that he declined means counseling

**E.** Arrange temporary storage outside the home with a trusted person, and revisit it

Answer Contents

**BQ-482**

**A 22-year-old woman with major depression is starting sertraline. She asks about the warning on the label. Which counseling statement best matches the label data?**

**A.** The warning applies only to adults older than 65

**B.** The warning applies only to children under 12

**C.** In trials, young adults aged 18 to 24 had 5 more cases of suicidal thoughts or behavior per 1,000 treated than placebo, so early close monitoring is advised

**D.** Adults aged 65 and older had 6 more cases per 1,000 than placebo

**E.** The boxed warning was removed, so no extra monitoring is needed

Answer Contents

**BQ-483**

**A 27-year-old man with schizophrenia has persistent suicidal ideation and two attempts despite two adequate antipsychotic trials. According to the 2024 VA/DoD guideline, which treatment has a weak recommendation for this situation?**

**A.** Lithium

**B.** Clozapine

**C.** Esketamine

**D.** Dialectical behavior therapy

**E.** Repetitive transcranial magnetic stimulation

Answer Contents

**BQ-484**

**A 35-year-old man with major depression and acute suicidal ideation is considered for esketamine nasal spray. Which statement is accurate?**

**A.** It can be dispensed for home use if the patient has a reliable partner

**B.** It carries no boxed warning

**C.** It replaces the need for a safety plan

**D.** It is given with an oral antidepressant in a certified setting, with observation for at least 2 hours

**E.** It is recommended first-line for all patients with suicidal ideation

Answer Contents

**BQ-485**

**A hospital wants to lower suicidal behavior after discharge from psychiatric admission for suicide risk. Which add-on to usual care has a weak recommendation in the 2024 VA/DoD guideline?**

**A.** Periodic caring communications, such as mail or text messages, for 12 months

**B.** Brief contact interventions after emergency department discharge

**C.** Crisis line referral alone

**D.** Standalone gatekeeper training for staff

**E.** Peer-to-peer programs

Answer Contents

**BQ-486**

**A 17-year-old describes cutting herself nightly for relief and says she does not want to die. Yesterday she took extra tablets "to see what would happen" and was not sure whether she wanted to die. What is the best action?**

**A.** Label the behavior as manipulation and set strict limits

**B.** Classify it as nonsuicidal self-injury and discharge her

**C.** Treat it as possible suicidal behavior, ask about intent again, and assess medical severity

**D.** Defer the psychosocial assessment until the medical team finishes

**E.** Ask her to sign an agreement to stop all self-injury

Answer Contents

**BQ-487**

**A staff team is responding to a shouting, pacing patient in a waiting room. Which team behavior is most consistent with the Project BETA de-escalation consensus?**

**A.** Several staff speak at once so that every demand is answered

**B.** Staff stand close together to show authority

**C.** Staff promise whatever the patient asks to end the encounter

**D.** Staff avoid setting any limits to prevent escalation

**E.** One staff member speaks while others clear bystanders and keep distance

Answer Contents

**BQ-488**

**A 40-year-old man is markedly intoxicated with alcohol, not withdrawing, and is agitated and threatening after verbal de-escalation has failed. Which approach is most consistent with the Project BETA psychopharmacology consensus?**

**A.** Benzodiazepine as the first-line agent, because the cause is undifferentiated

**B.** Use medication sparingly, avoid benzodiazepines, and if needed choose an antipsychotic

**C.** Begin an alcohol withdrawal protocol with scheduled benzodiazepines

**D.** Clonidine alone to blunt sympathetic overdrive

**E.** Combine several sedatives for rapid sleep

Answer Contents

**BQ-489**

**A 70-year-old man with hypokalemia who takes citalopram is agitated. A resident plans intravenous haloperidol. Which concern is most accurate?**

**A.** Haloperidol injection is approved for intravenous use at any dose

**B.** Intravenous use is safe if the daily dose stays below 20 mg

**C.** Hypokalemia lowers the risk of torsades in this setting

**D.** The injection is not approved intravenously, and hypokalemia and other QT drugs raise torsades risk

**E.** Citalopram prevents QT prolongation from haloperidol

Answer Contents

**BQ-490**

**A 36-year-old woman with schizophrenia is agitated. Her records note congenital long QT syndrome. Which agent is contraindicated by its label?**

**A.** Ziprasidone intramuscular

**B.** Olanzapine intramuscular

**C.** Lorazepam intramuscular

**D.** Benztropine intramuscular

**E.** Dexmedetomidine sublingual film

Answer Contents

**BQ-491**

**An emergency physician wants to give droperidol to a 45-year-old man. His baseline ECG shows a QTc of 455 msec. What does the label direct?**

**A.** Give 2.5 mg and monitor for 15 minutes

**B.** Give half the usual dose

**C.** Do not give it, because the QTc exceeds the label threshold for males

**D.** Give it only if potassium is normal

**E.** Give it intravenously rather than intramuscularly

Answer Contents

**BQ-492**

**A cooperative 31-year-old woman with schizophrenia has had asthma since childhood. She dislikes injections and requests the inhaled agent for agitation. Which statement is correct?**

**A.** It is acceptable if she is pretreated with albuterol

**B.** It is acceptable at half the usual dose

**C.** It is acceptable if she is monitored for 15 minutes

**D.** Asthma is only a relative caution

**E.** It is contraindicated because of the risk of bronchospasm

Answer Contents

**BQ-493**

**A 15-year-old on a medical unit is placed in restraint for violent behavior. Which approach is consistent with Centers for Medicare and Medicaid Services rules?**

**A.** A standing order for restraint as needed

**B.** An order lasting up to 2 hours before renewal, with a face-to-face evaluation within 1 hour

**C.** An order lasting up to 4 hours before renewal, with evaluation at the next shift

**D.** An order lasting up to 1 hour, with evaluation within 24 hours

**E.** Restraint for the patient′s convenience when staff are short

Answer Contents

**BQ-494**

**A 25-year-old man started haloperidol injections four days ago. He now has a temperature of 39.8 degrees C, lead-pipe rigidity, confusion, labile blood pressure, and a markedly raised creatine kinase. What is the best next step?**

**A.** Give another haloperidol dose for agitation

**B.** Add an anticholinergic and observe at home

**C.** Give cyproheptadine as the primary treatment

**D.** Stop the antipsychotic and begin intensive supportive care with cooling and fluids in a monitored setting

**E.** Start a serotonin reuptake inhibitor

Answer Contents

**BQ-495**

**A 22-year-old woman is mute, posturing, and resistant to movement. You give lorazepam 2 mg orally as a challenge after scoring the Bush-Francis Catatonia Rating Scale. When should you reassess, and what counts as a positive response?**

**A.** After 30 minutes; a 50 percent fall in the score

**B.** After 5 minutes; any change in appearance

**C.** After 24 hours; a 10 percent fall in the score

**D.** After 15 minutes; resolution of every sign

**E.** After 2 hours; a 25 percent fall in the score

Answer Contents

**BQ-496**

**A 24-year-old man, two days after starting haloperidol, has sudden twisting of his neck and upward deviation of his eyes. He is alert and breathing comfortably. Which treatment is labeled for this reaction?**

**A.** Cyproheptadine

**B.** Dantrolene

**C.** Benztropine by injection

**D.** A higher dose of haloperidol

**E.** Bromocriptine

Answer Contents

**BQ-497**

**A 45-year-old woman, 18 hours after her last drink, has a mild tremor and a CIWA-Ar score of 7. She has no history of withdrawal seizures or delirium. Which plan is most consistent with the ASAM guideline?**

**A.** Haloperidol to prevent hallucinations

**B.** A phenobarbital loading dose in the emergency department

**C.** Immediate discharge without any instructions

**D.** A large diazepam loading dose at once

**E.** Thiamine with repeated CIWA-Ar scoring, as medication is not yet indicated

Answer Contents

**BQ-498**

**During therapy, a patient tells you he plans to shoot his former partner and names her. Which statement best reflects *Tarasoff v. Regents of the University of California*?**

**A.** Confidentiality always overrides any duty to a third party

**B.** The therapist must use reasonable care to protect her, such as warning her or the police

**C.** A duty arises only if the patient has been hospitalized

**D.** The duty applies only if the therapist predicts violence with certainty

**E.** Documenting the statement fully satisfies the duty

Answer Contents

**BQ-499**

**A resident asks whether a validated rating scale can tell her if an inpatient with aggressive ideas will act violently. Which answer is most consistent with the APA evaluation guideline?**

**A.** A high score on a structured scale reliably predicts violence in the individual

**B.** Prediction is possible only with a physical examination

**C.** A low score rules out violence

**D.** No scale has been shown to predict individual aggression; assess the factors and document a judgment-based estimate

**E.** Risk assessment is unnecessary unless the patient has a weapon

Answer Contents

SUBJECT 23

# Consultation-Liaison Psychiatry

Questions BQ-500 to BQ-518

**BQ-500**

**A 41-year-old man on a surgical ward, the day after hernia repair, is described as "anxious and agitated, please evaluate for an anxiety disorder." He picks at his bedclothes, cannot hold his attention for a short counting task, and has a pulse of 112 and a temperature of 38.1 degrees C. He receives oxycodone, diphenhydramine, and lorazepam as needed. What is the best first step?**

**A.** Start sertraline for generalized anxiety disorder

**B.** Test attention and review vital signs, medications, and withdrawal history for delirium

**C.** Refer for outpatient cognitive behavioral therapy after discharge

**D.** Give an additional dose of lorazepam for the agitation

**E.** Document no psychiatric disorder and sign off the consultation

Answer Contents

**BQ-501**

**Two patients on the same ward are assessed for capacity. One accepts a routine oral antibiotic. The other refuses an operation without which death is likely. Which statement best describes how the standard for capacity is applied?**

**A.** The same level of ability must be required for both decisions

**B.** A higher level of ability is required to refuse than to accept in every case

**C.** Capacity is present whenever the clinician agrees with the choice

**D.** The level of ability required rises as the seriousness of the consequences rises

**E.** Capacity depends on whether the patient has a psychiatric diagnosis

Answer Contents

**BQ-502**

**A note from a hospitalist reads, "Patient is incompetent to refuse dialysis; please document." Which response by the consulting psychiatrist is most accurate?**

**A.** Competence is a legal determination made by a court; clinicians assess decision-making capacity for the specific decision

**B.** Competence is a clinical finding that only a psychiatrist may make

**C.** Competence applies to all decisions once any one is impaired

**D.** Competence is established by an MMSE score below a cutoff

**E.** Competence is determined by the family when a patient disagrees with the team

Answer Contents

**BQ-503**

**A 70-year-old woman scores 27 on the MMSE. She refuses a recommended biopsy and insists she has no tumor despite clear results, saying the scan was "mixed up with someone else's." A colleague says her score proves she has capacity. What is the best response?**

**A.** Agree, because any score above 24 excludes impaired decision-making capacity

**B.** Agree, because a high score shows she can reason about the options

**C.** Disagree, because the MMSE has no relationship to capacity in any patient

**D.** Disagree, because only the MoCA may be used for this purpose in hospitals

**E.** A high score makes incapacity less likely but cannot exclude impaired appreciation, so assess the four abilities

Answer Contents

**BQ-504**

**An 80-year-old man lacks capacity after a stroke. He has no advance directive or designated agent. His adult daughter and son disagree about a recommended operation. Which step is most consistent with the AMA Code of Medical Ethics?**

**A.** The surgeon decides alone, because the family cannot agree on a plan

**B.** Seek a court order before taking any other step toward a decision

**C.** Identify the surrogate under law and policy, use substituted judgment, and request ethics consultation for the disagreement

**D.** Follow the wishes of the child who lives closest to the hospital

**E.** Withhold all treatment until the siblings reach agreement

Answer Contents

**BQ-505**

**A violent adult in a general hospital is placed in restraints by a nurse under a standing as-needed order. Which statement matches the federal hospital conditions of participation?**

**A.** Standing and as-needed orders are not allowed; adult orders for violent behavior last up to 4 hours, with a face-to-face evaluation within 1 hour

**B.** As-needed orders are allowed if the practitioner reviews them each morning

**C.** Any nurse may continue restraint for 24 hours without a practitioner evaluation

**D.** Restraint may be used for staff convenience when the unit is short-staffed

**E.** A drug is never considered a restraint, whatever its purpose

Answer Contents

**BQ-506**

**An 82-year-old man on day 3 after hip fracture repair is quiet, sleepy, slow to answer, and eats little. The team suspects depression. Which assessment tool does the NICE delirium guideline recommend?**

**A.** PHQ-9

**B.** MoCA alone

**C.** MMSE alone

**D.** 4AT

**E.** Geriatric Depression Scale

Answer Contents

**BQ-507**

**A mechanically ventilated 60-year-old is agitated and needs sedation. Which approach does the 2018 PADIS guideline suggest?**

**A.** A continuous midazolam infusion as first-line sedation

**B.** Propofol or dexmedetomidine rather than a benzodiazepine

**C.** A haloperidol infusion to prevent delirium in all patients

**D.** Ketamine to prevent delirium in all patients

**E.** A statin to treat established delirium

Answer Contents

**BQ-508**

**A palliative care physician considers adding risperidone or haloperidol for a dying patient's delirium. Which finding best summarizes the randomized trial in palliative care by Agar and colleagues?**

**A.** Both drugs lowered symptom scores more than placebo did over three days

**B.** Haloperidol improved survival compared with placebo in the same trial

**C.** Both drugs gave worse delirium symptom scores than placebo, with more extrapyramidal effects

**D.** There was no difference between either drug and placebo on any measure

**E.** Risperidone halved the duration of delirium compared with placebo

Answer Contents

**BQ-509**

**A man with HIV takes a ritonavir-containing regimen and needs a medication for a psychotic disorder. A clinician proposes quetiapine. Which statement is consistent with the ritonavir label?**

**A.** No dose adjustment is required when the two drugs are combined

**B.** The quetiapine dose should be doubled to offset the interaction

**C.** Quetiapine is contraindicated with ritonavir under all circumstances

**D.** Triazolam is a safe adjunct for sleep during the combination

**E.** An alternative antiretroviral is suggested; if the combination is unavoidable, reduce quetiapine to one sixth of the dose

Answer Contents

**BQ-510**

**A 58-year-old woman taking tamoxifen for breast cancer develops major depression. In a population-based cohort of women on tamoxifen, which antidepressant was linked to higher breast cancer mortality as overlap increased?**

**A.** Paroxetine

**B.** Sertraline

**C.** Citalopram

**D.** Escitalopram

**E.** Fluvoxamine

Answer Contents

**BQ-511**

**A 66-year-old man with a creatinine clearance of 45 mL/min takes lithium. Five days after an NSAID and a diuretic were added, he develops tremor, vomiting, and confusion. What is the best first action?**

**A.** Give an extra lithium dose to treat the tremor

**B.** Continue lithium and recheck the level in two weeks

**C.** Add haloperidol for the confusion and continue lithium

**D.** Hold lithium, obtain a level, and assess hydration and kidney function

**E.** Start valproate in addition to the lithium

Answer Contents

**BQ-512**

**A 36-year-old woman has months of left leg weakness. Imaging is normal. On examination her weakness varies with distraction and she has a positive Hoover sign. Which approach is best?**

**A.** Tell her the symptoms are imaginary and discharge her without follow-up

**B.** Explain that the symptoms are real and functional, based on positive signs, and arrange coordinated follow-up and rehabilitation

**C.** Repeat imaging and electromyography every week until a cause appears

**D.** Start an antipsychotic to treat presumed psychosis

**E.** Admit her involuntarily for psychiatric evaluation

Answer Contents

**BQ-513**

**A 29-year-old woman with a mood episode is mute and immobile, holds a posture for minutes, and repeats the examiner's words. Which step helps confirm catatonia and predict response to treatment?**

**A.** An overnight dexamethasone suppression test

**B.** A haloperidol challenge with repeat rating

**C.** A lorazepam challenge of 1 to 2 mg intravenously with repeat rating after about 5 minutes

**D.** A single serum ceruloplasmin level

**E.** Observation without medication for one week

Answer Contents

**BQ-514**

**A man with catatonia has received lorazepam up to 16 mg a day, tolerated without side effects, with no remission. According to the British Association for Psychopharmacology guideline, what should come next?**

**A.** Add haloperidol

**B.** Continue the same lorazepam dose for another month

**C.** Switch to a stimulant

**D.** Discharge on oral benzodiazepine alone

**E.** Electroconvulsive therapy

Answer Contents

**BQ-515**

**A woman on sertraline is given ondansetron and tramadol after surgery. She develops agitation, sweating, tremor, inducible clonus, hyperreflexia, and a temperature of 38.6 degrees C. What is the best first step?**

**A.** Add haloperidol for the agitation

**B.** Stop the serotonergic drugs and give supportive care and a benzodiazepine

**C.** Apply physical restraints to keep the patient safe

**D.** Start bromocriptine to reduce the rigidity

**E.** Give dantrolene to reduce muscle activity

Answer Contents

**BQ-516**

**A 33-year-old man has taken clozapine for 3 weeks. He develops a temperature of 38.7 degrees C and muscle rigidity. His absolute neutrophil count is normal. According to the clozapine label, what is the best step?**

**A.** Interrupt clozapine and evaluate for neuroleptic malignant syndrome and other causes of fever, since the count is normal

**B.** Continue clozapine and recheck the neutrophil count in 4 weeks

**C.** Treat as neutropenic sepsis with empiric antibiotics alone

**D.** Add lithium for mood stabilization and continue clozapine

**E.** Increase the clozapine dose to treat the rigidity

Answer Contents

**BQ-517**

**A 71-year-old woman with heart failure on a diuretic receives an antiemetic and an antipsychotic. Her QTc rises from 440 to 505 msec, and her potassium is low. What is the best next step?**

**A.** Continue both drugs and repeat the ECG next week

**B.** Add a third QT-prolonging drug for nausea

**C.** Give calcium and discharge her the same day

**D.** Stop the QT-prolonging drugs, replace potassium, give magnesium, and monitor

**E.** Reduce the diuretic dose and leave the other drugs unchanged

Answer Contents

**BQ-518**

**A 55-year-old man takes a long-term opioid for back pain and lorazepam nightly. Psychiatry is consulted for anxiety. Which approach reflects the 2022 CDC opioid guideline?**

**A.** Stop the opioid abruptly to reduce the overdose risk

**B.** Increase the benzodiazepine dose for anxiety

**C.** Use particular caution with the combination, offer naloxone, and avoid abrupt opioid discontinuation

**D.** Replace the opioid with methadone for pain without further assessment

**E.** Withhold naloxone so as not to encourage opioid use

Answer Contents

SUBJECT 24

# Forensic Psychiatry, Ethics, and the Law

Questions BQ-519 to BQ-534

**BQ-519**

**A 44-year-old man with schizophrenia is brought to a state hearing for continued involuntary hospitalization. His attorney asks what the Constitution requires of the standard of proof. Which standard is the constitutional minimum?**

**A.** Preponderance of the evidence

**B.** Clear and convincing evidence

**C.** Probable cause

**D.** Beyond a reasonable doubt

**E.** Reasonable suspicion

Answer Contents

**BQ-520**

**A psychiatrist is interviewed on television about a presidential candidate whose speech patterns a host calls "obviously disordered." The psychiatrist has never met the candidate. Which response is most consistent with the APA Principles of Medical Ethics annotations?**

**A.** Offer a provisional diagnosis, labeled as unofficial, for the viewers

**B.** Describe the candidate's likely personality disorder in general terms

**C.** Predict how the candidate would behave under stress in office

**D.** Share general knowledge but decline to give an opinion about him

**E.** Give a diagnosis if the candidate's staff approves of the interview

Answer Contents

**BQ-521**

**A 28-year-old woman under a court commitment order for bipolar disorder declines her mood stabilizer. She is calm, sleeping, and cooperative, and she has made no threats. A colleague says the commitment order alone authorizes medicating her. What is the best response?**

**A.** Commitment does not by itself authorize medication over objection

**B.** Agree, because any committed patient can be medicated over objection

**C.** Agree, because refusal is itself proof of lack of capacity

**D.** Begin an injection and document the reasons afterward

**E.** Discharge her immediately because she refused treatment

Answer Contents

**BQ-522**

**Before starting a medication with a small risk of a serious adverse effect, a psychiatrist asks what must be disclosed. Under the approach in *Canterbury v. Spence*, which risk should be disclosed?**

**A.** Only risks that most physicians in the community routinely disclose

**B.** Only risks that occur in more than half of patients treated

**C.** Only risks that the patient specifically asks about

**D.** Only risks listed in the first paragraph of the label

**E.** Risks a reasonable patient would likely consider significant

Answer Contents

**BQ-523**

**An 81-year-old man with advanced dementia is unable to take part in decisions about starting a feeding tube. He never named a health care agent, and no written directive exists. His adult daughter has cared for him for years. Which approach is best?**

**A.** Ask the hospital administrator to decide on his behalf

**B.** Proceed because no one has objected to the tube

**C.** Ask a family member to decide as he would have chosen

**D.** Wait until he regains capacity before any decision is made

**E.** Defer to the physician′s preference alone in this case

Answer Contents

**BQ-524**

**A 36-year-old woman with schizoaffective disorder signed a psychiatric advance directive refusing a specific medication. She is now acutely psychotic and arrives in the emergency department unable to make decisions. Which statement is best?**

**A.** The directive has no meaning because she is psychotic now

**B.** It applies when she lacks capacity; emergencies may override

**C.** The directive can be ignored without any documentation

**D.** The directive takes effect only while she has capacity

**E.** The directive applies only to inpatients on a locked unit

Answer Contents

**BQ-525**

**A psychiatrist keeps notes of her own reflections on the content of therapy conversations in a file separate from the chart. Which item belongs in this separate file under the HIPAA definition of psychotherapy notes?**

**A.** Medication prescription and monitoring notes

**B.** Results of clinical tests and imaging studies

**C.** Summary of the treatment plan and progress

**D.** Session start and stop times and frequency

**E.** Analysis of the content of a counseling session

Answer Contents

**BQ-526**

**A 52-year-old man with depression has capacity and tells his psychiatrist he does not want his wife told anything about his care. His wife calls the clinic, worried. Which response is best?**

**A.** Listen, share nothing protected, and invite his agreement

**B.** Confirm his diagnosis because she is his legal spouse

**C.** Tell her his medication list so that she can help him

**D.** Refuse to accept any information she offers about him

**E.** Call her employer to verify her identity as his wife

Answer Contents

**BQ-527**

**A patient's attorney asks his treating psychiatrist to testify that he was legally insane at the time of an alleged offense. The psychiatrist has treated him for two years. What is the best response?**

**A.** Agree, since no one knows him better than his psychiatrist

**B.** Agree if the attorney pays a percentage of the settlement

**C.** Refuse to produce any records to the attorney at all

**D.** Give authorized facts; suggest an independent evaluator

**E.** Give the opinion but omit the treatment history from it

Answer Contents

**BQ-528**

**A 33-year-old man charged with a serious felony is found incompetent to stand trial and is not dangerous. The prosecution asks to medicate him over objection to restore competence. Under *Sell v. United States*, which finding is required?**

**A.** The defendant has refused medication during earlier admissions

**B.** The prosecution has filed a motion supported by a treating opinion

**C.** Medication will likely restore competence without harming his defense

**D.** The defendant has a history of nonadherence to prior treatment

**E.** The hospital has a bed available for a restoration program

Answer Contents

**BQ-529**

**A psychiatrist ended treatment with a patient eight months ago and asks a colleague whether a romantic relationship now would be acceptable. Which statement reflects the APA annotation?**

**A.** Sexual activity with a current or former patient is unethical

**B.** It is acceptable after any waiting period

**C.** It is acceptable if the patient initiates it

**D.** It is acceptable if therapy ended by mutual agreement

**E.** It is acceptable if the patient was only treated with medication

Answer Contents

**BQ-530**

**A defense attorney challenges a psychiatrist's testimony in a federal trial on the grounds that the opinion rests on an unreliable method. Under *Daubert* and Federal Rule of Evidence 702, who decides whether the testimony may be heard?**

**A.** The jury, without any preliminary screening by the court

**B.** The opposing expert, after reviewing the report

**C.** The treating clinician, who knows the defendant best

**D.** The state medical board, after a complaint

**E.** The judge, acting as gatekeeper for reliability and fit

Answer Contents

**BQ-531**

**A psychiatrist licensed in one state sees an established patient by video while the patient is temporarily visiting another state for several weeks. Which step is best?**

**A.** Assume the home license is valid anywhere the patient travels

**B.** Confirm where the patient is, and verify licensure there first

**C.** Rely on the home-state license because the stay is temporary

**D.** Switch to a phone call to avoid the licensure rules

**E.** Continue unchanged unless a controlled substance is involved

Answer Contents

**BQ-532**

**In November 2026, a DEA-registered psychiatrist plans to prescribe a Schedule IV medication by video to a new patient with no prior in-person visit. Under the temporary DEA and HHS rule published December 31, 2025, which statement is correct?**

**A.** Telemedicine prescribing of controlled substances is never permitted

**B.** A prior in-person visit is required in every case

**C.** Only Schedule I and II substances are covered by the rule

**D.** It is permitted through December 31, 2026 under listed conditions

**E.** It is permitted for practitioners who hold no DEA registration

Answer Contents

**BQ-533**

**A school counselor tells a psychiatrist that a 9-year-old patient has bruises and says her father hits her. The psychiatrist suspects abuse. The patient's mother asks that nothing be reported. What is the best action?**

**A.** Wait for the child to disclose more before acting

**B.** Honor the mother's request for confidentiality in this case

**C.** Report to child protection, since suspicion is the trigger

**D.** Discharge the family from care and document the reasons

**E.** Ask the father to explain the bruises before acting

Answer Contents

**BQ-534**

**A man on death row has vascular dementia and cannot remember the crime. He can state that the State plans to execute him as punishment for a killing but believes he is being punished for a different crime he committed in another city. Which statement best reflects *Panetti* and *Madison*?**

**A.** Memory loss alone bars execution under the Constitution

**B.** The test is rational understanding of the reason; dementia can count

**C.** Only psychosis can bar execution under the Constitution

**D.** Awareness of the State's stated reason always ends the inquiry

**E.** Execution competence is decided by the treating psychiatrist alone

Answer Contents

SUBJECT 25

# Professionalism, Communication, Cultural Care, and Systems of Practice

Questions BQ-535 to BQ-550

**BQ-535**

**A 45-year-old woman with bipolar disorder is starting a new mood stabilizer, and her dose schedule has changed from her previous medication. The psychiatrist has finished explaining the plan. Which next step best checks whether the explanation worked?**

**A.** Ask, "Do you understand everything I have explained?"

**B.** Ask her to restate the plan and show how she will take it

**C.** Give her a printed handout and ask her to sign that she received it

**D.** Ask her husband to repeat the instructions back to you

**E.** Repeat the full explanation more slowly, in the same words

Answer Contents

**BQ-536**

**A 33-year-old man with depression has heard two options from his psychiatrist, each with its benefits and drawbacks, using the shared decision-making approach recommended by AHRQ. He appears undecided. Which is the next step in that approach?**

**A.** Select the option with the stronger trial evidence and begin it

**B.** Schedule a follow-up visit to evaluate the decision later

**C.** Tell him that he must choose one of the options today

**D.** Ask what matters most to him, such as side effects and cost

**E.** Hand him a patient-decision aid and end the visit there

Answer Contents

**BQ-537**

**A 50-year-old man with alcohol use disorder says, "I know I drink too much, but work is brutal and a few drinks are the only thing that lets me sleep." Which response best fits motivational interviewing?**

**A.** "Drinking helps you cope with work, and you also worry about it."

**B.** "Alcohol makes sleep worse, so drinking is not really helping you."

**C.** "You need to stop drinking completely by the end of this month."

**D.** "Many people with your problem deny it, so I would not trust that."

**E.** "Why don't you simply look for a less stressful job somewhere else?"

Answer Contents

**BQ-538**

**A psychiatrist must tell a 24-year-old woman and her mother that her symptoms meet the diagnosis of schizophrenia. The psychiatrist has arranged a private room and silenced the phone. Using the SPIKES protocol, what is the next step?**

**A.** State the diagnosis and treatment plan at once, in full

**B.** Offer to refer to a specialist to deliver the news

**C.** Ask the mother to leave so the patient is alone

**D.** Describe the prognosis in detail before anything else

**E.** Ask the patient what she already understands about her illness

Answer Contents

**BQ-539**

**A busy clinic has a bilingual receptionist who says she speaks fluent Spanish. A new patient who speaks only Spanish arrives for a psychiatric intake. Under the HHS interpretation of the Section 1557 rule, which statement is most accurate?**

**A.** Her self-reported fluency is enough to qualify her as the interpreter

**B.** The patient may be asked to bring a friend who speaks English

**C.** She may interpret only if her qualification is established

**D.** The clinic may use a free web translation tool for the whole interview

**E.** The clinic need not provide language help for a first visit

Answer Contents

**BQ-540**

**A clinic wants to use measurement-based care for patients with depression. Which practice best reflects the approach?**

**A.** Review a validated scale with her each visit and adjust treatment

**B.** Administer a scale at intake only and file it in the chart

**C.** Rely on the clinician's impression and use scales only for research

**D.** Change the medication at every visit regardless of scores

**E.** Give the scale to the care manager and not discuss it with the patient

Answer Contents

**BQ-541**

**A 38-year-old man with schizophrenia has had four hospitalizations in two years, misses clinic appointments, and has lost his housing. He accepts help when staff come to him. Which service is best supported for this pattern by the 2020 APA schizophrenia guideline?**

**A.** Cognitive remediation alone

**B.** Weekly supportive psychotherapy in the office

**C.** Assertive community treatment

**D.** A single psychoeducation session

**E.** Referral to primary care collaborative care

Answer Contents

**BQ-542**

**A 19-year-old college student has had a first episode of psychosis for three months. Which program structure is best described as the evidence-supported model for early psychosis?**

**A.** Long-term inpatient care with limited contact after discharge to the community

**B.** Medication management alone in a general outpatient clinic, with visits as needed

**C.** Referral to a peer support group without any contact with a clinical team

**D.** Weekly individual therapy delivered by a single clinician in the office

**E.** A team-based coordinated specialty care program for early psychosis

Answer Contents

**BQ-543**

**A 29-year-old woman with schizophrenia lives with her mother, who is anxious and asks how to help. Which approach is most consistent with the APA 2020 guideline?**

**A.** Withhold information from the mother in all cases of illness

**B.** Offer psychoeducation and a family intervention, as contact is ongoing

**C.** Recommend family therapy only if relapse occurs later

**D.** Refer the mother to individual psychotherapy and exclude her from care

**E.** Offer no family involvement because trials show no effect

Answer Contents

**BQ-544**

**A patient asks how the national suicide and crisis line works and whether it costs money. Which answer is correct?**

**A.** It is a text-only service staffed by emergency dispatchers

**B.** It charges a fee based on the caller's insurance coverage

**C.** It routes all callers to a single national call center

**D.** It is free and confidential, by call, text, or chat, any hour

**E.** It works only for people who are already in treatment

Answer Contents

**BQ-545**

**During a video visit, a patient who is known to have depression appears flushed and slurs words. The clinician is in one state and the patient says she is traveling out of state. What is the best first step?**

**A.** Continue the visit because the clinician is licensed where the clinic is located

**B.** End the visit at once and bill for the time that was used

**C.** Prescribe a new medication and schedule a follow-up visit by video

**D.** Ask the patient to call back another day when she is rested

**E.** Confirm her location and a call-back number, then assess safety

Answer Contents

**BQ-546**

**A psychiatrist realizes that a lithium level result was delayed by a clinic error and a patient was not told to hold her dose. She has no symptoms yet. Which is the best action?**

**A.** Disclose promptly, apologize, report the error, and repeat the test

**B.** Wait to tell her until symptoms appear, so as not to cause needless worry

**C.** Document that a staff member made the error and say nothing to the patient

**D.** Quietly adjust the dose at the next visit without explaining the reason

**E.** Tell the patient about the delay only if she asks about her results

Answer Contents

**BQ-547**

**A psychiatrist chooses to select the visit level by time for a Medicare office visit. Which time counts?**

**A.** Time spent by the receptionist scheduling the visit and checking insurance

**B.** Only the minutes of direct face-to-face time with the patient

**C.** Time spent on the previous day reviewing the chart before the visit

**D.** The clinician's own qualifying time on the date of the visit

**E.** Time spent by a nurse taking vital signs and updating the medication list

Answer Contents

**BQ-548**

**The parents of a 16-year-old who recently disclosed that he is gay ask for therapy to change his sexual orientation. Which response is most consistent with APA policy?**

**A.** Agree to the request, since parents have the right to choose the treatment

**B.** Begin a short, time-limited course of reparative therapy as requested

**C.** Decline conversion therapy, explain why, and offer affirming psychotherapy

**D.** Refer the family to a religious counselor for the request without further comment

**E.** Refuse all contact with the family until they drop the request

Answer Contents

**BQ-549**

**A guideline panel reports "neither for nor against" a treatment that is widely used in practice. How should a clinician interpret this?**

**A.** The panel recommends against its use in clinical practice

**B.** The evidence was too thin to recommend either way

**C.** The treatment has been proven harmful in controlled trials

**D.** The treatment has been proven effective in controlled trials

**E.** Clinicians must stop using the treatment immediately

Answer Contents

**BQ-550**

**A patient with depression has been denied coverage for outpatient therapy visits by a group health plan that covers many medical visits without prior authorization. Which statement about the federal parity law is most accurate?**

**A.** It requires every plan to cover mental health care at no cost to the patient

**B.** It applies identically to Medicare and Medicaid as it does to group plans

**C.** It prohibits plans from imposing any limits on mental health services

**D.** It applies only to inpatient mental health care and not to outpatient visits

**E.** It bars stricter mental health limits but does not itself require coverage

Answer Contents

SUBJECT 1

# The Psychiatric Interview, Mental Status Examination, and Formulation

Answers BQ-001 to BQ-016

**BQ-001 · ANSWER B**

Recall of two words gives 2 points and an abnormal clock gives 0, a total of 2, which is in the positive screen range (0 to 2) for the Mini-Cog. A screen does not diagnose (C) and a higher score would not have excluded impairment (A). Delirium (D) is a separate syndrome of acute inattention and fluctuation, not defined by one abnormal clock.

Back to question Contents

**BQ-002 · ANSWER D**

In the original scoring, one point is added for 12 years of education or less when the total is below 30, so 25 becomes 26, which meets the original normal cutoff of 26. Subtracting (C) is wrong. Option A ignores the education rule, and the test does not require an interpreter to be scored (E).

Back to question Contents

**BQ-003 · ANSWER A**

Thought insertion is the experience that thoughts are placed in the mind from outside; it is a passivity experience and a disorder of thought content. Thought broadcasting (B) is the belief that thoughts are heard by others. Thought blocking (C) and flight of ideas (E) are disorders of process, and an obsession (D) is recognized as one's own.

Back to question Contents

**BQ-004 · ANSWER E**

Affect that does not fit the content of speech is incongruent. Blunted (A), flat (B), and constricted (D) describe reduced range or intensity, and labile (C) describes rapid shifts. Her smiling is not reduced, but it does not match the subject.

Back to question Contents

**BQ-005 · ANSWER C**

An illusion is a misperception of a real stimulus, here a coat read as a person, and he corrected it in better light. It is common in delirium. A hallucination (A) has no external stimulus, and a delusion of reference (B) is a fixed belief, which he abandoned immediately.

Back to question Contents

**BQ-006 · ANSWER B**

Hypnagogic experiences occur at sleep onset and are not psychotic by themselves. The absence of other symptoms, substance use, and a sleep problem makes a pathologic cause (A, D, E) unlikely, and command hallucinations (C) need content that orders action.

Back to question Contents

**BQ-007 · ANSWER E**

An obsession is an intrusive, unwanted thought the person recognizes as their own and resists; the checking is the compulsion. A delusion (A) is held with fixed conviction, and an overvalued idea (B) is held as true and dominates life without the resisted, intrusive quality. A phobia (C) is a fear of a specific object or situation.

Back to question Contents

**BQ-008 · ANSWER A**

Pressured speech and distractibility call for short, focused questions, gentle redirection, a low-stimulation setting, and collateral because insight is limited. Long open questions with silence (B) lose the thread. Arguing about sleep (C) and demanding agreement that he is ill (E) damage the alliance.

Back to question Contents

**BQ-009 · ANSWER D**

Holding a posture against gentle repositioning suggests catatonia, which can occur in mood, psychotic, and medical conditions. Examine for catatonic signs and look for a medical cause; do not assume the behavior is voluntary (B) or start a high-dose antipsychotic before the cause is known (A).

Back to question Contents

**BQ-010 · ANSWER C**

The AACAP practice parameter advises gathering information from several sources, including the child, parents, and school, and seeing parents and child both separately and together. Using only one informant (A, D) or one scale (B) misses the setting in which the behavior occurs.

Back to question Contents

**BQ-011 · ANSWER B**

Sensory deficits can mimic or worsen apparent confusion. Make sure aids are used, reduce noise, and repeat questions before interpreting the answers. Starting medication (A) or imaging (C) before a basic assessment is premature, and delirium (D) is not established by off-topic answers alone.

Back to question Contents

**BQ-012 · ANSWER E**

Listening does not require you to disclose; you may hear and record the information. What you disclose is governed by HIPAA and state law, and HHS notes that certain disclosures are permitted for health and safety. Ignoring her (C) discards important collateral, refusing to listen (A) misreads the rules, and sharing the whole chart (B) is not justified. A signed promise (D) does not replace assessment.

Back to question Contents

**BQ-013 · ANSWER A**

A trauma-informed interview gives choice, slows down, and does not require detailed trauma narratives at the first visit. Pressing on (B) or making care conditional (C) risks re-traumatization, and asking the partner to answer (E) removes her voice.

Back to question Contents

**BQ-014 · ANSWER D**

Section 3 of the CFI covers what the patient has done to cope and what sources of help have been used before, including medical, religious, and traditional options. Section 4 (C) covers current help-seeking, and section 2 (B) covers causes and supports. Option E is not a CFI section.

Back to question Contents

**BQ-015 · ANSWER C**

A formulation explains why this person has this problem now, using predisposing, precipitating, perpetuating, and protective factors, and guides what to do first. A second diagnosis (B) or list of symptoms (A) is not an explanation. Predicting a recovery date (D) is beyond what a formulation claims.

Back to question Contents

**BQ-016 · ANSWER E**

When aggressive ideas are present, the APA guideline recommends also assessing impulsivity, access to firearms, specific individuals or groups targeted, and family history of violent behavior. The demographic items (A to D) are not among the elements it names.

Back to question Contents

SUBJECT 2

# Diagnosis, Rating Scales, and the Medical Workup

Answers BQ-017 to BQ-038

**BQ-017 · ANSWER B**

WHO zone III covers scores of 16 to 19. Zone II is 8 to 15 (C) and zone IV is 20 to 40 (D).

Back to question Contents

**BQ-018 · ANSWER E**

When all opening questions are no, only the car question is asked and the interview stops. A score of 1 is below the positive threshold of 2, so A and D add steps the form does not call for, and B ignores the threshold.

Back to question Contents

**BQ-019 · ANSWER A**

The EPDS is a screening scale, not diagnostic, and any positive response on its self-harm item needs assessment. It does not distinguish depression from PTSD (C), and it does not replace the interview (D).

Back to question Contents

**BQ-020 · ANSWER D**

Initial research suggests PCL-5 cutoffs of 31 to 33, so 33 is within that range and calls for structured assessment, not a diagnosis (C). The range is 0 to 80 (B), and the measure is in the public domain (E).

Back to question Contents

**BQ-021 · ANSWER C**

The C-SSRS has no total score or universal risk cutoff, and organizations define their own response thresholds. A and D name cutoffs the scale does not define.

Back to question Contents

**BQ-022 · ANSWER A**

The education point applies only to people with 12 years of education or less, so none is added, and 25 or below is the impaired range in the original scoring. The result is a screen that calls for further evaluation. The publisher does not require certification when only the total score is used for screening (D), and no points are subtracted for advanced education (E).

Back to question Contents

**BQ-023 · ANSWER C**

The algorithm needs features 1 and 2 plus either disorganized thinking or an altered level of consciousness. He has features 1 and 2 only, so the screen is negative, and A, B, and D treat fewer features as sufficient.

Back to question Contents

**BQ-024 · ANSWER E**

Cut points of 5, 10, and 15 mark mild, moderate, and severe, and 10 or more is the usual threshold, so 12 is moderate and calls for an interview. The scale ranges from 0 to 21 (D), and a score alone does not establish the diagnosis (C).

Back to question Contents

**BQ-025 · ANSWER B**

The Task Force gave grade A to HIV screening for ages 15 to 65, and a first psychosis is a reason to test. A, C, and D tie testing to risk or timing that the recommendation does not require.

Back to question Contents

**BQ-026 · ANSWER D**

Neurologic involvement (here a cranial nerve palsy and cognitive-behavioral change) calls for a cerebrospinal fluid examination before treatment. Latent treatment (C) and delay (B) miss neurosyphilis.

Back to question Contents

**BQ-027 · ANSWER A**

Hyperthyroidism in older adults can appear as withdrawal and apathy that mimics depression or dementia, and weight loss with tremor supports it. TSH is the first test. Imaging (C) and EEG (D) are not first-line here.

Back to question Contents

**BQ-028 · ANSWER C**

The prednisone label warns of psychiatric derangements from euphoria and insomnia to psychosis, so the timing points to the drug. Contact the prescriber and manage symptoms safely. Diagnosing bipolar disorder (A) anchors on a label, and abrupt unsupervised stopping (D) is unsafe.

Back to question Contents

**BQ-029 · ANSWER E**

Montelukast carries a boxed warning for neuropsychiatric events, including aggression, sleep disturbance, and irritability, and the FDA advises weighing risks and benefits with the prescriber. Attributing it to age (A) misses a drug cause.

Back to question Contents

**BQ-030 · ANSWER B**

The label reports nonpsychotic behavioral symptoms in 38% versus 19% at ages 4 to 16. Statements that limit the effect to adults (A) or deny it (C, D) are wrong.

Back to question Contents

**BQ-031 · ANSWER D**

The thioridazine label states that patients with a QTc above 450 msec should not receive the drug. A QTc of 500 msec is the stop signal for other drugs (B), and CYP2D6 inhibitors are contraindicated (E).

Back to question Contents

**BQ-032 · ANSWER A**

The ziprasidone label lists recent acute myocardial infarction as a contraindication. Weekly ECGs (C) and other QT-prolonging drugs (D) do not change that.

Back to question Contents

**BQ-033 · ANSWER C**

Pseudoephedrine can cause a false-positive amphetamine immunoassay, and a disputed unexpected result should be confirmed by mass spectrometry. Acting on the screen (A, B) or repeating the same assay (D) does not resolve the question.

Back to question Contents

**BQ-034 · ANSWER B**

The meta-analysis found clinically relevant abnormalities in 5.9% (number needed to assess 18). The 26.4% figure (C) is any abnormality, including incidental findings.

Back to question Contents

**BQ-035 · ANSWER E**

Unexplained staring spells with fluctuating confusion raise nonconvulsive seizures, which EEG can detect. EEG does not diagnose depression, anxiety, or attention-deficit/hyperactivity disorder (A, B, C).

Back to question Contents

**BQ-036 · ANSWER D**

The pooled kappa was 0.67 for PTSD, 0.56 for bipolar I (E), 0.46 for schizophrenia (C), 0.28 for major depression (A), and 0.20 for generalized anxiety disorder at one site (B).

Back to question Contents

**BQ-037 · ANSWER A**

Attributing a new physical complaint to a known mental illness without examination is diagnostic overshadowing. Cross-sectional labeling (B) concerns judging from a single time point.

Back to question Contents

**BQ-038 · ANSWER C**

The task force advises dropping "medical clearance" in favor of a note stating what was assessed, the findings, and stability. Routine batteries (B) and universal drug screens (A) are not supported for every patient.

Back to question Contents

SUBJECT 3

# Neuroscience and Mechanisms of Disease

Answers BQ-039 to BQ-065

**BQ-039 · ANSWER B**

Reduced dopamine function in prefrontal pathways is suspected to contribute to negative and cognitive symptoms, and D2 blockers treat these symptoms poorly, though this is not proven. Option A is tempting, but mesolimbic blockade is the intended therapeutic action and is not proven to cause negative symptoms.

Back to question Contents

**BQ-040 · ANSWER D**

Acute dystonia is a motor effect of D2 blockade in the nigrostriatal pathway. Option C is tempting because it is also a D2 effect, but the tuberoinfundibular pathway causes raised prolactin, not muscle spasm.

Back to question Contents

**BQ-041 · ANSWER A**

NMDA antagonists produce symptoms that resemble schizophrenia, and the hypothesis proposes that reduced NMDA activity on inhibitory interneurons disinhibits glutamate neurons, especially in prefrontal cortex. It remains a hypothesis. Option B reverses the proposed direction, because the model involves reduced inhibition.

Back to question Contents

**BQ-042 · ANSWER E**

The clozapine label states the exact mechanism is not known and proposes D2 and 5-HT2A blockade. The fast-dissociation hypothesis, supported by lower and falling D2 occupancy, is one proposed explanation. Option B is the tempting error, because clozapine's D2 occupancy is relatively low or transient.

Back to question Contents

**BQ-043 · ANSWER C**

Xanomeline is a muscarinic agonist, thought to act at M1 and M4 receptors, and trospium is a muscarinic antagonist acting mainly in peripheral tissues. Option A is tempting because most antipsychotics block D2, but this combination does not rely on D2 blockade.

Back to question Contents

**BQ-044 · ANSWER D**

The label describes enhancement of GABA-mediated synaptic inhibition at GABA-A receptors, and the boxed warning covers concomitant opioid use with profound sedation and respiratory depression. Option A is wrong because alprazolam does not block D2 and the combination is not safe.

Back to question Contents

**BQ-045 · ANSWER A**

The label describes zuranolone as thought to be a positive allosteric modulator of GABA-A receptors, with the mechanism in depression not fully understood, and its boxed warning concerns impaired ability to drive or do hazardous tasks. Option B describes esketamine, which is the tempting confusion with another rapid-acting treatment.

Back to question Contents

**BQ-046 · ANSWER B**

The suprachiasmatic nucleus is the master clock, and light signaled by melanopsin-containing retinal cells synchronizes it. The pineal gland (A) releases melatonin under the clock's control but is not the clock.

Back to question Contents

**BQ-047 · ANSWER E**

The lithium label states that the mechanism by which it acts as a mood stabilizer is unknown. Option B is tempting because SSRIs share a mood indication, but lithium is not a reuptake inhibitor.

Back to question Contents

**BQ-048 · ANSWER C**

Transporter block begins at once while benefit takes weeks, which suggests downstream adaptation contributes. Option D is a tempting teaching error, because neurogenesis evidence in adult humans is contested and not proven as the cause.

Back to question Contents

**BQ-049 · ANSWER B**

The umbrella review examined whether lowered serotonin activity causes depression and found no consistent support. It did not test whether antidepressants work. Option A is the common misreading.

Back to question Contents

**BQ-050 · ANSWER A**

Tricyclics block muscarinic receptors, causing dry mouth, constipation, urinary retention, and confusion, particularly in older adults. Option B is tempting but D2 blockade causes motor and prolactin effects, not this cluster.

Back to question Contents

**BQ-051 · ANSWER D**

In the randomized trial there was no overall benefit; in an exploratory subgroup with higher CRP, response was 62 percent versus 33 percent, which was not statistically significant. Option B overstates the subgroup finding, and infliximab is not approved for depression.

Back to question Contents

**BQ-052 · ANSWER E**

A meta-analysis of 21 longitudinal studies found a pooled odds ratio of 2.10 (fixed effects) and 2.60 (random effects) for later depression. Option B is tempting but ignores the predictive, bidirectional relationship.

Back to question Contents

**BQ-053 · ANSWER C**

The label describes noncompetitive NMDA antagonism, states that it has not been shown to prevent suicide, and restricts administration to certified settings under a REMS. Option E is wrong because the antidepressant pathway has not been established.

Back to question Contents

**BQ-054 · ANSWER B**

The hypothesis is contested: mice lacking NMDA receptors in parvalbumin interneurons still responded to ketamine. Option A overstates the evidence, which is largely rodent.

Back to question Contents

**BQ-055 · ANSWER D**

Structural variation in C4 genes explains much of the major histocompatibility complex association, C4A expression was higher in schizophrenia brain tissue, and C4 helped eliminate synapses in mice. Option E is tempting but a mechanism involving complement does not prove an autoimmune disorder.

Back to question Contents

**BQ-056 · ANSWER A**

Genes at the loci are expressed in neurons and enriched for synaptic biology, and risk is polygenic. Option D is tempting but genome-wide results do not allow genetic diagnosis.

Back to question Contents

**BQ-057 · ANSWER C**

The cross-disorder analysis found 109 loci associated with at least two disorders, so risk overlaps across diagnostic categories. Option A is the traditional but unsupported view.

Back to question Contents

**BQ-058 · ANSWER E**

A preregistered analysis of 18 candidate genes in very large samples found no support for candidate genes or gene-by-environment interactions. Option A reflects early claims that did not replicate.

Back to question Contents

**BQ-059 · ANSWER B**

The meta-analysis found similar odds ratios across case-control (2.72), prospective (2.75), and cross-sectional (2.99) designs, with a population attributable risk of about 33 percent if causal. Option C is wrong because prospective studies did show an association.

Back to question Contents

**BQ-060 · ANSWER D**

The study compared postmortem tissue and showed lower glucocorticoid receptor expression and higher promoter methylation, paralleling rat work on maternal care, but association in postmortem tissue cannot establish causation. Option A overreaches.

Back to question Contents

**BQ-061 · ANSWER A**

The meta-analysis found peak onset of any mental disorder at 14.5 years, with 62.5 percent of cases beginning before age 25. Option B is the opposite of the finding.

Back to question Contents

**BQ-062 · ANSWER C**

A hippocampal effect size of 0.14 means the distributions overlap almost entirely, so a scan cannot confirm a diagnosis. Option A is the tempting misuse of group findings.

Back to question Contents

**BQ-063 · ANSWER E**

Gray matter loss converged in the dorsal anterior cingulate and both insulae, with few diagnosis-specific effects. Option A is the tempting traditional assumption that each disorder has a unique pattern.

Back to question Contents

**BQ-064 · ANSWER D**

One study found neurogenesis falling to undetectable levels in adults, and another found preserved neurogenesis from age 14 to 79, so the issue is unresolved. Option A and B each overstate one side.

Back to question Contents

**BQ-065 · ANSWER B**

Dopamine is viewed mainly as a learning signal that reinforces behavior, and drug-linked cues can trigger craving years after use stops. Option A reflects the older pleasure-chemical view, and option D confuses cue-triggered craving with prolonged withdrawal.

Back to question Contents

SUBJECT 4

# Principles of Psychopharmacology

Answers BQ-066 to BQ-087

**BQ-066 · ANSWER D**

NSAIDs raise lithium levels, and a concentration at or above 1.5 mEq/L is toxic. Hold lithium and evaluate for toxicity. The idea that NSAIDs induce clearance (A) is the opposite of the real effect.

Back to question Contents

**BQ-067 · ANSWER B**

The divalproex boxed warning describes major malformations and lower IQ, and the drug is not for women of childbearing potential unless alternatives have failed or are unacceptable. Folic acid is advised, but the label states it is not known to lower the risk in valproate-exposed pregnancies (A).

Back to question Contents

**BQ-068 · ANSWER E**

The label advises screening patients of at-risk Asian ancestry for HLA-B*15:02 before starting, and the FDA table says to avoid carbamazepine in carriers unless benefit outweighs risk. A level (A) does not address the skin-reaction risk, and delayed screening (C) misses the point.

Back to question Contents

**BQ-069 · ANSWER A**

With valproate the lamotrigine escalation starts at 25 mg every other day with a target of 100 mg/day, versus 200 without. Rash risk rises with valproate and with faster escalation. Valproate slows lamotrigine clearance, so C is backward.

Back to question Contents

**BQ-070 · ANSWER C**

Paroxetine lowers activation of tamoxifen, and the label suggests an antidepressant with little CYP2D6 inhibition. Fluoxetine is also a strong CYP2D6 inhibitor, so B does not solve the problem.

Back to question Contents

**BQ-071 · ANSWER D**

The aripiprazole label calls for half the usual dose with strong CYP2D6 inhibitors such as fluoxetine. A quarter dose (C) applies when both a strong CYP2D6 and a strong CYP3A4 inhibitor are present.

Back to question Contents

**BQ-072 · ANSWER A**

The quetiapine label calls for one sixth of the original dose with potent CYP3A4 inhibitors, and a rise back when the inhibitor stops. Doubling (B) applies to inducers, not inhibitors.

Back to question Contents

**BQ-073 · ANSWER B**

Lorazepam is conjugated by glucuronidation, but the label advises lower doses in severe hepatic insufficiency and warns it may worsen hepatic encephalopathy. It is not contraindicated in all liver disease (C).

Back to question Contents

**BQ-074 · ANSWER E**

The lorazepam boxed warning describes profound sedation, respiratory depression, coma, and death with opioids, and advises reserving the combination for patients without alternatives and limiting dose and duration. Switching to alprazolam (C) does not remove the warning, which applies to all benzodiazepines.

Back to question Contents

**BQ-075 · ANSWER C**

Per the label's Table 2, excess events per 1,000 were 14 more under 18 and 5 more at 18 to 24, with 1 fewer at 25 to 64 and 6 fewer at 65 and older. The warning is therefore age-graded, not uniform (A, D).

Back to question Contents

**BQ-076 · ANSWER A**

VA/DoD 2025 gives CBT-I its strong recommendation and suggests it before medication; if a drug is later needed, the zolpidem label starts older or debilitated patients at 5 mg and carries a boxed warning for complex sleep behaviors, a particular concern after a fall. Zolpidem 10 mg (B) exceeds the older-adult dose, and VA/DoD 2025 suggests against benzodiazepines (C) and diphenhydramine (D).

Back to question Contents

**BQ-077 · ANSWER D**

The boxed warning reports mortality of about 4.5 percent on drug versus 2.6 percent on placebo in dementia-related psychosis, so look for causes and use nonpharmacologic measures first, and consider an antipsychotic only if they fail or harm is substantial, after discussing the warning. Benzodiazepines (B) and sedating antihistamines (C) add sedation and fall risk without treating the cause.

Back to question Contents

**BQ-078 · ANSWER B**

The lorazepam label advises pausing, or returning to the previous dose, then reducing more slowly if withdrawal appears, and the FDA says no single schedule suits everyone. Doubling the rate (D) risks severe withdrawal.

Back to question Contents

**BQ-079 · ANSWER C**

Dizziness, nausea, irritability, and sensory symptoms within days of missed doses fit paroxetine discontinuation, which labels describe. Relapse builds over weeks (A), and serotonin syndrome would show clonus (B).

Back to question Contents

**BQ-080 · ANSWER E**

The fluoxetine label advises at least 5 weeks after stopping fluoxetine before an MAOI because fluoxetine and norfluoxetine persist. The 14-day interval (D) applies to most other antidepressants.

Back to question Contents

**BQ-081 · ANSWER A**

Esketamine is available only in certified settings with direct observation, never dispensed for home use, and sessions require observation for at least 2 hours. The REMS was not removed (E); only the clozapine REMS ended.

Back to question Contents

**BQ-082 · ANSWER D**

The FDA removed the clozapine REMS effective 13 June 2025, but prescribers should keep monitoring ANC at the label frequencies. Monitoring is therefore still needed (B), and pharmacies no longer verify ANC (C).

Back to question Contents

**BQ-083 · ANSWER B**

The label requires administration in a certified healthcare setting with at least 3 hours of observation, under a restricted distribution REMS, because of post-injection delirium and sedation syndrome. Shorter observation (A) does not meet the label.

Back to question Contents

**BQ-084 · ANSWER C**

APA 2020 suggests offering a long-acting injectable if the patient prefers it or has a history of poor or uncertain adherence. Meta-analysis shows fewer hospitalizations or relapses with long-acting formulations in randomized trials. Adding a second oral drug (B) lacks high-quality support.

Back to question Contents

**BQ-085 · ANSWER E**

The nortriptyline label notes that poor CYP2D6 metabolizers reach much higher exposure (up to an 8-fold higher AUC), and it asks for plasma level monitoring at higher doses, so avoid the drug or use a reduced dose with level monitoring. Paroxetine (D) would inhibit CYP2D6 further.

Back to question Contents

**BQ-086 · ANSWER A**

The ziprasidone label lists known QT prolongation, including congenital long QT syndrome, as a contraindication. Quetiapine's label does not list it, though its QT effect was not studied in a thorough QT study, and clozapine carries a QT warning rather than a contraindication.

Back to question Contents

**BQ-087 · ANSWER D**

TDM is useful when adherence is uncertain, and a level far below the expected value suggests missed doses or induction, such as continued smoking, which the clozapine label describes. A clozapine range is not given in the label, but doubling the dose without exploring adherence (A) risks toxicity if he suddenly takes it.

Back to question Contents

SUBJECT 5

# Antidepressants, Anxiolytics, and Hypnotics

Answers BQ-088 to BQ-112

**BQ-088 · ANSWER D**

The venlafaxine label asks you to monitor blood pressure before and during treatment, control hypertension, and consider dose reduction or discontinuation for sustained elevation. A ignores a treatable risk. C risks discontinuation symptoms, and E adds a hypertensive-crisis drug.

Back to question Contents

**BQ-089 · ANSWER A**

The duloxetine label says to avoid it in chronic liver disease or cirrhosis and warns of hepatotoxicity. The tempting alternatives call for lower doses in hepatic impairment (venlafaxine, escitalopram, bupropion) or caution (mirtazapine), but none is labeled for avoidance in cirrhosis.

Back to question Contents

**BQ-090 · ANSWER E**

Fluvoxamine is a potent CYP1A2 inhibitor, and tizanidine exposure rose about 33-fold in the label's data, so the combination is contraindicated. Sertraline, escitalopram, paroxetine, and fluoxetine are not contraindicated with tizanidine.

Back to question Contents

**BQ-091 · ANSWER B**

The paroxetine label notes that its CYP2D6 inhibition is linked to lower tamoxifen levels and advises an antidepressant with little or no CYP2D6 inhibition. Venlafaxine, mirtazapine, escitalopram, and vortioxetine are weak or non-inhibitors, with paroxetine the best-known inhibitor.

Back to question Contents

**BQ-092 · ANSWER C**

Mirtazapine carries a warning for rare agranulocytosis and severe neutropenia, which recovered after stopping in the premarketing cases. Fever with sore throat needs a count and a pause. A and D miss the signal, and B risks continued exposure.

Back to question Contents

**BQ-093 · ANSWER A**

The trazodone label says an erection lasting over 4 hours, painful or not, requires stopping the drug and emergency care. Waiting (C, D) risks permanent damage. B and E continue the drug.

Back to question Contents

**BQ-094 · ANSWER D**

Vilazodone absorption falls when taken fasting (about a 50 percent lower exposure), so it must be taken with food. A exceeds the usual labeled maximum of 40 mg, and the other options abandon an easily corrected problem.

Back to question Contents

**BQ-095 · ANSWER B**

Vortioxetine's dose is halved with strong CYP2D6 inhibitors such as bupropion, fluoxetine, and paroxetine. A has the direction wrong, and D is false: bupropion is not an MAOI.

Back to question Contents

**BQ-096 · ANSWER E**

Gepirone is contraindicated when the baseline QTc exceeds 450 msec. C is tempting because electrolytes matter, but correcting potassium does not remove the contraindication.

Back to question Contents

**BQ-097 · ANSWER C**

The zuranolone boxed warning advises against driving or hazardous activities for at least 12 hours after each dose during the 14-day course. A is wrong because it is taken in the evening with fat-containing food, and E is wrong because contraception is advised through 1 week after the last dose.

Back to question Contents

**BQ-098 · ANSWER B**

Esketamine is given in certified settings under a REMS with observation for at least 2 hours, and is never dispensed for home use. A and C contradict that, and E reverses the schedule, which is twice weekly at first.

Back to question Contents

**BQ-099 · ANSWER E**

Sodium bicarbonate to a serum pH of 7.45 to 7.55 with cardiac monitoring is the labeled approach for tricyclic toxicity with QRS widening. Do not induce vomiting; dialysis and forced diuresis are generally ineffective; flumazenil does not treat tricyclic toxicity.

Back to question Contents

**BQ-100 · ANSWER A**

The picture is a hypertensive crisis. The label advice is to stop the MAOI and lower the pressure immediately. D adds meperidine, which is contraindicated with MAOIs, and C and E leave a possibly life-threatening crisis untreated.

Back to question Contents

**BQ-101 · ANSWER D**

The tranylcypromine label says to stop at least 10 days before elective surgery, as does the phenelzine label. Shorter intervals (B, C, E) do not clear the drug's effect.

Back to question Contents

**BQ-102 · ANSWER C**

In VAST-D, 12-week remission was 28.9 percent with aripiprazole augmentation versus 22.3 percent with a switch to bupropion, with more somnolence, akathisia, and weight gain on aripiprazole. A reverses the finding, B and E are false, and D ignores the adverse-effect differences.

Back to question Contents

**BQ-103 · ANSWER E**

VA/DoD 2022 suggests against tricyclics, MAOIs, nefazodone, ketamine, and esketamine as initial therapy (weak against). Bupropion, mirtazapine, vortioxetine, and SNRIs are all among the options offered initially.

Back to question Contents

**BQ-104 · ANSWER A**

Rec 21 recommends continuing the effective antidepressant at the therapeutic dose for at least 6 months after remission (strong for). Stopping early (B, C) raises relapse risk, and D and E have no support.

Back to question Contents

**BQ-105 · ANSWER C**

For mild to moderate depression in pregnancy or breastfeeding, VA/DoD makes a strong recommendation for psychotherapy first. Paroxetine is the tempting wrong answer; its label advises weighing alternatives before first-trimester use.

Back to question Contents

**BQ-106 · ANSWER B**

The benzodiazepine boxed warning describes profound sedation, respiratory depression, coma, and death with opioids. Treat the anxiety with an SSRI or psychotherapy. C adds naloxone but not a safe indication, and E changes the drug but not the risk.

Back to question Contents

**BQ-107 · ANSWER D**

Buspirone does not block benzodiazepine withdrawal, and its effect is gradual, so lorazepam still needs a slow taper. A and B state the opposite, C swaps one sedative for another, and E is advised against in the 2025 guideline.

Back to question Contents

**BQ-108 · ANSWER E**

Hydroxyzine's label lists prolonged QT as a contraindication and reports torsades. Sertraline, venlafaxine, and mirtazapine are not contraindicated by label in prolonged QT, and buspirone has no QT contraindication.

Back to question Contents

**BQ-109 · ANSWER B**

VA/DoD 2023 suggests prazosin for PTSD-associated nightmares and against it for overall PTSD symptoms, and the label lists hypertension only. A is wrong, and C ignores the suggestion against it for broad PTSD symptoms.

Back to question Contents

**BQ-110 · ANSWER A**

Orexin receptor antagonists, including lemborexant, are contraindicated in narcolepsy. B and E ignore this, C and D address dosing, which is irrelevant to the contraindication.

Back to question Contents

**BQ-111 · ANSWER D**

Symptoms within days of stopping paroxetine suggest a discontinuation syndrome; paroxetine carries a higher withdrawal risk. Restarting the dose and tapering more slowly is the NICE-consistent approach. C overshoots, and A, B, and E do not address the cause.

Back to question Contents

**BQ-112 · ANSWER C**

Hyponatremia from SSRIs is more likely in older adults and diuretic users, and sodium below 110 mmol/L has been reported. Check the sodium first. B and D add sedatives, and E misattributes new symptoms in a patient still on the drug.

Back to question Contents

SUBJECT 6

# Antipsychotics, Mood Stabilizers, and Other Agents

Answers BQ-113 to BQ-137

**BQ-113 · ANSWER B**

Histamine H1 blockade causes sedation and weight gain. Alpha-1 blockade (option C) causes orthostatic hypotension and does not explain weight gain.

Back to question Contents

**BQ-114 · ANSWER D**

The quetiapine label advises reducing the dose to one-sixth of the original with strong CYP3A4 inhibitors such as ketoconazole, and restoring it after the inhibitor stops. Increasing the dose (option A) would risk toxicity. Option C understates the reduction.

Back to question Contents

**BQ-115 · ANSWER A**

Food increases ziprasidone absorption up to two-fold, so it must be taken with meals. Raising the dose (option B) is not appropriate because doses above 80 mg twice daily are generally not recommended.

Back to question Contents

**BQ-116 · ANSWER E**

Lurasidone is contraindicated with strong CYP3A4 inhibitors, and clarithromycin is one. Halving the dose (option B) applies to moderate inhibitors such as diltiazem, not strong ones. Choose another antibiotic.

Back to question Contents

**BQ-117 · ANSWER C**

Asenapine is placed under the tongue to dissolve, and eating or drinking is avoided for 10 minutes. Swallowing or chewing the tablet defeats sublingual absorption.

Back to question Contents

**BQ-118 · ANSWER A**

Because of the long half-lives of cariprazine and its active metabolites, plasma levels take weeks to reflect a dose change, so monitor for adverse reactions and response for several weeks. Option E is the opposite of the label.

Back to question Contents

**BQ-119 · ANSWER E**

Xanomeline-trospium is contraindicated in urinary retention and in gastric retention, among other conditions. Dose reduction (option A) does not remove this contraindication.

Back to question Contents

**BQ-120 · ANSWER B**

Antipsychotics carry a boxed warning for increased mortality in elderly patients with dementia-related psychosis, about 4.5% on drug versus 2.6% on placebo over about 10 weeks in the label's trials. Tardive dyskinesia (option A) develops with longer exposure.

Back to question Contents

**BQ-121 · ANSWER D**

The oral dose of 12 mg daily is above 10 mg, so the label's multiple is 10 to 20 times, giving 120 to 240 mg. The first injection should not exceed 100 mg, and the remainder is given 3 to 7 days later. A single 240 mg injection (option A) exceeds the first-injection maximum.

Back to question Contents

**BQ-122 · ANSWER C**

Monthly paliperidone palmitate is not recommended at a creatinine clearance below 50 mL/min. The reduced initiation doses (option B) are for mild impairment, 50 to under 80 mL/min.

Back to question Contents

**BQ-123 · ANSWER B**

The label calls for oral risperidone or another antipsychotic with the first injection and for 3 weeks, then stopping, because release from the microspheres begins after a lag.

Back to question Contents

**BQ-124 · ANSWER A**

The olanzapine label advises against concomitant parenteral benzodiazepines because of excessive sedation and cardiorespiratory depression, and says to assess orthostatic hypotension before repeat doses. Mixing lorazepam with olanzapine (options B and C) is also warned against.

Back to question Contents

**BQ-125 · ANSWER E**

Tobacco smoke induces CYP1A2, and stopping smoking removes the induction so clozapine levels rise, producing sedation, hypersalivation, and seizures. Neutropenia (option A) does not cause this picture.

Back to question Contents

**BQ-126 · ANSWER C**

Fever with tachycardia and chest discomfort early in clozapine treatment suggests myocarditis, which most often appears within the first 2 months. A normal ANC argues against neutropenia, so stop clozapine and evaluate for myocarditis, including troponin and C-reactive protein.

Back to question Contents

**BQ-127 · ANSWER D**

In benign ethnic neutropenia the label allows starting at an ANC of 1000 or higher, with at least two baseline values, and monitoring on the usual schedule. Option B is outdated because the REMS was removed in June 2025.

Back to question Contents

**BQ-128 · ANSWER A**

The FDA removed the REMS effective 13 June 2025, but still expects ANC monitoring at the label frequencies, and the boxed warning remains. Option C confuses removing paperwork with removing monitoring.

Back to question Contents

**BQ-129 · ANSWER C**

Hyperpyrexia, rigidity, altered mental status, autonomic instability, and elevated creatine kinase after an antipsychotic suggest neuroleptic malignant syndrome. Stop the antipsychotic and provide intensive supportive care. Benztropine (option A) does not treat it and continuing haloperidol is dangerous.

Back to question Contents

**BQ-130 · ANSWER E**

A level drawn 2 hours after a dose overstates the trough. The label specifies levels about 12 hours after the last dose, so repeat the level correctly before any change.

Back to question Contents

**BQ-131 · ANSWER B**

Polyuria and thirst on lithium suggest nephrogenic diabetes insipidus, which can develop within weeks, is usually reversible, and may be treated with amiloride. Hydrochlorothiazide (option A) is tempting but diuretics raise lithium levels.

Back to question Contents

**BQ-132 · ANSWER D**

Carbapenems can drop valproate to subtherapeutic levels, which explains the relapse and the level of 18 mcg/mL against a trial range of 50 to 125 mcg/mL. Poor adherence (option A) is not supported by the previous level of 85.

Back to question Contents

**BQ-133 · ANSWER A**

Estrogen-containing contraceptives lower lamotrigine concentrations substantially, and the label expects the maintenance dose may need up to doubling. Option B reverses the direction of the effect, and the combination is not contraindicated.

Back to question Contents

**BQ-134 · ANSWER C**

Carbamazepine is a strong CYP3A4 inducer, and the aripiprazole label advises doubling the dose over 1 to 2 weeks. Halving (option A) applies to strong inhibitors.

Back to question Contents

**BQ-135 · ANSWER E**

With strong CYP2D6 inhibitors such as paroxetine, the maximum deutetrabenazine dose is 36 mg daily. The usual maximum of 48 mg daily (option B) applies without such an inhibitor.

Back to question Contents

**BQ-136 · ANSWER B**

Atomoxetine carries a boxed warning for suicidal ideation in children and adolescents. Abuse and addiction (option A) is the stimulant boxed warning, and atomoxetine is not a controlled substance.

Back to question Contents

**BQ-137 · ANSWER D**

The lecanemab label calls for a recent baseline MRI, ApoE e4 genotype testing to inform the risk of amyloid-related imaging abnormalities, and confirmation of amyloid pathology. Lumbar puncture alone (option A) does not meet this work-up.

Back to question Contents

SUBJECT 7

# Psychotherapies

Answers BQ-138 to BQ-159

**BQ-138 · ANSWER B**

Behavioral activation monitors activity and mood and schedules small valued activities to restore reward. A describes cognitive restructuring, C a psychodynamic approach, D exposure, and E relapse prevention with MBCT.

Back to question Contents

**BQ-139 · ANSWER D**

Testing a feared prediction in real life and comparing the outcome is a behavioral experiment. A thought record is written work (A), chain analysis belongs to DBT (B), the interpersonal inventory belongs to IPT (C), and defusion belongs to ACT (E).

Back to question Contents

**BQ-140 · ANSWER A**

A bereaved patient with withdrawal after a loss fits the grief focus. Role transition (C) is tempting because widowhood is a change, but the central problem here is the loss itself. Role dispute and interpersonal deficits do not describe this picture, and social rhythm work (E) is for bipolar disorder.

Back to question Contents

**BQ-141 · ANSWER E**

Transference is the patient's displacement of feelings about an earlier figure onto the therapist. Countertransference (A) is the therapist's reaction to the patient. The other options are defense mechanisms or other concepts.

Back to question Contents

**BQ-142 · ANSWER C**

A recently relapsed, disorganized patient needs supportive therapy focused on stabilization, adherence, and practical problems. Deep interpretation can overwhelm him (A). Exposure to the relapse memory (B) and open-ended groups (D) are not indicated, and couples work (E) does not apply.

Back to question Contents

**BQ-143 · ANSWER B**

Exploring what matters (values) and identifying a small step with anxiety present is committed action in ACT. Thought stopping, desensitization, and restructuring (A, C, D) belong to other models, and distraction (E) is an avoidance strategy.

Back to question Contents

**BQ-144 · ANSWER A**

Contingency management depends on a clear link between verified behavior and its consequence. Resetting after a positive sample keeps the contingency tied to the behavior. The escalating schedule (D) is a separate feature.

Back to question Contents

**BQ-145 · ANSWER D**

Family psychoeducation and communication training reduce relapse in schizophrenia (a Cochrane review found a relapse risk ratio of 0.55). Cutting contact (A) removes support, and parent-only therapy or patient-only groups (C, E) lack the family component. Reducing medication without a plan (B) raises relapse risk.

Back to question Contents

**BQ-146 · ANSWER C**

The leader should intervene in the group process, restate ground rules, and meet individually with members at risk. Leaving the group alone (A) allows harm to continue. Removing the criticized member (B) punishes the target. Referring away (E) or pressuring the member to stay (D) does not address the process.

Back to question Contents

**BQ-147 · ANSWER E**

The VA/DoD guideline suggests that co-occurring substance use or other disorders should not preclude first-line trauma-focused therapy. Benzodiazepines are recommended against for PTSD (D). The guideline found insufficient evidence for specific group therapies (C).

Back to question Contents

**BQ-148 · ANSWER A**

For pregnancy or breastfeeding with mild to moderate depression, the VA/DoD guideline strongly recommends psychotherapy first. The guideline places psychotherapy first for mild to moderate depression in pregnancy, so an SSRI (B) is not the first-line approach, and ECT, ketamine, and watchful waiting (C, D, E) are not recommended first.

Back to question Contents

**BQ-149 · ANSWER B**

For adults aged 65 or older with mild to moderate depression, the VA/DoD guideline suggests psychotherapy as first-line treatment. The other options add risk or delay treatment.

Back to question Contents

**BQ-150 · ANSWER D**

A meta-analysis of 52 trials found combined treatment superior to medication alone in depression ($g = 0.41$) and anxiety disorders ($g = 0.47$). A, B, and E contradict that finding, and C is incorrect because depression was included.

Back to question Contents

**BQ-151 · ANSWER C**

NICE advises against medication specifically for borderline personality disorder and its symptoms, including long-term antipsychotics, and advises against brief interventions under 3 months unless the service is structured and supervised. Structured psychological therapy is the main treatment.

Back to question Contents

**BQ-152 · ANSWER E**

The overall effect was small and fell with masked assessors and shrank further when sources of bias were controlled, so benefit is modest and may be overestimated. CBT remains an adjunct to medication. A overstates the finding, B and C are unsupported, and D reverses the finding.

Back to question Contents

**BQ-153 · ANSWER A**

The VA/DoD PTSD guideline strongly recommends delivering first-line trauma-focused therapies by secure video when validated for that mode or when other options are unavailable. The other options contradict this.

Back to question Contents

**BQ-154 · ANSWER D**

Decline kindly, acknowledge the gratitude, explore its meaning, and document the exchange. Accepting an expensive gift risks exploiting the relationship or distorting the therapy (A, B, C). Ignoring the gift (E) misses the chance to address it.

Back to question Contents

**BQ-155 · ANSWER B**

A large meta-analysis found a consistent association between alliance and outcome across treatments, but the data are correlational, and the association was smaller in substance use samples. A and C are wrong. D is wrong because the effect was present in face-to-face therapy too. E is wrong because late alliance correlated more strongly with outcome than early alliance.

Back to question Contents

**BQ-156 · ANSWER E**

Coordinated care means prompt contact with consent, a risk assessment now, and clarity about who manages a crisis. Waiting (A) delays risk assessment. Changing medication alone (B), relaying messages through the patient (C), and withholding prescribing (D) are unsafe or unprofessional.

Back to question Contents

**BQ-157 · ANSWER C**

"I should probably cut back" is change talk, specifically desire or need language, mixed with sustain talk ("it's not easy"). The righting reflex (B) is the clinician's urge to fix. Commitment language of a completed action (E) would sound like "I cut back last week."

Back to question Contents

**BQ-158 · ANSWER A**

Reassurance feeds the cycle, so family members should respond with supportive, non-reassuring statements and help the patient follow the plan. NICE advises CBT with ERP involving family or carers for children and young people with moderate to severe impairment. B, C, D, and E worsen accommodation or harm the relationship.

Back to question Contents

**BQ-159 · ANSWER D**

Validation, grounding, and staying engaged at a manageable pace preserves the learning that exposure offers. Stopping and avoiding (A) reinforces avoidance. Benzodiazepines may dampen learning and are recommended against (B). Switching therapy at the first difficulty (C) is premature, and coercion (E) violates the alliance.

Back to question Contents

SUBJECT 8

# Neuromodulation and Interventional Psychiatry

Answers BQ-160 to BQ-175

**BQ-160 · ANSWER B**

The 2018 final order kept mania and the other psychotic-spectrum uses in class III, and FDA does not regulate how physicians use a cleared device, so off-label ECT is a clinical judgment. A is wrong because the class II use is catatonia or a severe major depressive episode. C and D misstate the regulation, and E confuses age with indication.

Back to question Contents

**BQ-161 · ANSWER D**

CANMAT 2016 lists right unilateral ultrabrief pulse, below 0.5 ms, as second line mainly to reduce short-term cognitive impairment, and bitemporal placement as the one with more cognitive effects. Bilateral ultrabrief (B) was the least effective arm in the randomized trial, with 35% remission, which makes it the tempting but poorer choice.

Back to question Contents

**BQ-162 · ANSWER A**

CANMAT advises switching to bilateral placement if right unilateral shows no response after 4 to 6 treatments. More than three treatments a week is not recommended (C), and a short trial does not justify declaring resistance (B).

Back to question Contents

**BQ-163 · ANSWER E**

The meta-analysis of 84 studies found deficits mainly in the first 3 days, recovery by 4 to 15 days, and no negative effects after 15 days, with processing speed, working memory, and anterograde memory above baseline. The findings concern objective tests, so subjective complaints still deserve attention, but A and C contradict the data.

Back to question Contents

**BQ-164 · ANSWER C**

CANMAT 2016 advises an antidepressant not tried before, nortriptyline plus lithium, or venlafaxine plus lithium, and supports maintenance ECT as about as effective as medication. Resuming a failed drug (B) or stopping (A) leaves the patient exposed to relapse, which the CORE trial showed exceeds 30% even with active continuation treatment.

Back to question Contents

**BQ-165 · ANSWER B**

In THREE-D, with 414 patients, intermittent theta burst was noninferior to 10 Hz stimulation, and the session time was about 3 minutes versus 37.5. The trial did not show superiority (A), and headache occurred in about two thirds of both groups (C).

Back to question Contents

**BQ-166 · ANSWER E**

A morning bright light box is low risk and weakly supported by VA/DoD, and a trial used 10,000 lux for 30 minutes. NICE calls the evidence for winter depression uncertain, so A overstates, and bipolar disorder and eye disease should be screened for.

Back to question Contents

**BQ-167 · ANSWER A**

The label tells clinicians to weigh risks and benefits when pressure is above about 140/90 and not to give the drug when a rise poses a serious risk, with a recheck about 40 minutes after dosing. Skipping checks (B) or shortening observation (D) departs from the REMS requirement of at least 2 hours.

Back to question Contents

**BQ-168 · ANSWER D**

Prior intracerebral hemorrhage is a labeled contraindication to esketamine, along with aneurysmal vascular disease and arteriovenous malformation. Dose or setting changes (A, B, C) do not remove the contraindication, and IV ketamine (E) raises blood pressure too.

Back to question Contents

**BQ-169 · ANSWER C**

For treatment-resistant depression the schedule is twice weekly in weeks 1 to 4, once weekly in weeks 5 to 8, then every 2 weeks or weekly. Esketamine is never dispensed for home use (E).

Back to question Contents

**BQ-170 · ANSWER A**

Both documents describe 0.5 mg/kg over 40 minutes with cardiovascular and respiratory monitoring and an escort home. Compounded and at-home products (B, D) are discouraged, and maintenance should be decided case by case rather than indefinitely (E).

Back to question Contents

**BQ-171 · ANSWER B**

FDA stated on October 10, 2023 that ketamine is not approved for any psychiatric disorder, that compounded products have not been evaluated, and that they lack the REMS. Option E is wrong because variable compounded doses make risk harder to predict.

Back to question Contents

**BQ-172 · ANSWER E**

FDA approved VNS on July 15, 2005 as adjunctive long-term treatment for chronic or recurrent depression in adults 18 and older in a current episode after four or more failed antidepressant treatments. Medicare covers it only within a CMS-approved study, and VA/DoD weakly advises against it outside research.

Back to question Contents

**BQ-173 · ANSWER D**

FDA granted the humanitarian device exemption on February 10, 2009 for bilateral stimulation of the anterior limb of the internal capsule as an adjunct in adults with chronic, severe, treatment-resistant OCD who failed at least three SSRIs. DBS for depression remains investigational, and VA/DoD strongly recommends against it outside research.

Back to question Contents

**BQ-174 · ANSWER C**

On December 8, 2025 FDA approved the home-use headset as a class III device for adults with moderate to severe depression not considered treatment refractory, as monotherapy or adjunct. VA/DoD 2022 does not address tDCS, so D is unsupported.

Back to question Contents

**BQ-175 · ANSWER A**

Psilocybin is not an FDA-approved product, and VA/DoD strongly recommends against psilocybin, MDMA, and other unapproved drugs for depression outside research. Counsel her on approved options and trials, and ask about safety. Option C is wrong: psilocybin has no approval for PTSD or any other indication.

Back to question Contents

SUBJECT 9

# Depressive Disorders

Answers BQ-176 to BQ-202

**BQ-176 · ANSWER D**

The guideline suggests using a quantitative severity measure at initial treatment planning and at regular intervals to guide shared decisions (weak for). Clinician impression alone (A) is the tempting habit, but it misses partial response and residual symptoms. Baseline-only measurement (B) cannot show change.

Back to question Contents

**BQ-177 · ANSWER A**

VA/DoD gives a strong recommendation for collaborative or integrated care models for major depression treated in primary care. Antidepressant monotherapy in every case (B) is tempting but ignores preference-based psychotherapy, and referring every patient to psychiatry (C) is not what the guideline recommends.

Back to question Contents

**BQ-178 · ANSWER C**

Depressed or irritable mood lasting 1 year or more in a youth, without a break and without prominent outbursts, fits persistent depressive disorder. Single-episode major depression (A) is the tempting alternative but implies a discrete episode, and disruptive mood dysregulation disorder (D) requires frequent severe outbursts, which are not prominent here.

Back to question Contents

**BQ-179 · ANSWER E**

The label directs that bupropion XL be started in autumn before depressive symptoms begin, continued through winter, and tapered in early spring. Starting in March (A) misses the high-risk season. Starting once symptoms begin (B) is treatment, not the prevention the label describes.

Back to question Contents

**BQ-180 · ANSWER B**

Pervasive anhedonia without mood reactivity, early waking, morning worsening, marked slowing, weight loss, and guilt define melancholic features. Atypical features (A) is the mirror image, with mood reactivity, oversleeping, and overeating.

Back to question Contents

**BQ-181 · ANSWER D**

Grief produces waves of longing with preserved self-worth. Support and monitoring are appropriate, and treatment for major depression is added if a full depressive syndrome emerges. Starting an antidepressant at once (A) is tempting but treats a normal bereavement response as an illness.

Back to question Contents

**BQ-182 · ANSWER A**

A depressive episode with some manic symptoms that fall short of mania carries the mixed features specifier and raises concern for bipolar disorder. Atypical features (B) do not confirm unipolar illness, and anxious distress (D) does not rule out bipolar disorder.

Back to question Contents

**BQ-183 · ANSWER B**

Fatigue, weight gain, constipation, and cold intolerance suggest hypothyroidism, and thyroid-stimulating hormone is the sensible first test. Brain MRI (A) is not routine and is reserved for focal signs or cognitive change, and polysomnography (D) is for suspected sleep apnea.

Back to question Contents

**BQ-184 · ANSWER E**

The label allows luteal-phase sertraline starting 14 days before expected menses and continuing through menses onset, at up to 100 mg daily. The 150 mg ceiling (A) applies to continuous dosing, not luteal-phase dosing.

Back to question Contents

**BQ-185 · ANSWER C**

The review reports that a drospirenone and ethinyl estradiol regimen taken 24 days active and 4 days placebo reduced symptoms versus placebo, whereas standard 21-day regimens have not shown benefit over placebo. The tempting claim in A conflicts with that finding. GnRH agonists (B) are not contraceptives and are reserved for severe, refractory cases.

Back to question Contents

**BQ-186 · ANSWER B**

The diagnosis is not made for the first time before age 6, so this child needs another formulation. Duration and frequency (A) are tempting, but age is an exclusion. The mood disorders in C to E are less fitting without episodic mood change or persistent depressive symptoms.

Back to question Contents

**BQ-187 · ANSWER D**

The label's age-stratified data show 5 more suicidal events per 1,000 patients than placebo at ages 18 to 24. The 14 more per 1,000 figure (E) applies to patients under 18, and the figures of 1 fewer and 6 fewer apply to ages 25 to 64 and 65 and older.

Back to question Contents

**BQ-188 · ANSWER A**

In mirtazapine trials, somnolence occurred in 54% versus 18% on placebo, and weight gain of 7% or more in 7.5% versus 0%. Ejaculation failure (B) is characteristic of SSRIs such as sertraline, and sustained hypertension (C) is a venlafaxine concern.

Back to question Contents

**BQ-189 · ANSWER C**

Bupropion is contraindicated in current or prior bulimia or anorexia nervosa because of seizure risk. The other drugs have no such contraindication. Mirtazapine (D) is a tempting choice for appetite, but it is not contraindicated here.

Back to question Contents

**BQ-190 · ANSWER E**

The label advises measuring blood pressure before starting and regularly during treatment, controlling pre-existing hypertension first, and considering a dose reduction or discontinuation for sustained increases. A baseline-only check (D) misses dose-related rises.

Back to question Contents

**BQ-191 · ANSWER B**

This presentation fits serotonin syndrome from combining a serotonergic drug with linezolid, an MAOI. The label directs stopping the drug and any serotonergic agents and starting supportive treatment. Raising the dose (C) worsens the syndrome, and dantrolene alone (D) does not address the cause.

Back to question Contents

**BQ-192 · ANSWER D**

The fluoxetine label advises at least 5 weeks after stopping fluoxetine before starting an MAOI, because of the long half-lives of fluoxetine and norfluoxetine. The 14-day gap (C) is the general rule for most other antidepressants and is the tempting but unsafe answer.

Back to question Contents

**BQ-193 · ANSWER A**

Akathisia is the characteristic early adverse effect of adjunctive brexpiprazole, reported in 9% versus 2% on placebo, and more often at higher doses. Serotonin syndrome (B) has clonus, fever, and autonomic signs, and tardive dyskinesia (C) develops with longer exposure.

Back to question Contents

**BQ-194 · ANSWER C**

The quetiapine XR label recommends fasting glucose and lipid testing at the beginning of treatment and periodically, and regular weight monitoring. Skipping tests (A) ignores the metabolic warnings.

Back to question Contents

**BQ-195 · ANSWER E**

NICE advises checking weight, renal and thyroid function, and calcium before lithium augmentation. Checking liver enzymes alone (A) misses the renal and thyroid effects of lithium, and a head CT (C) has no role.

Back to question Contents

**BQ-196 · ANSWER B**

VA/DoD suggests offering rTMS after partial or no response to two or more adequate drug trials (weak for). It recommends against deep brain stimulation outside research (A), suggests against vagus nerve stimulation outside research (C), recommends against psilocybin outside research (D), and suggests against omega-3 fatty acids as treatment (E).

Back to question Contents

**BQ-197 · ANSWER D**

Esketamine is available only through a REMS and is dispensed and administered only in certified healthcare settings, with direct observation. Each session needs observation of at least 2 hours. A reliable patient (A, C, E) does not change the requirement.

Back to question Contents

**BQ-198 · ANSWER A**

Remission rates in STAR*D were 36.8% at step 1, 30.6% at step 2, 13.7% at step 3, and 13.0% at step 4, so each step yielded fewer remitters. The rising pattern in B is the opposite of what was reported.

Back to question Contents

**BQ-199 · ANSWER C**

VA/DoD strongly recommends continuing the antidepressant at a therapeutic dose for at least 6 months after remission to reduce relapse. Stopping now (A) is tempting because she is well, but remission is not recovery. The herbal and omega-3 options (D, E) have weak or negative support.

Back to question Contents

**BQ-200 · ANSWER B**

VA/DoD weakly suggests a course of CBT, interpersonal therapy, or mindfulness-based cognitive therapy in the continuation phase for patients at high relapse risk, such as those with two or more prior episodes. It suggests against vitamin D and omega-3 fatty acids as treatment (A, C).

Back to question Contents

**BQ-201 · ANSWER E**

For pregnant or breastfeeding patients with mild to moderate major depression, VA/DoD strongly recommends evidence-based psychotherapy as first-line treatment. Starting sertraline at once (A) is tempting, but medication is weighed against risk. Esketamine (B) and ECT (C) are not first-line.

Back to question Contents

**BQ-202 · ANSWER D**

VA/DoD weakly suggests standardized St. John's wort extract as monotherapy for mild depression in patients who prefer an herb and are not pregnant or breastfeeding. NICE advises against prescribing it because of dose uncertainty and serious drug interactions. The two guidelines therefore differ.

Back to question Contents

SUBJECT 10

# Bipolar and Related Disorders

Answers BQ-203 to BQ-224

**BQ-203 · ANSWER C**

Ten days of elevated mood with reduced sleep, rapid speech, and increased activity, with grandiose delusions and a mood-congruent hallucination that required hospital care, is a manic episode with psychotic features, so this is bipolar I disorder. Brief psychotic disorder (A) and schizophreniform disorder (D) do not account for the full mood syndrome that preceded and accompanied the psychosis.

Back to question Contents

**BQ-204 · ANSWER A**

Cyclothymic disorder involves two years or more of many hypomanic-like and depressive periods that never reach full episodes, present at least half the time, with no symptom-free stretch longer than two months. Persistent depressive disorder (D) lacks the elevated periods.

Back to question Contents

**BQ-205 · ANSWER D**

Mood elevation that begins days after a high-dose corticosteroid is a substance/medication-induced syndrome until proven otherwise. Bipolar I disorder (A) would be considered only if the syndrome persisted beyond the drug's expected effect or if episodes occurred apart from it.

Back to question Contents

**BQ-206 · ANSWER B**

A full manic syndrome that emerges during antidepressant treatment and persists well beyond the drug's expected physiological effect is evidence for bipolar disorder, and a manic episode makes it bipolar I. A drug-induced diagnosis (A) would resolve once the effect had worn off.

Back to question Contents

**BQ-207 · ANSWER E**

This is a depressive episode with prominent manic symptoms, which is mixed features with high suicide risk. The ISBD advises avoiding antidepressants with current mixed features, so an antipsychotic with bipolar evidence and a safety assessment are needed. Raising the antidepressant (A) or adding a stimulant (C) can worsen it.

Back to question Contents

**BQ-208 · ANSWER C**

Five episodes in a year is rapid cycling, and fatigue, cold intolerance, and weight gain suggest hypothyroidism, which lithium can cause. The label asks for thyroid monitoring before treatment, at 3 months, and every 6 to 12 months. Ammonia (A) relates to valproate.

Back to question Contents

**BQ-209 · ANSWER B**

Psychosis that persists for weeks after mood symptoms resolve points to schizoaffective disorder. In bipolar I disorder with psychotic features, psychosis occurs only during mood episodes. Brief psychotic disorder (C) is too short and would not follow a manic episode.

Back to question Contents

**BQ-210 · ANSWER A**

A lifelong, persistent pattern from childhood with no distinct episodes, no elevated mood, and a normal need for sleep favors ADHD. Mania is episodic and reduces the need for sleep. Hyperthyroid mania (E) would show systemic signs and a new onset.

Back to question Contents

**BQ-211 · ANSWER D**

Before lithium, check kidney function, electrolytes, thyroid function, medications, vital signs, and pregnancy status. Liver tests, platelets, and ammonia (A, C) are valproate concerns, and HLA-B*1502 (B) relates to carbamazepine.

Back to question Contents

**BQ-212 · ANSWER E**

The label asks for the first level after 3 days, drawn 12 hours after the last oral dose, then 12-hour troughs until stable. A level drawn sooner than 12 hours after the dose is not a trough and reads falsely high.

Back to question Contents

**BQ-213 · ANSWER B**

Lithium is dangerous in overdose, with toxic levels from 1.5 mEq/L and seizures and coma above 3 mEq/L, so limit the supply while keeping the mood benefit and other safety steps. The VA/DoD suicide guideline makes no recommendation for or against lithium to reduce suicide risk, so a lithium prescription does not replace reassessment (E).

Back to question Contents

**BQ-214 · ANSWER C**

With a level of 3.4 mEq/L, seizure, and kidney impairment, hemodialysis is the treatment of choice for severe toxicity according to the label. The label states that no specific antidote exists (B) and that activated charcoal is not recommended (A). Diuretics raise lithium levels (D).

Back to question Contents

**BQ-215 · ANSWER D**

Chronic lithium can reduce renal concentrating ability and cause nephrogenic diabetes insipidus, with polyuria, thirst, and dilute urine. Psychogenic polydipsia (B) is a diagnosis of exclusion after years of lithium exposure, and diabetes mellitus (C) would produce glycosuria.

Back to question Contents

**BQ-216 · ANSWER A**

Valproate can cause hyperammonemia with encephalopathy even when liver tests are normal, so measure ammonia for unexplained lethargy or vomiting. A lithium level (B) is irrelevant to a patient who does not take lithium.

Back to question Contents

**BQ-217 · ANSWER E**

Valproate carries a boxed warning for life-threatening pancreatitis that can occur early or after years of use. Stop the drug and evaluate. Continuing at a lower dose (A) or checking a level (D) delays recognition of a potentially fatal complication.

Back to question Contents

**BQ-218 · ANSWER B**

Rash with fever and mucosal lesions in the first weeks of lamotrigine suggests a serious reaction such as Stevens-Johnson syndrome, and nearly all life-threatening cases occur within 2 to 8 weeks. Stop the drug and evaluate urgently. Antihistamines and a slower titration (A, C) are not safe responses.

Back to question Contents

**BQ-219 · ANSWER D**

Estrogen-containing contraceptives lower lamotrigine levels, so the maintenance dose usually needs adjusting when they are started or stopped. The concern is a fall in lamotrigine levels with loss of mood control, not toxicity (A).

Back to question Contents

**BQ-220 · ANSWER C**

The label advises HLA-B*1502 screening in patients of at-risk Asian ancestry before starting carbamazepine because of the risk of serious skin reactions. A blood count is also needed. TPMT (B) relates to thiopurines, not carbamazepine.

Back to question Contents

**BQ-221 · ANSWER A**

The lurasidone label requires administration with at least 350 calories, and grapefruit should be avoided. Taking it fasting reduces absorption. Grapefruit juice (B) interacts through CYP3A4 and should be avoided.

Back to question Contents

**BQ-222 · ANSWER D**

VA/DoD weakly suggests continuing lithium at the lowest effective dose in people who are stabilized and become pregnant, with psychoeducation and shared decision-making. Abrupt discontinuation (A) risks relapse, and valproate and carbamazepine (B, E) are strongly advised against in people of childbearing potential.

Back to question Contents

**BQ-223 · ANSWER E**

CANMAT/ISBD lists lurasidone alone as first-line for pediatric bipolar depression, and the lurasidone label covers ages 10 to 17. Antidepressants alone (A, B) are not recommended, and the antidepressant boxed warning applies to the young.

Back to question Contents

**BQ-224 · ANSWER B**

Renin-angiotensin system antagonists raise lithium concentrations, so check a level and kidney function soon after starting and monitor more often. Preemptive halving (C) is arbitrary, and a diuretic (E) would raise the level further.

Back to question Contents

SUBJECT 11

# Anxiety and Obsessive-Compulsive Disorders

Answers BQ-225 to BQ-251

**BQ-225 · ANSWER C**

Duloxetine is labeled for GAD in adults and in children 7 and older (brand escitalopram also carries a pediatric GAD indication from age 7, but it is not offered here). Paroxetine and venlafaxine extended-release are not approved in pediatric patients, sertraline is labeled for pediatric OCD only, and buspirone has no established pediatric safety or effectiveness.

Back to question Contents

**BQ-226 · ANSWER C**

NICE prefers individual CBT developed for social anxiety disorder, not group CBT by default. Alprazolam and daily propranolol are not recommended for generalized social anxiety.

Back to question Contents

**BQ-227 · ANSWER E**

NICE suggests imipramine or clomipramine (off-label) when an SSRI is unsuitable or there is no improvement after a 12-week course. NICE advises against benzodiazepines in panic disorder, which is why A is the tempting wrong answer.

Back to question Contents

**BQ-228 · ANSWER B**

Exposure-based treatment is effective for specific phobia, and in vivo exposure is the usual format. A sedative before each exposure is tempting but blunts learning and keeps the fear intact.

Back to question Contents

**BQ-229 · ANSWER D**

Selective mutism is an anxiety disorder treated first with graded behavioral work coordinated with the school. Punishing silence (B) deepens avoidance, and medication is reserved for moderate to severe cases.

Back to question Contents

**BQ-230 · ANSWER C**

Label Table 2 shows excess events under 18 and at ages 18 to 24, one fewer at 25 to 64, and six fewer at 65 and older, per 1,000 patients. A and B reverse or flatten the pattern.

Back to question Contents

**BQ-231 · ANSWER A**

Obsessions with compulsions and no other psychotic features fit OCD even when insight is poor. Delusional disorder (C), the tempting alternative, lacks the neutralizing ritual structure.

Back to question Contents

**BQ-232 · ANSWER D**

Perfectionism that feels right to the person, without intrusive obsessions or harm-preventing rituals, indicates obsessive-compulsive personality disorder rather than OCD. Hoarding disorder (E) involves difficulty discarding possessions.

Back to question Contents

**BQ-233 · ANSWER E**

The device is approved for adults with chronic, severe, treatment-resistant OCD who have failed at least three SSRIs. Two failures (B) and adolescence (C) fall outside the labeling.

Back to question Contents

**BQ-234 · ANSWER B**

Deep TMS is contraindicated with metal or implanted devices in or near the head, including cochlear implants. It is authorized as an adjunct to stable treatment, so a stable SSRI dose (E) is expected, not prohibitive.

Back to question Contents

**BQ-235 · ANSWER E**

The label reports cumulative seizure incidence at doses up to 300 mg daily of 0.64 percent at 90 days, 1.12 percent at 180 days, and 1.45 percent at 365 days. Risk accumulates with time and dose, so A, C, and D are wrong.

Back to question Contents

**BQ-236 · ANSWER D**

Tizanidine is contraindicated with fluvoxamine because of CYP1A2 inhibition. The other options are not listed contraindications on the fluvoxamine label.

Back to question Contents

**BQ-237 · ANSWER B**

The fluoxetine label requires at least 5 weeks after stopping before an MAOI used for psychiatric disorders. Fourteen days (D) is the interval after most other SSRIs, the tempting wrong answer.

Back to question Contents

**BQ-238 · ANSWER A**

In POTS, remission was 53.6 percent with combined treatment, 39.3 with CBT, 21.4 with sertraline, and 3.6 with placebo; combined treatment beat CBT alone, sertraline alone, and placebo, and each monotherapy beat placebo without differing from the other.

Back to question Contents

**BQ-239 · ANSWER C**

At week 12 all active arms beat placebo; ERP alone did not differ from the combination, and both outperformed clomipramine alone. Option D reverses the finding.

Back to question Contents

**BQ-240 · ANSWER B**

In a meta-analysis of 14 placebo-controlled trials, aripiprazole, haloperidol, and risperidone each showed significant benefit over placebo. Benzodiazepines and the others lack that support.

Back to question Contents

**BQ-241 · ANSWER E**

Preoccupation with a nonexistent or slight defect plus repetitive checking and camouflage is body dysmorphic disorder; conviction at a delusional level is still diagnosed as BDD. Delusional disorder (A) is the tempting alternative.

Back to question Contents

**BQ-242 · ANSWER C**

Habit reversal training is the usual first step for trichotillomania; drug evidence is limited and conflicting. N-acetylcysteine has trial support but is not an approved treatment.

Back to question Contents

**BQ-243 · ANSWER D**

Severe anxiety that began with cannabis and is absent during abstinence points to substance-induced anxiety rather than an independent panic or generalized disorder. Persistence long after the substance clears would suggest an independent disorder.

Back to question Contents

**BQ-244 · ANSWER A**

Akathisia after starting an antipsychotic can mimic anxiety and needs a dose review, not more sedation or an antidepressant. Raising the dose (D) is the tempting error.

Back to question Contents

**BQ-245 · ANSWER D**

The hydroxyzine label contraindicates use with a prolonged QT interval. Buspirone, sertraline, duloxetine, and pregabalin are not contraindicated on this basis.

Back to question Contents

**BQ-246 · ANSWER B**

The paroxetine label describes less than a 2-fold rise in cardiovascular malformations, with about 2 to 2.5-fold for right ventricular outflow tract defects, against a 2 to 4 percent background risk. Stopping without a plan (C) risks relapse.

Back to question Contents

**BQ-247 · ANSWER A**

FDA required the class boxed warning to add abuse, misuse, addiction, physical dependence, and withdrawal reactions. The other options were not part of the update.

Back to question Contents

**BQ-248 · ANSWER C**

Buspirone has no cross-tolerance with benzodiazepines and does not block their withdrawal syndrome. Abrupt cessation risks seizures, and flumazenil is to be avoided.

Back to question Contents

**BQ-249 · ANSWER E**

Venlafaxine can raise blood pressure; sustained hypertension occurred in 0.5 to 3 percent of premarketing patients depending on indication. The other tests are used for lithium, clozapine, antipsychotics, and valproate.

Back to question Contents

**BQ-250 · ANSWER A**

NICE advises seeing people under 30 within a week of starting an SSRI or SNRI and monitoring suicidal thinking weekly for the first month. Later reviews follow every 2 to 4 weeks for 3 months.

Back to question Contents

**BQ-251 · ANSWER D**

For children with moderate to severe impairment, NICE recommends family-involved CBT with ERP adapted to developmental age; an SSRI is added only after a poor response, and clomipramine later. Sertraline alone (A) is the tempting shortcut.

Back to question Contents

SUBJECT 12

# Trauma, Stressor, and Dissociative Disorders

Answers BQ-252 to BQ-273

**BQ-252 · ANSWER B**

The PC-PTSD-5 pairs an exposure question with 5 yes or no items scored 0 to 5. The PCL-5 is the tempting choice, but it has 20 items scored 0 to 80 and is used for severity and monitoring.

Back to question Contents

**BQ-253 · ANSWER D**

A 10-point drop counts as response, and this fall is 13 points, yet the score remains above the 31 to 33 provisional range and above the 28 mark for clinically significant change. Remission is the tempting choice, but a score of 41 is not close to that mark.

Back to question Contents

**BQ-254 · ANSWER A**

VA/DoD weakly suggests individual trauma-focused cognitive behavioral therapy for acute stress disorder and found insufficient evidence for any medication. Debriefing and drug prevention with benzodiazepines are not supported by NICE.

Back to question Contents

**BQ-255 · ANSWER E**

Adjustment disorder usually resolves within about 6 months of the stressor ending, and talk therapy is the mainstay. A life-threatening event is not required, which is what separates it from PTSD, the tempting alternative.

Back to question Contents

**BQ-256 · ANSWER C**

Full criteria first met at least 6 months after the event define delayed expression. Acute stress disorder is excluded by the timing, and the dissociative specifier needs depersonalization or derealization.

Back to question Contents

**BQ-257 · ANSWER B**

The label starts PTSD treatment at 25 mg daily, raises it in 25 to 50 mg increments no faster than weekly, and caps the dose at 200 mg. The faster schedule in A ignores the label's one-week interval between increases.

Back to question Contents

**BQ-258 · ANSWER E**

The paroxetine label describes a less than 2-fold increase in cardiovascular malformations after first-trimester exposure, against a general background of 2 to 4 percent for major birth defects, so counseling is a risk-benefit discussion. Contraindication (A) overstates the label.

Back to question Contents

**BQ-259 · ANSWER A**

Paroxetine irreversibly inhibits CYP2D6 and can reduce tamoxifen efficacy, so the label advises choosing an antidepressant with little or no CYP2D6 inhibition. Paroxetine's PTSD labeling is the tempting reason in B, but it does not remove the interaction.

Back to question Contents

**BQ-260 · ANSWER D**

VA/DoD notes that venlafaxine can raise blood pressure and calls for caution in hypertension. It is not FDA labeled for PTSD, which makes A wrong.

Back to question Contents

**BQ-261 · ANSWER C**

Sertraline can cause hyponatremia, and VA/DoD notes the risk is higher in people over 65. Confusion and falls call for a sodium level before any other step; adding sedatives (B) would worsen both.

Back to question Contents

**BQ-262 · ANSWER B**

The class label table shows 5 more cases per 1,000 for ages 18 to 24, 14 more for under 18, 1 fewer for 25 to 64, and 6 fewer for 65 and older. D swaps the under-18 figure into the wrong age band.

Back to question Contents

**BQ-263 · ANSWER E**

VA/DoD starts prazosin at 1 mg at bedtime because syncope can occur 30 to 90 minutes after the first dose, and it notes additive hypotension with diuretics and PDE5 inhibitors. Starting at a higher dose (B) is the dangerous distractor.

Back to question Contents

**BQ-264 · ANSWER A**

FDA issued a complete response letter dated 8 August 2024, and VA/DoD found insufficient evidence for or against MDMA-assisted therapy. FDA's list of 2026 novel drug approvals, current to 20 October 2026, does not include it.

Back to question Contents

**BQ-265 · ANSWER D**

Trauma-informed care explains steps, offers choice and control, and avoids retraumatization. Forcing a narrative (B) or restraint (C) risks repeating harm.

Back to question Contents

**BQ-266 · ANSWER C**

VA/DoD weakly suggests that co-occurring substance use should not preclude core psychotherapies, and the American Psychological Association guideline strongly recommends trauma-focused CBT with usual substance use treatment. Delaying therapy for abstinence is the tempting but unsupported choice.

Back to question Contents

**BQ-267 · ANSWER B**

Trauma-focused CBT, used for ages 3 to 18 over 8 to 25 sessions with caregiver involvement, outperformed child-centered therapy in a trial of 229 children aged 8 to 14. The sertraline label says pediatric effectiveness is not established outside OCD, and NICE advises against drug treatment under 18.

Back to question Contents

**BQ-268 · ANSWER E**

Indiscriminate approach to unfamiliar adults with disregard of social boundaries after institutional care defines disinhibited social engagement disorder. Reactive attachment disorder, the tempting choice, shows emotional withdrawal and little comfort-seeking.

Back to question Contents

**BQ-269 · ANSWER A**

Fear of abandonment, unstable identity, and unstable relationships point to borderline personality disorder, though trauma history is common and the two diagnoses can coexist. Complex PTSD features a stable negative self-concept and avoidance of closeness.

Back to question Contents

**BQ-270 · ANSWER D**

Loss of memory for a circumscribed period after a stressor is localized amnesia, the most common type. Transient global amnesia is the tempting neurologic option, but it is anterograde and has no psychological trigger.

Back to question Contents

**BQ-271 · ANSWER C**

Preserved reality testing separates depersonalization/derealization disorder from psychosis. Onset in adolescence (average age 16) is typical but does not distinguish it.

Back to question Contents

**BQ-272 · ANSWER A**

Dissociation is one of the esketamine boxed warning headings, and each session requires observation for at least 2 hours. Dissociation here reflects the drug and does not indicate a dissociative disorder.

Back to question Contents

**BQ-273 · ANSWER B**

The minimum time since the death is at least 1 year for adults and at least 6 months for children and adolescents, so 8 months can qualify. The 12-month rule in A applies to adults.

Back to question Contents

SUBJECT 13

# Schizophrenia Spectrum and Other Psychotic Disorders

Answers BQ-274 to BQ-300

**BQ-274 · ANSWER B**

NAVIGATE participants stayed in treatment longer and improved more in quality of life and symptoms, with more work or school involvement. Hospitalization did not differ (A), and gains were larger with shorter, not longer, untreated psychosis (D).

Back to question Contents

**BQ-275 · ANSWER D**

Delusional disorder responds to alliance, not argument. Exploring distress and sleep gives something the patient will accept. Challenging the belief (A) or colluding with it (C) damages trust, and involuntary admission (B) is not indicated without danger.

Back to question Contents

**BQ-276 · ANSWER A**

A psychotic episode lasting from 1 day to under 1 month with full recovery is brief psychotic disorder. Schizophreniform disorder (B) lasts at least 1 month.

Back to question Contents

**BQ-277 · ANSWER E**

Schizoaffective disorder requires at least 2 weeks of delusions or hallucinations without a mood episode. Mood-congruent delusions (A) occur in both.

Back to question Contents

**BQ-278 · ANSWER C**

The pooled transition rate was 34% for cannabis, against 26% for hallucinogens, 22% for amphetamines, 12% for opioids, 10% for alcohol, and 9% for sedatives. Amphetamines (E) are tempting but lower.

Back to question Contents

**BQ-279 · ANSWER A**

New restlessness after a dose increase of a partial agonist suggests akathisia, and raising the dose (B) would worsen it. Options include lowering the dose or adding a benzodiazepine.

Back to question Contents

**BQ-280 · ANSWER C**

The risperidone label limits adults to 8 mg daily with fluoxetine or paroxetine, which inhibit CYP2D6.

Back to question Contents

**BQ-281 · ANSWER E**

VA/DoD lists glucose or HbA1c and lipids at baseline, at 3 months for clozapine and olanzapine, and at least yearly. Baseline only (A) is insufficient.

Back to question Contents

**BQ-282 · ANSWER B**

The label target is 300 to 450 mg daily by the end of 2 weeks, raised from 12.5 mg. The maximum is 900 mg daily, so E is a dose above it.

Back to question Contents

**BQ-283 · ANSWER D**

In benign ethnic neutropenia the label requires at least two baseline values and advises against starting below 1000 per microliter. Using 1500 (E) applies to the general population.

Back to question Contents

**BQ-284 · ANSWER C**

An ANC of 500 to 999 is moderate neutropenia: interrupt, hematology consultation, daily tests. Discontinuation (E) is for an ANC below 500.

Back to question Contents

**BQ-285 · ANSWER A**

The REMS requirements ended, but FDA still expects ANC monitoring at the label frequencies, and the Boxed Warning remains. Option D describes the removed pharmacist step.

Back to question Contents

**BQ-286 · ANSWER D**

Myocarditis most often presents within the first 2 months, and the label says to stop clozapine and obtain cardiac evaluation if it is suspected. Troponin and C-reactive protein monitoring is advised by VA/DoD in the first month.

Back to question Contents

**BQ-287 · ANSWER B**

Fluvoxamine is a strong CYP1A2 inhibitor, and the label advises about one-third of the dose. Doubling (C) would risk toxicity.

Back to question Contents

**BQ-288 · ANSWER E**

The label does not recommend use with strong CYP3A4 inducers such as carbamazepine. Carbamazepine lowers, not raises, clozapine levels (A).

Back to question Contents

**BQ-289 · ANSWER C**

The label notes that subjective constipation may not reflect the degree of hypomotility, which can progress to paralytic ileus. Distension with vomiting needs urgent assessment.

Back to question Contents

**BQ-290 · ANSWER E**

The label caps the first injection at 100 mg, with the remainder 3 to 7 days later. It also forbids intravenous use (A) and sets a 4-week interval, not 2 weeks (C).

Back to question Contents

**BQ-291 · ANSWER A**

Oral cover continues for 3 weeks after the first microsphere injection. Oral cover is not needed (E) only for some other long-acting products.

Back to question Contents

**BQ-292 · ANSWER D**

Urinary retention is a labeled contraindication, along with moderate or severe hepatic impairment, gastric retention, and untreated narrow-angle glaucoma. Hypertension (A) is an adverse reaction, not a contraindication.

Back to question Contents

**BQ-293 · ANSWER B**

The deutetrabenazine label caps total daily dose at 36 mg with strong CYP2D6 inhibitors such as paroxetine. The usual maximum is 48 mg (C).

Back to question Contents

**BQ-294 · ANSWER C**

Acute dystonia responds to an anticholinergic (APA statement 11, 1C). More haloperidol (A) worsens it.

Back to question Contents

**BQ-295 · ANSWER A**

The screen is positive at 2 or more of 14 items, and the BFCRS is the only scale validated against a lorazepam challenge. Waiting for eight items (C) would delay care.

Back to question Contents

**BQ-296 · ANSWER E**

The review describes 1 or 2 mg intravenously with reexamination after 5 minutes and a second dose if nothing changes. Intramuscular and oral routes need longer waits.

Back to question Contents

**BQ-297 · ANSWER B**

Catatonia with fever and autonomic instability is malignant catatonia, which needs urgent medical care and ECT. Raising antipsychotics (A) may be harmful.

Back to question Contents

**BQ-298 · ANSWER D**

Antipsychotic labels carry a boxed warning for increased mortality in elderly patients with dementia-related psychosis. The olanzapine label cites deaths of about 4.5% versus 2.6% over about 10 weeks.

Back to question Contents

**BQ-299 · ANSWER A**

Labels warn of extrapyramidal and withdrawal symptoms in neonates exposed in the third trimester. Neural tube defects (B) are a valproate concern.

Back to question Contents

**BQ-300 · ANSWER B**

APA recommends assertive community treatment for patients with poor engagement leading to frequent relapse or social disruption such as homelessness (statement 19, 1B). VA/DoD also recommends it strongly for severe impairment or repeated hospitalization.

Back to question Contents

SUBJECT 14

# Substance-Related and Addictive Disorders

Answers BQ-301 to BQ-327

**BQ-301 · ANSWER A**

Withdrawal delirium is treated with close monitoring, usually in intensive care, and benzodiazepines titrated to light somnolence (ASAM). Antipsychotics are adjuncts, not monotherapy, and beta-blockers or alpha-2 agonists should not be used for delirium.

Back to question Contents

**BQ-302 · ANSWER D**

The picture suggests Wernicke encephalopathy, which needs parenteral thiamine at once. ASAM notes thiamine and glucose may be given in either order or together.

Back to question Contents

**BQ-303 · ANSWER B**

Mild withdrawal without complicating factors, in a patient who can attend follow-up and is not intoxicated, can be managed ambulatory. Mild is a CIWA-Ar below 10.

Back to question Contents

**BQ-304 · ANSWER E**

Naltrexone is contraindicated with opioid analgesics and in physiologic opioid dependence because it can precipitate withdrawal. The other options are not contraindicated by opioid use.

Back to question Contents

**BQ-305 · ANSWER C**

Acamprosate is renally cleared, whereas disulfiram and naltrexone labels carry hepatotoxicity warnings. Disulfiram is also contraindicated with metronidazole.

Back to question Contents

**BQ-306 · ANSWER B**

The label states disulfiram should not be given until at least 12 hours of abstinence, and never during intoxication or without the patient's full knowledge.

Back to question Contents

**BQ-307 · ANSWER E**

The label describes severe injection-site reactions, including necrosis requiring surgery, reported mainly in female patients.

Back to question Contents

**BQ-308 · ANSWER C**

The label recommends a 7 to 10 day opioid-free interval for short-acting opioids. The 28-day figure is the approximate duration of opioid blockade after an injection, not the required opioid-free interval.

Back to question Contents

**BQ-309 · ANSWER A**

ASAM states that 7 to 14 days should typically elapse between the last buprenorphine dose and starting naltrexone, so that the person is no longer physically dependent on opioids.

Back to question Contents

**BQ-310 · ANSWER D**

ASAM advises caution but not withholding treatment because of benzodiazepines. From methadone, a long wait is needed for buprenorphine, and an abrupt benzodiazepine stop is dangerous.

Back to question Contents

**BQ-311 · ANSWER C**

The label has patients receive an initial transmucosal buprenorphine dose (for example 4 mg, observed for an hour) or already take transmucosal buprenorphine, and the product is for administration by a health care provider only.

Back to question Contents

**BQ-312 · ANSWER A**

Tolerance falls during abstinence, and a previous dose can be fatal. This is why withdrawal management alone is not recommended.

Back to question Contents

**BQ-313 · ANSWER D**

Withdrawal after naloxone is uncomfortable but not usually life-threatening. This is an opportunity to start buprenorphine and supply take-home naloxone.

Back to question Contents

**BQ-314 · ANSWER B**

The label states methadone for opioid addiction is dispensed only through certified programs under 42 CFR 8.12, with limited exceptions. The initial dose should not exceed 30 mg, so a 60 mg first dose (D) is wrong.

Back to question Contents

**BQ-315 · ANSWER E**

The final rule allows up to 7 days in days 1 to 14, 14 days in days 15 to 30, and up to 28 days from day 31, and removed the one-year history requirement.

Back to question Contents

**BQ-316 · ANSWER D**

ASAM states there is no set time limit, and tapers are slow, usually over several months. Patients who stop should be counseled about overdose risk.

Back to question Contents

**BQ-317 · ANSWER B**
COWS 25 to 36 is moderately severe; 13 to 24 is moderate and above 36 is severe.

Back to question Contents

**BQ-318 · ANSWER E**
The label states lofexidine is not a treatment for opioid use disorder and warns of hypotension, bradycardia, and QT prolongation.

Back to question Contents

**BQ-319 · ANSWER C**
Chest pain after stimulant use must be evaluated as cardiac. Benzodiazepines treat agitation.

Back to question Contents

**BQ-320 · ANSWER A**
The tables rate the evidence for 12-week abstinence as high; the advantage over CBT is not present at last follow-up.

Back to question Contents

**BQ-321 · ANSWER E**
NIDA states that cannabinoid hyperemesis syndrome resolves only when cannabis use stops completely.

Back to question Contents

**BQ-322 · ANSWER C**
NIDA states no medications are approved. VA/DoD suggests CBT or motivational enhancement and suggests against brief interventions as treatment.

Back to question Contents

**BQ-323 · ANSWER A**

NIDA notes phencyclidine can cause psychosis lasting days to weeks and dangerous behavior and injuries.

Back to question Contents

**BQ-324 · ANSWER D**

NIDA describes sudden sniffing death from irregular heart rhythm, possible after a single use.

Back to question Contents

**BQ-325 · ANSWER B**

The label read shows no boxed warning. Section 5.1 describes neuropsychiatric events, and in the large trial confidence intervals for the psychiatric cohort included zero.

Back to question Contents

**BQ-326 · ANSWER A**

For more than 10 cigarettes a day the label schedule is 21 mg for weeks 1 to 6, 14 mg for weeks 7 and 8, and 7 mg for weeks 9 and 10.

Back to question Contents

**BQ-327 · ANSWER D**

Gambling disorder carries a high suicide risk according to the APA. No FDA-approved medication exists for it.

Back to question Contents

SUBJECT 15

# Personality, Impulse-Control, and Conduct Disorders

Answers BQ-328 to BQ-349

**BQ-328 · ANSWER C**

Abandonment fear, idealization and devaluation, hours-long mood shifts after rejection, emptiness, and self-harm form the borderline pattern. Bipolar II (A) requires distinct hypomanic episodes with reduced need for sleep, which are absent here.

Back to question Contents

**BQ-329 · ANSWER A**

Episodes lasting days with reduced need for sleep and no relational trigger suggest bipolar disorder, which can coexist with BPD. Attributing them to stress (B) misses a treatable condition. NICE advises reviewing both diagnoses before treating a comorbid condition.

Back to question Contents

**BQ-330 · ANSWER E**

NICE advises against brief interventions under 3 months for BPD unless the service has an integrated theoretical approach, structured care, and therapist supervision. Option B misses those conditions.

Back to question Contents

**BQ-331 · ANSWER B**

NICE advises a single drug with low side-effect, misuse, and overdose risk at the minimum effective dose, and stopping crisis-started drugs, usually within 1 week. Polypharmacy (A) is advised against.

Back to question Contents

**BQ-332 · ANSWER D**

In ICD-11 the borderline pattern is an optional qualifier added to a graded personality disorder diagnosis.

Back to question Contents

**BQ-333 · ANSWER B**

Social communication differences and restricted interests since early childhood point to autism spectrum disorder. Schizoid personality disorder (the tempting answer) is considered only after autism is excluded.

Back to question Contents

**BQ-334 · ANSWER D**

Impulsive, disproportionate outbursts with remorse and no mood episode fit intermittent explosive disorder. Antisocial personality disorder (A) involves a broader pattern of rights violation and deceit.

Back to question Contents

**BQ-335 · ANSWER A**

The urge-tension-relief cycle and no need for the objects define kleptomania, unlike theft for gain (D).

Back to question Contents

**BQ-336 · ANSWER E**

Fire setting driven by anger within a pattern of rule breaking and cruelty fits conduct disorder. Pyromania (A) is fire setting for tension relief or fascination, not anger or gain.

Back to question Contents

**BQ-337 · ANSWER C**

The pull to rescue suggests idealization and fear of abandonment. Supervision and a warm restatement of the frame protect the alliance. Continuing the boundary drift (A) increases risk of later rupture.

Back to question Contents

**BQ-338 · ANSWER E**

The lamotrigine boxed warning names serious rash, and the label advises discontinuation at the first sign of rash unless clearly not drug related. Continuing (A) risks serious harm.

Back to question Contents

**BQ-339 · ANSWER C**

NICE guidance on antisocial personality disorder states that the diagnosis does not limit access to treatment for comorbid disorders, and depression and anxiety are often missed.

Back to question Contents

**BQ-340 · ANSWER B**

NICE advises a formal Care Programme Approach review if the person is admitted twice or more in 6 months.

Back to question Contents

**BQ-341 · ANSWER A**

In a 16-year cohort, remission rates were high, recovery was lower, and recurrence and loss of recovery occurred.

Back to question Contents

**BQ-342 · ANSWER D**

The picture is avoidant personality disorder. Exposure, cognitive therapy, and skills training address it; medication does not treat the personality pattern itself.

Back to question Contents

**BQ-343 · ANSWER A**

The CDC describes training and therapy that involves the child, family, and school as an often-used effective option for school-age children and teens.

Back to question Contents

**BQ-344 · ANSWER E**

Only the effect on BPD symptom severity met the review's threshold for clinically meaningful improvement. Self-harm (B) improved statistically but did not meet it.

Back to question Contents

**BQ-345 · ANSWER D**

NG225 advises a structured, person-centered, CBT-informed intervention, typically 4 to 10 sessions, and advises against drugs as a specific intervention to reduce self-harm and against risk scales to predict repetition.

Back to question Contents

**BQ-346 · ANSWER C**

The VA/DoD 2024 guideline made no recommendation for or against DBT to reduce suicide attempts, which reflects the strength of available trial evidence, not advice against it.

Back to question Contents

**BQ-347 · ANSWER B**

Theatrical, suggestible, shifting emotion with attention seeking and no self-harm or abandonment panic fits histrionic personality disorder. BPD (A) features self-harm and abandonment fear.

Back to question Contents

**BQ-348 · ANSWER D**

A shame-driven collapse after a humiliating loss raises suicide risk, so direct risk assessment comes first. Interpretation or confrontation can deepen shame.

Back to question Contents

**BQ-349 · ANSWER B**

Reframing indirect requests as expressions of need and a skills deficit keeps the clinician curious. Labels such as manipulative predict worse care, and NICE advises teams to review their tolerance toward people who pose risk.

Back to question Contents

SUBJECT 16

# Neurocognitive Disorders and Delirium

Answers BQ-350 to BQ-374

**BQ-350 · ANSWER A**

Acute change after illness with inattention, drowsiness, and poor intake is hypoactive delirium, which is often mislabeled as depression. Depression (B) develops over weeks and does not cause inattention of this kind.

Back to question Contents

**BQ-351 · ANSWER C**

The CAM is positive when acute onset or fluctuation and inattention are both present together with either disorganized thinking or an altered level of consciousness. Here the first two plus disorganized thinking are present, so the alert patient is still positive; agitation (E) is not required, and an altered level of consciousness (A) is only an alternative to disorganized thinking.

Back to question Contents

**BQ-352 · ANSWER E**

Haloperidol and other strong dopamine blockers can cause severe motor worsening in Parkinson disease and Lewy body disease, and NICE flags the risk. The other measures treat causes.

Back to question Contents

**BQ-353 · ANSWER B**

The program targeted cognitive impairment, sleep deprivation, immobility, visual impairment, hearing impairment, and dehydration, and delirium incidence fell from 15.0% to 9.9%.

Back to question Contents

**BQ-354 · ANSWER D**

Median days alive without delirium or coma were 8.5 with placebo, 7.9 with haloperidol, and 8.7 with ziprasidone, not significantly different.

Back to question Contents

**BQ-355 · ANSWER C**

Decline that interferes with independence in daily activities is major neurocognitive disorder; mild preserves independence.

Back to question Contents

**BQ-356 · ANSWER A**

The donepezil label permits 23 mg only after at least 3 months on 10 mg, for moderate to severe disease. Nausea and weight loss are more frequent at 23 mg.

Back to question Contents

**BQ-357 · ANSWER E**

After more than 3 days off, the label restarts at 4.6 mg/24 h and retitrates, to avoid severe gastrointestinal effects.

Back to question Contents

**BQ-358 · ANSWER B**

The galantamine label generally limits dosing to 16 mg/day at creatinine clearance 9 to 59 mL/min and does not recommend it below 9.

Back to question Contents

**BQ-359 · ANSWER D**

The memantine label gives 5 mg twice daily as the target for severe renal impairment (creatinine clearance 5 to 29 mL/min).

Back to question Contents

**BQ-360 · ANSWER B**

Cholinesterase inhibitors have vagotonic effects causing bradycardia, heart block, and syncope. Check an ECG and reduce or stop the drug.

Back to question Contents

**BQ-361 · ANSWER E**

The donanemab label obtains MRI before the 2nd, 3rd, 4th, and 7th infusions. Lecanemab's schedule differs (1, 2, 3, and 6 months).

Back to question Contents

**BQ-362 · ANSWER B**

The boxed warning states that ARIA-E can cause focal deficits mimicking ischemic stroke and that clinicians should consider it before thrombolytics.

Back to question Contents

**BQ-363 · ANSWER A**

The labels say about 15 percent of Alzheimer patients are homozygotes and that they have a higher incidence of symptomatic, serious, and severe radiographic ARIA.

Back to question Contents

**BQ-364 · ANSWER D**

The FDA cleared the test for adults 55 and older with signs and symptoms, in specialized care, and states it is not a screening or stand-alone diagnostic test.

Back to question Contents

**BQ-365 · ANSWER A**

An abnormal Core 1 biomarker is sufficient; Core 2 markers stage and prognosticate but are not stand-alone diagnostic tests.

Back to question Contents

**BQ-366 · ANSWER E**

Dementia plus one core feature and an indicative biomarker (reduced dopamine transporter uptake), or two core features, gives probable dementia with Lewy bodies; here visual hallucinations and parkinsonism are two core features.

Back to question Contents

**BQ-367 · ANSWER C**

Three of six features define possible behavioral variant frontotemporal dementia; probable adds functional decline and characteristic imaging.

Back to question Contents

**BQ-368 · ANSWER B**

Rapidly progressive dementia plus at least two of four clinical features, supported by MRI changes, a typical EEG, or a positive CSF 14-3-3 assay, defines probable disease, and a positive CSF RT-QuIC with a neuropsychiatric disorder is a separate route.

Back to question Contents

**BQ-369 · ANSWER D**

Pimavanserin is labeled for hallucinations and delusions associated with Parkinson disease psychosis (boxed mortality warning). Risperidone and olanzapine (B, C) worsen motor function.

Back to question Contents

**BQ-370 · ANSWER E**

NICE advises a personalized multicomponent approach and not melatonin for insomnia in Alzheimer disease, and advises against valproate for agitation.

Back to question Contents

**BQ-371 · ANSWER A**

In the randomized trial of 295 patients, continuing donepezil gave a 1.9-point higher standardized MMSE and a 3.0-point better BADLS than stopping; the combination with memantine was not significantly better than donepezil alone.

Back to question Contents

**BQ-372 · ANSWER C**

The label carries a boxed warning for depression and suicidality; assess, treat, and reduce or stop the drug.

Back to question Contents

**BQ-373 · ANSWER C**

Strongly anticholinergic drugs worsen cognition; NICE advises reducing anticholinergic burden. Adding a sedative or an antipsychotic compounds harm.

Back to question Contents

**BQ-374 · ANSWER D**

NICE advises against routine enteral feeding in severe dementia unless treating a potentially reversible comorbidity, and favors comfort-focused care.

Back to question Contents

SUBJECT 17

# Neurologic Disorders in Psychiatric Practice

Answers BQ-375 to BQ-396

**BQ-375 · ANSWER D**

The 2019 AAN guideline says to offer comprehensive behavioral intervention for tics first when it is available. Antipsychotics (A, B, C) may be used when benefit outweighs risk, but they carry metabolic and movement risks. Topiramate (E) is a later option.

Back to question Contents

**BQ-376 · ANSWER A**

Hours to days after a seizure cluster, psychosis fits postictal psychosis, which usually resolves within days and is treated with seizure control and a benzodiazepine, a low-dose antipsychotic, or both. B commits to a chronic disorder too early. D would provoke more seizures, and C is wrong because bupropion lowers seizure threshold.

Back to question Contents

**BQ-377 · ANSWER C**

Topiramate causes dose-related cognitive reactions, including word-finding trouble, and the usual response is a lower dose or a switch. A and E overlook the obvious drug cause. D is possible, but the timing points to the drug.

Back to question Contents

**BQ-378 · ANSWER E**

With valproate, lamotrigine starts at 25 mg every other day and the bipolar target is 100 mg daily, reached by a slow schedule. A and B are the schedules without valproate and with inducers. D ignores the interaction that raises rash risk.

Back to question Contents

**BQ-379 · ANSWER B**

The carbamazepine boxed warning concerns serious skin reactions linked to HLA-B*1502, and patients of at-risk Asian ancestry should be screened before treatment. Lithium level (A) does not apply.

Back to question Contents

**BQ-380 · ANSWER A**

The valproate label advises against use in women of childbearing potential for epilepsy or bipolar disorder unless other medications have failed or are unacceptable, because of malformations and lower IQ in exposed children. B is wrong, since the malformation rate is about four times that of other antiseizure monotherapies.

Back to question Contents

**BQ-381 · ANSWER C**

Perampanel carries a boxed warning for serious psychiatric and behavioral reactions; hostility and aggression were dose related and mostly appeared in the first weeks, so monitor especially during titration. A and B are wrong, and D is wrong because perampanel lowers levonorgestrel exposure. E is wrong because reactions occurred with and without prior history.

Back to question Contents

**BQ-382 · ANSWER B**

In the FOCUS trial of 3,127 patients, fluoxetine did not improve functional outcome, new depression was lower, and fractures were higher. A contradicts the trial. E is wrong because fluoxetine is not contraindicated; it is simply not indicated for recovery.

Back to question Contents

**BQ-383 · ANSWER E**

Dextromethorphan with quinidine is labeled for pseudobulbar affect, studied in ALS and MS, with a check for QT prolongation and interacting drugs. Sertraline (D) is used off label and does not carry this indication. Haloperidol and lorazepam do not treat the disorder.

Back to question Contents

**BQ-384 · ANSWER D**

In the cohort of 559 hospitalized adults, depression at injury, a history of depression, and lifetime alcohol dependence predicted major depression, and older age was associated with lower risk. A and E point the wrong way.

Back to question Contents

**BQ-385 · ANSWER E**

The Amsterdam consensus defines persisting symptoms as lasting more than 4 weeks and recommends a multimodal evaluation, which includes sleep and mental health, with referral for rehabilitation. A conflicts with the advice for relative, not strict, rest. B is unsafe.

Back to question Contents

**BQ-386 · ANSWER C**

Interferon beta can cause depression. Screen for it, treat it, and discuss alternatives. A and E are wrong, and B ignores that depression is common in multiple sclerosis and worsens fatigue and quality of life.

Back to question Contents

**BQ-387 · ANSWER B**

Corticosteroid labels warn of psychiatric derangements from euphoria and insomnia to psychosis. Lowering or stopping the steroid where possible, and treating symptoms, is the first step. A mislabels a drug effect as a lifelong disorder.

Back to question Contents

**BQ-388 · ANSWER E**

The metoclopramide boxed warning cites tardive dyskinesia that can be irreversible and advises against treatment beyond 12 weeks; stop the drug when movements appear. A and D can mask or worsen the problem, and B adds a second dopamine blocker.

Back to question Contents

**BQ-389 · ANSWER D**

The clozapine label describes dose-related seizure risk (about 5% cumulative incidence at one year) and advises slow titration with divided dosing. A and C are contradicted by the label. B is too absolute: the label advises caution rather than absolute contraindication.

Back to question Contents

**BQ-390 · ANSWER C**

Serum concentrations above 3 mEq/L are associated with seizures, coma, and brain damage, and NSAIDs and volume loss raise lithium levels. A is possible but unlikely given the level and context. B and D lack the history.

Back to question Contents

**BQ-391 · ANSWER A**

In the AAN/AHS guideline amitriptyline is probably effective for migraine prevention and also treats depression. B and C should not be offered for migraine prevention, D has inadequate evidence, and E has inadequate or conflicting data.

Back to question Contents

**BQ-392 · ANSWER B**

A weak hip that extends with normal force when the opposite hip is flexed against resistance is the Hoover sign, which had pooled sensitivity 94% and specificity 99% for functional leg weakness. D is wrong because functional weakness is not feigned.

Back to question Contents

**BQ-393 · ANSWER D**

In the CODES trial the primary outcome (seizure frequency at 12 months) was not significantly different, but work and social adjustment improved and adverse events were similar in the two arms. A overpromises, and B is not supported by the safety findings.

Back to question Contents

**BQ-394 · ANSWER E**

Recovery can continue for up to 18 months, and 81% of patients followed to 24 months in the large cohort had a good outcome. B is wrong, since relapse occurred in 12% within 2 years. D is wrong because second-line treatment was associated with better outcome.

Back to question Contents

**BQ-395 · ANSWER C**

The Graus criteria classify Hashimoto encephalopathy (encephalopathy with seizures or hallucinations, thyroid antibodies, normal or nonspecific MRI, no well-characterized neuronal antibody) as probable autoimmune encephalitis. A is tempting but understates the category.

Back to question Contents

**BQ-396 · ANSWER A**

For CYP2D6 poor metabolizers the valbenazine label limits the dose to 40 mg once daily. B and C exceed the label (the maximum is 80 mg), and D ignores the pharmacogenetic risk.

Back to question Contents

SUBJECT 18

# Somatic Symptom and Eating Disorders

Answers BQ-397 to BQ-418

**BQ-397 · ANSWER B**

Preoccupation with having a serious illness, minimal symptoms, checking, and avoidance of care fit illness anxiety disorder, care-avoidant type, which requires at least 6 months. Somatic symptom disorder is the tempting choice, but it requires significant symptoms driving the worry. Delusional disorder would show fixed belief unmoved by evidence.

Back to question Contents

**BQ-398 · ANSWER E**

The PHQ-15 cutoffs of 5, 10, and 15 mark low, medium, and high severity, so 12 is medium. High severity is the tempting choice, but it begins at 15. The score measures burden and tracks change; it does not make the diagnosis.

Back to question Contents

**BQ-399 · ANSWER A**

In a 2007 review of randomized trials, a consultation letter to the primary care physician was the most consistent benefit among non-CBT approaches. Repeating the workup is the tempting choice, but it multiplies testing without benefit, and benzodiazepines and opioids carry abuse and dependence risks.

Back to question Contents

**BQ-400 · ANSWER D**

The CODES trial (368 adults) found no significant difference in monthly seizure frequency at 12 months, but psychosocial functioning and several secondary outcomes were better with CBT, and serious adverse events were similar. "Markedly reduced frequency" is the tempting choice, but the primary outcome was negative.

Back to question Contents

**BQ-401 · ANSWER C**

The diagnosis is real and rests on positive signs, so explain it that way, then use physiotherapy and CBT. A standard reference finds no evidence that drugs treat the condition itself. Telling him it is imaginary is the tempting wrong choice; it damages trust, and the symptoms are not feigned.

Back to question Contents

**BQ-402 · ANSWER B**

In suspected imposed-on-another cases, the victim's safety comes first: secure the child, document facts, and report to child protection as state law requires. Confronting the caregiver alone is the tempting choice, but it can endanger the child and compromise evidence.

Back to question Contents

**BQ-403 · ANSWER E**

A clear external incentive (the lawsuit) with inconsistent effort points to malingering, which is a focus of clinical attention and not a mental disorder. Factitious disorder is the tempting choice, but it has no obvious external reward.

Back to question Contents

**BQ-404 · ANSWER A**

NICE offers individual CBT for eating disorders, MANTRA, or specialist supportive clinical management, and the patient chooses after explanation. Family therapy is the tempting choice, but it is the NICE option for children and young people. NICE advises against medication as the sole treatment.

Back to question Contents

**BQ-405 · ANSWER C**

NICE advises against an absolute weight or BMI threshold and points to the rate of weight loss, the need for active monitoring, and ECG and laboratory findings such as prolonged QT. Waiting for a lower weight is the tempting wrong choice and delays care.

Back to question Contents

**BQ-406 · ANSWER D**

The SAHM criteria include a daytime heart rate under 50 beats per minute. The nighttime threshold is under 45, so 52 does not qualify. Blood pressure must be under 90/45, temperature under 35.6 degrees C, and the adolescent orthostatic heart rate rise over 40, so options A, B, and C fall short.

Back to question Contents

**BQ-407 · ANSWER B**

In a 16-week trial of 152 adults, olanzapine gave a modest BMI gain versus placebo (0.26 versus 0.10 kg/m2 per month) and no significant benefit on obsessionality. The label does not list anorexia nervosa. Body image improvement is the tempting choice, but shape concern scores rose more with olanzapine.

Back to question Contents

**BQ-408 · ANSWER A**

ASPEN advises close electrolyte attention and more aggressive repletion when levels are low before feeding, and leaves preemptive thiamine to clinical judgment of risk; in a high-risk patient it is usual to give it before feeding or dextrose-containing fluids. Delaying thiamine is the tempting choice, but it defeats the purpose. Refeeding syndrome is not limited to children.

Back to question Contents

**BQ-409 · ANSWER E**

Vomiting causes hypokalemia and metabolic alkalosis. Hyperkalemia with acidosis is the tempting choice because it reverses the direction, but vomiting loses acid and potassium.

Back to question Contents

**BQ-410 · ANSWER C**

The bupropion label contraindicates use in patients who currently have or previously had bulimia nervosa or anorexia nervosa because of a higher seizure incidence. Remission is the tempting reason to prescribe, but the contraindication includes past diagnosis.

Back to question Contents

**BQ-411 · ANSWER D**

The APA suggests family-based treatment for adolescents with bulimia nervosa and an involved caregiver (2C), and NICE offers family therapy for 18 to 20 sessions over 6 months. Fluoxetine alone is the tempting choice, but medication is not advised as the sole treatment.

Back to question Contents

**BQ-412 · ANSWER B**

NICE offers guided self-help first and then group CBT, and states that weight loss is not a treatment target in binge-eating disorder. The APA recommends CBT or interpersonal therapy. A strict diet is the tempting choice, but weight loss is not the target, and lisdexamfetamine is not for weight loss.

Back to question Contents

**BQ-413 · ANSWER A**

The label starts 30 mg each morning, raises by 20 mg at about weekly intervals to a target of 50 to 70 mg daily, and sets 70 mg as the maximum. Keeping 30 mg is the tempting choice, but 30 mg did not beat placebo in the forced-dose trial.

Back to question Contents

**BQ-414 · ANSWER E**

Recurrent binge eating with loss of control and distress, without regular compensatory behavior, fits binge-eating disorder. Bulimia nervosa is the tempting choice, but it requires recurrent compensatory behavior as well.

Back to question Contents

**BQ-415 · ANSWER C**

Food restriction after a choking episode with fear of aversive consequences, weight loss, and no disturbance of weight or shape fits avoidant/restrictive food intake disorder. Anorexia nervosa is the tempting choice, but it involves weight and shape concern.

Back to question Contents

**BQ-416 · ANSWER D**

CBT or family-based treatment is commonly used, but the APA noted that rigorous trial data were lacking, and the best-known adult study was small and uncontrolled. Family-based treatment superiority is the tempting claim, but trials have not established it.

Back to question Contents

**BQ-417 · ANSWER A**

Pica in pregnancy is associated with iron deficiency, so evaluate nutritional status, and test blood lead if paint or soil exposure is possible. Reassurance is the tempting choice, but pica can cause deficiency, obstruction, lead poisoning, and parasitic infection.

Back to question Contents

**BQ-418 · ANSWER B**

Regurgitation with rechewing, without nausea or retching, fits rumination disorder, and diaphragmatic breathing with behavioral support is the main treatment in a critical review. A proton pump inhibitor is the tempting choice, but reflux is not the mechanism. Surgery is not indicated.

Back to question Contents

SUBJECT 19

# Sleep-Wake, Sexual, Gender, and Paraphilic Disorders

Answers BQ-419 to BQ-437

**BQ-419 · ANSWER C**

The VA/DoD algorithm points to positive airway pressure first for severe disease (above 30 events an hour). Mandibular advancement is an option for mild to moderate disease. Oxygen alone and atomoxetine are advised against, and modafinil treats residual sleepiness rather than the airway.

Back to question Contents

**BQ-420 · ANSWER A**

Solriamfetol is contraindicated with an MAO inhibitor or within 14 days after stopping one. Modafinil calls for caution with MAO inhibitors but is not contraindicated. Pitolisant and oxybate have other cautions, not this one.

Back to question Contents

**BQ-421 · ANSWER E**

Pitolisant can prolong the QT interval, so avoid other QT-prolonging drugs, and it may make hormonal contraception less effective, so a non-hormonal method is used during treatment and for at least 21 days after stopping. It is not a controlled drug and is given in the morning.

Back to question Contents

**BQ-422 · ANSWER B**

The oxybate label advises lowering the oxybate dose by at least 20 percent when divalproex is started in a patient on a stable oxybate dose. Raising the dose would add to central nervous system depression.

Back to question Contents

**BQ-423 · ANSWER D**

For adults 65 and older, the label starts at 3 mg, taken within 30 minutes of bedtime and not within 3 hours of a meal, with a maximum of 6 mg. The 10 to 100-fold higher doses used for depression are not the insomnia dose.

Back to question Contents

**BQ-424 · ANSWER A**

Ramelteon is contraindicated with fluvoxamine, a strong CYP1A2 inhibitor. It is not controlled, is indicated for sleep onset, and carries no boxed warning. VA/DoD found insufficient evidence for or against it.

Back to question Contents

**BQ-425 · ANSWER C**

With a moderate CYP3A inhibitor the label calls for 5 mg, generally not exceeding 10 mg. Use with a strong inhibitor is not recommended. Standard dosing without adjustment would raise exposure.

Back to question Contents

**BQ-426 · ANSWER E**

The FDA required the benzodiazepine boxed warning to cover abuse, misuse, addiction, physical dependence, and withdrawal reactions across the class. Complex sleep behaviors are the warning for the z-drugs, and the other options are not class warnings.

Back to question Contents

**BQ-427 · ANSWER B**

AASM and VA/DoD both weakly advise against trazodone for chronic insomnia, and VA/DoD names other drugs with weak support. Neither guideline recommends it for sleep onset, sleep maintenance, or older adults.

Back to question Contents

**BQ-428 · ANSWER D**

Worsening earlier in the day and spread to the arms describe augmentation. The AASM 2024 guideline asks for iron studies, favors gabapentinoids, and advises against standard use of dopamine agonists. Raising the dose worsens augmentation, and adding a second dopamine agonist or clonazepam does not address iron status or the underlying problem. Bupropion is not the next step.

Back to question Contents

**BQ-429 · ANSWER C**

Normal sleep when timing is free points to delayed sleep-wake phase disorder. The AASM weakly recommends timed melatonin, and light therapy in adolescents, with schedule changes. A hypnotic, stimulant, or antipsychotic does not address the clock.

Back to question Contents

**BQ-430 · ANSWER A**

Fluvoxamine raised tasimelteon exposure about 7-fold, so the label advises avoiding the combination. Tasimelteon is not controlled, is taken an hour before bed, and is labeled for non-24-hour disorder in adults.

Back to question Contents

**BQ-431 · ANSWER E**

The AASM 2018 position paper recommends image rehearsal therapy. Prazosin and several other drugs may be used but are not recommended as first-line. Clonazepam and venlafaxine were not recommended for nightmare disorder.

Back to question Contents

**BQ-432 · ANSWER D**

The flibanserin label says to skip the dose when three or more drinks are taken that evening. Fewer drinks require a wait of at least 2 hours before the bedtime dose. Doubling or timing the dose differently does not remove the risk of hypotension and syncope.

Back to question Contents

**BQ-433 · ANSWER B**

Bremelanotide can significantly lower exposure to oral naltrexone, so the label advises avoiding the combination. It is labeled for premenopausal women and is given by subcutaneous injection, not by mouth.

Back to question Contents

**BQ-434 · ANSWER A**

The AUA/SMSNA guideline names daily SSRIs, on-demand clomipramine or dapoxetine where available, and topical anesthetics as first-line. Tramadol and alpha-blockers are for use after first-line treatment fails, and PDE5 inhibitors are not among the first-line choices for premature ejaculation.

Back to question Contents

**BQ-435 · ANSWER C**

The European regulator advised adding persistent sexual dysfunction after SSRI or SNRI withdrawal to product information in May 2019, and its frequency is unknown. The regulator paper states that the prevalence is unknown.

Back to question Contents

**BQ-436 · ANSWER E**

The 2021 review of 20 studies found associations with less depression, less anxiety, and better quality of life, rated limited in certainty because of bias, small samples, and confounding, and drew no conclusion on suicide deaths. It is not strong evidence and did not find mental health harm.

Back to question Contents

**BQ-437 · ANSWER B**

Unwanted, horrifying intrusive images without arousal, with avoidance and checking, fit obsessive-compulsive disorder. Pedophilic disorder involves recurrent sexual arousal to children. Compulsive sexual behavior disorder involves failed attempts to control sexual behavior.

Back to question Contents

SUBJECT 20

# Child and Adolescent Psychiatry

Answers BQ-438 to BQ-461

**BQ-438 · ANSWER B**

In CAMS, 80.7 percent of children given CBT plus sertraline were much or very much improved, compared with 59.7 percent with CBT, 54.9 percent with sertraline, and 23.7 percent with placebo, and AACAP suggests combination treatment. Watchful waiting (D) leaves impairing anxiety untreated, and AACAP makes no treatment recommendation for benzodiazepines (A).

Back to question Contents

**BQ-439 · ANSWER D**

The label's pooled analysis found suicidal ideation in 0.4 percent of atomoxetine-treated children versus 0 percent on placebo, with no suicides and all cases in the first month at ages 6 to 12. The warning is for pediatric patients (A), and the liver warning is separate (C).

Back to question Contents

**BQ-440 · ANSWER A**

Abrupt discontinuation of clonidine extended-release can cause rebound hypertension, which is why the label advises tapering by no more than 0.1 mg every 3 to 7 days. Stimulant withdrawal (D) causes fatigue and mood change rather than hypertension and headache.

Back to question Contents

**BQ-441 · ANSWER C**

A hearing test is part of any evaluation of delayed speech, and speech-language evaluation is appropriate because two-word combinations are expected by about age 2. Intact pointing, pretend play, and social seeking make autism less likely but do not remove the need for assessment, so reassurance and delay (B) are wrong, and jumping to autism intervention alone (A) skips hearing.

Back to question Contents

**BQ-442 · ANSWER E**

About 75 percent of children who stutter recover, but evaluation is advised if stuttering lasts 3 to 6 months, shows struggle, or runs in the family; this boy meets all three. No drug is approved for stuttering (D), and asking him to slow down and repeat (A) can increase pressure.

Back to question Contents

**BQ-443 · ANSWER A**

The AAN gives CBIT first-line status when available (level B), and in the pivotal trial 52.5 percent of CBIT-treated children were much or very much improved versus 18.5 percent with supportive therapy. Antipsychotics (B, C) are reserved for tics that cause pain, injury, or major impairment after behavioral treatment, and deep brain stimulation (D) is for refractory cases.

Back to question Contents

**BQ-444 · ANSWER C**

Any bruising in an infant 4.99 months old or younger makes the rule positive. The rule had sensitivity 95.6 percent and specificity 87.1 percent, but it is a screening aid that signals the need for further evaluation, not a diagnosis (D).

Back to question Contents

**BQ-445 · ANSWER E**

The AAP lists whole exome sequencing as the step after history, examination, microarray, and fragile X testing do not identify a cause, with reported yields of 8 to 20 percent. Repeating a karyotype (A) adds little to microarray, and stopping (D) ignores the likely yield.

Back to question Contents

**BQ-446 · ANSWER B**

IDEA Part B provides special education and related services from ages 3 through 21 through an IEP, which is the route to specialized instruction. Section 504 protects students with disabilities and supports accommodations, so accommodations alone do not meet her instructional need, and Part C (D) covers birth through age 2.

Back to question Contents

**BQ-447 · ANSWER D**

For Tourette disorder at 50 kg or more, the aripiprazole label starts at 2 mg, targets 10 mg, and sets a maximum of 20 mg; under 50 kg the target is 5 mg and the maximum 10 mg (B). Metabolic and movement monitoring is still needed.

Back to question Contents

**BQ-448 · ANSWER D**

Activation with decreased need for sleep, pressured speech, and impulsivity, with a family history of bipolar I disorder, suggests mania or hypomania. Antidepressant activation can unmask mania, and AACAP says antidepressants belong only alongside a mood stabilizer in bipolar disorder. Raising the dose (A) or adding a sedative (C) would not address the cause.

Back to question Contents

**BQ-449 · ANSWER A**

Snoring, restless sleep, and daytime sleepiness point to sleep-disordered breathing, which can mimic or worsen ADHD, and the AAP says to exclude alternative causes before diagnosing ADHD. Starting a stimulant (B) or another drug first would treat the wrong problem.

Back to question Contents

**BQ-450 · ANSWER E**

NICE says an ECG is not routine unless specific cardiac features are present, and AACAP found no support for routine cardiac testing in healthy patients. Baseline height, weight, pulse, and blood pressure are recommended, and routine labs (B) are unnecessary when the history is unremarkable.

Back to question Contents

**BQ-451 · ANSWER C**

The lithium label calls for renal and thyroid assessment before treatment and a level after 3 days, drawn 12 hours after the dose. In pediatric trials TSH rose in 25 percent of lithium-treated patients versus 0 percent on placebo, so thyroid monitoring is specific to this drug.

Back to question Contents

**BQ-452 · ANSWER B**

AACAP says the validity of bipolar diagnosis in young children is not established and advises avoiding it under age 6. Chronic irritability without discrete episodes calls for a broader assessment and parent training, and starting lithium (A) or an antipsychotic (C) on a family history alone is premature.

Back to question Contents

**BQ-453 · ANSWER B**

On CRAFFT 2.1, substance use in the past year with a score of 2 or more is high risk, which calls for brief advice, follow-up, and consideration of referral. The tool is a screen, so a urine test (E) is not required to classify risk.

Back to question Contents

**BQ-454 · ANSWER E**

Any yes on the four-question ASQ is a positive screen and should lead to a brief suicide safety assessment, whatever the presenting complaint. Reassurance (A) ignores the screen, and automatic involuntary admission (D) is not indicated before assessment.

Back to question Contents

**BQ-455 · ANSWER A**

The boxed warning describes a high potential for abuse and misuse, and safe storage, locked where possible, and no sharing are standard counseling. Extended-release products can still be misused (B), and the warning is not limited to adults (C).

Back to question Contents

**BQ-456 · ANSWER C**

Stimulant labels advise closely monitoring growth, and a patient who is not growing or gaining weight as expected may need treatment interrupted. Options include dose and timing review, a drug holiday, or a change of agent. Assuming growth effects always resolve (A) is wrong, because one cohort lost about 2 cm of height over 3 years without rebound.

Back to question Contents

**BQ-457 · ANSWER D**

Trials show gains with early intervention, for example IQ rising 17.6 points versus 7.0 in a small randomized trial of the Early Start Denver Model, but the large meta-analysis found benefits mostly for specific outcomes and low to moderate certainty, with adverse events poorly reported. A promise of cure (A) overstates the data, and denying any evidence (B) understates it.

Back to question Contents

**BQ-458 · ANSWER C**

NICE says to review effectiveness and side effects at 3 to 4 weeks and to stop an antipsychotic if there is no clinically important response by 6 weeks. Escalating the dose weekly (B) or continuing without review (A) conflicts with using the minimum effective dose.

Back to question Contents

**BQ-459 · ANSWER A**

The label advises limiting fluids from 1 hour before the dose until the next morning and interrupting treatment during illness with fluid or electrolyte disturbance such as vomiting, diarrhea, or fever, because of the risk of hyponatremia. The maximum is 0.6 mg (E), and SSRIs call for caution rather than preference (D).

Back to question Contents

**BQ-460 · ANSWER D**

The USPSTF recommends screening adolescents aged 12 to 18 for major depressive disorder (grade B) and found insufficient evidence for suicide risk screening (grade I). It recommended anxiety screening at ages 8 to 18 (grade B), so E is wrong, and it did not endorse specific PHQ-9 cutoffs (C).

Back to question Contents

**BQ-461 · ANSWER B**

AACAP advises continuing an effective SSRI dose for about 12 months after remission, then tapering slowly and monitoring for several months. Stopping right after remission (A, E) raises the chance of relapse, and abrupt discontinuation (D) risks withdrawal symptoms.

Back to question Contents

SUBJECT 21

# Perinatal, Reproductive, and Geriatric Psychiatry

Answers BQ-462 to BQ-477

**BQ-462 · ANSWER D**

ACOG's table says any answer to the self-harm item other than "never" needs further assessment even when the total is low, so a direct suicide risk assessment is the next step. A total below the cutoff (A, E) does not exclude risk, and starting a drug on a single item (B) skips the assessment.

Back to question Contents

**BQ-463 · ANSWER A**

ACOG recommends validated screening for depression and anxiety at the first prenatal visit, later in pregnancy, and at postpartum visits, with systems for assessment and follow-up. Screening only high-risk patients (B) or only when patients raise concerns (C) misses most cases, and a single time point (D, E) cannot detect onset at other times.

Back to question Contents

**BQ-464 · ANSWER C**

ACOG recommends screening for bipolar disorder before starting treatment for depression or anxiety, and this history strongly suggests bipolar disorder. Starting sertraline alone (A) risks destabilizing the illness. Valproate (D) is the drug ACOG says to avoid in pregnancy.

Back to question Contents

**BQ-465 · ANSWER E**

The lithium label advises reducing or stopping lithium 2 to 3 days before delivery and restarting at the preconception dose once the patient is medically stable, with close serum monitoring. Doubling the dose (B) or ignoring levels (C) raises toxicity risk, and valproate (D) is to be avoided in pregnancy.

Back to question Contents

**BQ-466 · ANSWER B**

The sertraline label describes low milk levels, with infants averaging about 2% of maternal serum levels, and ACOG says breastfeeding can continue with SSRIs. Stopping breastfeeding (A) is unnecessary, and the label does not recommend breastfeeding on lithium (D).

Back to question Contents

**BQ-467 · ANSWER A**

Jitteriness, poor feeding, and breathing difficulty in the first days after late SSRI exposure fit neonatal adaptation, which ACOG says usually resolves within 2 weeks. The other syndromes need other exposures (valproate, lithium, opioids), and the cardiac risk with SSRIs is small (D).

Back to question Contents

**BQ-468 · ANSWER D**

Carbamazepine induces CYP3A4, and the label warns that hormonal contraception may fail. Folic acid (E) does not reverse induction. Because carbamazepine is also associated with spina bifida, pregnancy planning matters.

Back to question Contents

**BQ-469 · ANSWER C**

ACOG recommends SSRIs as first-line pharmacotherapy for perinatal anxiety, names sertraline and escitalopram as reasonable first choices without a prior history, and recommends avoiding or sparingly prescribing benzodiazepines (Strong; moderate quality). Alprazolam as first-line therapy (A) conflicts with this, and withholding treatment (B) ignores the benefit of therapy.

Back to question Contents

**BQ-470 · ANSWER E**

Unwanted, feared images with preserved insight, avoidance, and no psychosis describe perinatal OCD, which is treated with an SSRI and CBT. Treating it as psychosis (A, B) or as intent (C) is a harmful error, and a hypnotic (D) does not address the disorder.

Back to question Contents

**BQ-471 · ANSWER B**

ACOG notes that lamotrigine clearance rises in pregnancy and that the dose must be carefully down-titrated after delivery to avoid toxicity. Dizziness, diplopia, and ataxia are classic toxicity signs. Postpartum psychosis (A) would present with confusion and delusions, not ataxia.

Back to question Contents

**BQ-472 · ANSWER D**

The 2018 NAMS and National Network of Depression Centers consensus keeps antidepressants and psychotherapy first-line and states that estradiol is not FDA approved to treat mood disturbance, though some evidence shows antidepressant effects in perimenopause. Only desvenlafaxine has proven efficacy in large randomized trials, so (B) is wrong.

Back to question Contents

**BQ-473 · ANSWER A**

Fezolinetant is primarily metabolized by CYP1A2, the label contraindicates it with CYP1A2 inhibitors, and fluvoxamine, a strong inhibitor, raised exposure about 840%. CBT, hypnosis, and gabapentin are recommended nonhormone options in the NAMS 2023 statement, and none is contraindicated here.

Back to question Contents

**BQ-474 · ANSWER E**

The citalopram label caps the dose at 20 mg daily above age 60 because of QT prolongation, and cautions against use with hypokalemia. Raising the dose (A) is the opposite of the label, and amitriptyline (D) is on the Beers avoid list.

Back to question Contents

**BQ-475 · ANSWER C**

Beers advises avoiding benzodiazepines, Z-drugs, and first-generation antihistamines in older adults, and these drugs raise the risk of falls and delirium. A gradual taper with CBT for insomnia is the preferred approach. Adding temazepam (A) or quetiapine (E) adds risk, and hydroxyzine (B) is on the same avoid list.

Back to question Contents

**BQ-476 · ANSWER B**

After age 60, secondary causes dominate: delirium, neurocognitive disorders, medications, sensory loss, and substances. The 2000 consensus advises very low starting doses and slow increases if an antipsychotic is needed. A standard adult dose (A) risks harm, and a benzodiazepine (D) is on the Beers avoid list.

Back to question Contents

**BQ-477 · ANSWER E**

Persistent yearning, disbelief, avoidance, and loneliness at 14 months, with impairment, fit prolonged grief disorder, whose adult threshold is 12 months. In a randomized trial, grief-focused therapy plus placebo had a response rate of 82.5%, and citalopram added no benefit to response. Citalopram alone (A) is less effective, and watchful waiting (C) ignores impairment.

Back to question Contents

SUBJECT 22

# Suicide, Violence, and Psychiatric Emergencies

Answers BQ-478 to BQ-499

**BQ-478 · ANSWER B**

The C-SSRS rates actual attempts by lethality, separately from ideation severity (A), ideation intensity (C), and behavior types such as preparation (D) and aborted attempts (E). Nonsuicidal self-injury is recorded under behavior.

Back to question Contents

**BQ-479 · ANSWER D**

The positive predictive value of 5.5 percent means about 95 percent of patients labeled high risk did not die by suicide, and the sensitivity of 56 percent means many deaths came from patients not labeled high risk. E reverses the meaning of sensitivity. Use the data to temper reliance on categories, not to abandon assessment.

Back to question Contents

**BQ-480 · ANSWER A**

NICE advises against using risk tools or global low, medium, or high strata to decide who is offered treatment or who is discharged, and recommends a risk formulation at every psychosocial assessment. C is wrong because NICE says not to delay the assessment until medical treatment ends.

Back to question Contents

**BQ-481 · ANSWER E**

The VA/DoD counseling resource advises temporary, least restrictive storage with a trusted holder, collaboration, and follow-up. It states that clinicians should not take possession of firearms, ask patients to bring them to clinics, or tell patients to give them away (B, C). Firearm access carried a pooled odds ratio of 3.24 for suicide.

Back to question Contents

**BQ-482 · ANSWER C**

The label's age-stratified table shows 5 more cases per 1,000 treated at ages 18 to 24, 14 more under 18, 1 fewer at 25 to 64, and 6 fewer at 65 and older. D reverses the older-adult figure.

Back to question Contents

**BQ-483 · ANSWER B**

The guideline gives a weak recommendation for clozapine in schizophrenia or schizoaffective disorder with suicidal ideation or attempts. It makes no recommendation for or against lithium, esketamine, dialectical behavior therapy, or transcranial magnetic stimulation. Clozapine still requires label neutrophil monitoring.

Back to question Contents

**BQ-484 · ANSWER D**

The label indication is depressive symptoms in adults with major depression and acute suicidal ideation or behavior, given with an oral antidepressant, under a REMS with observation for at least 2 hours. A is wrong because it is never dispensed for home use. B is wrong because there are five boxed warning headings.

Back to question Contents

**BQ-485 · ANSWER A**

The guideline suggests periodic caring communications for 12 months after hospitalization for suicide risk (weak for). It found insufficient evidence for brief contact after emergency department discharge, crisis lines, standalone gatekeeper training, and peer-to-peer programs.

Back to question Contents

**BQ-486 · ANSWER C**

Intent can be mixed or shift, so an ambiguous episode is treated as potentially suicidal and intent is asked about again. NICE advises against delaying the psychosocial assessment until medical treatment is complete and advises exploring the function of self-harm. A is wrong because attributing behavior to manipulation closes the conversation.

Back to question Contents

**BQ-487 · ANSWER E**

The consensus advises one designated speaker, clearing bystanders, at least 2 arm's lengths of distance, calm limit setting, and honest choices. A and B escalate. C and D abandon the limit setting the consensus recommends.

Back to question Contents

**BQ-488 · ANSWER B**

Project BETA advises sparing medication in alcohol intoxication, avoiding benzodiazepines because of respiratory depression with alcohol, and preferring an antipsychotic if needed, with haloperidol preferred for its safety record. A applies to undifferentiated agitation without alcohol intoxication. C applies to withdrawal.

Back to question Contents

**BQ-489 · ANSWER D**

The haloperidol label states it is not approved for intravenous administration, and a QTc above 500 msec is associated with a higher risk of torsades. Hypokalemia, other QT-prolonging drugs, and intravenous use add risk. A and B contradict the label.

Back to question Contents

**BQ-490 · ANSWER A**

The ziprasidone label contraindicates use with known QT prolongation, including congenital long QT syndrome. Olanzapine intramuscular, lorazepam, and benztropine do not carry that contraindication. E is tempting because the dexmedetomidine label lists no contraindications, but it advises avoiding use in patients at risk for torsades.

Back to question Contents

**BQ-491 · ANSWER C**

The droperidol boxed warning directs a 12-lead ECG first and says not to give the drug if QTc exceeds 440 msec in males or 450 msec in females. Its labeled use is perioperative nausea and vomiting, so agitation use is off-label, and Project BETA advises avoiding it for agitation while the boxed warning stands.

Back to question Contents

**BQ-492 · ANSWER E**

The loxapine inhalation label carries a boxed warning for bronchospasm and contraindicates use with a history of asthma, COPD, or other lung disease associated with bronchospasm. Pretreatment or a smaller dose does not remove the contraindication (A, B).

Back to question Contents

**BQ-493 · ANSWER B**

For violent or self-destructive behavior, orders last 4 hours for adults, 2 hours for ages 9 to 17, and 1 hour under age 9, and a face-to-face evaluation must occur within 1 hour. Standing and as-needed orders are prohibited (A), and restraint cannot be used for convenience (E).

Back to question Contents

**BQ-494 · ANSWER D**

The picture is neuroleptic malignant syndrome. Stop all antipsychotics and give intensive supportive care with cooling and fluids; lorazepam, dantrolene for severe cases, or ECT may be considered. B and E would worsen or miss the diagnosis. C is for serotonin syndrome, which is faster and hyperreflexic with clonus.

Back to question Contents

**BQ-495 · ANSWER A**

The British Association for Psychopharmacology guideline advises reassessing after 5 minutes for intravenous, 15 minutes for intramuscular, and 30 minutes for oral lorazepam, with a 50 percent fall in the rating-scale score as the positive response.

Back to question Contents

**BQ-496 · ANSWER C**

Benztropine injection is labeled for extrapyramidal disorders caused by neuroleptics, and for acute dystonic reaction 1 to 2 mL of the 1 mg/mL injection (1 to 2 mg) usually relieves it quickly. The other agents treat different conditions. Diphenhydramine is sometimes used, but its label does not list drug-induced extrapyramidal reactions.

Back to question Contents

**BQ-497 · ANSWER E**

A CIWA-Ar below 10 is mild; the ASAM guideline starts medication at moderate withdrawal (10 to 18), advises thiamine 100 mg daily for 3 to 5 days, and notes that mild, low-risk patients may be observed for up to 36 hours. A is wrong because antipsychotics are adjuncts only.

Back to question Contents

**BQ-498 · ANSWER B**

The court held that a therapist who determines, or should determine, that a patient poses a serious danger of violence to another must use reasonable care to protect the intended victim; steps may include warning the victim or police. A mistaken prediction does not erase the duty, and documentation alone does not discharge it (D, E).

Back to question Contents

**BQ-499 · ANSWER D**

The guideline states that the factors are associated with aggression in populations, that no evidence shows assessing them predicts aggression in an individual, and that no rating scale has been shown to; it recommends assessing the identified factors and documenting a risk estimate. A and C overstate what scales can do.

Back to question Contents

SUBJECT 23

# Consultation-Liaison Psychiatry

Answers BQ-500 to BQ-518

**BQ-500 · ANSWER B**

A short attention span, fever, tachycardia, and deliriogenic drugs point to delirium or withdrawal, so test attention and review the medication record first. Sertraline (A) and therapy (C) treat a disorder that has not been shown, and more lorazepam (D) is a recognized delirium risk factor.

Back to question Contents

**BQ-501 · ANSWER D**

The ability required rises with the seriousness of the consequences, which Appelbaum calls a sliding scale. A treats every decision alike, and B is wrong because refusing a recommended treatment does not by itself demand more.

Back to question Contents

**BQ-502 · ANSWER A**

Competence is a legal status determined by a court, and clinicians describe capacity for a specific decision. B is wrong because any treating physician can assess capacity, and D replaces the four abilities with a score.

Back to question Contents

**BQ-503 · ANSWER E**

In the Sessums review an MMSE above 24 lowered the likelihood of incapacity (LR 0.14) but did not exclude it. Her fixed false belief about the scan suggests a failure of appreciation that the MMSE does not test. A overstates the score.

Back to question Contents

**BQ-504 · ANSWER C**

The AMA Code directs physicians to a designated agent first and then to a family member or close associate, with substituted judgment, and advises ethics consultation when surrogates disagree. B skips the usual process, and D has no ethical basis.

Back to question Contents

**BQ-505 · ANSWER A**

Standing and as-needed restraint orders are prohibited. Adult orders for violent or self-destructive behavior last no more than 4 hours before renewal, and a face-to-face evaluation is needed within 1 hour. B is wrong because as-needed orders are not permitted at all, and E is wrong because a drug that manages behavior and is not standard treatment counts as a restraint.

Back to question Contents

**BQ-506 · ANSWER D**

NICE recommends the 4AT to assess for hypoactive delirium. The PHQ-9 and the Geriatric Depression Scale (A, E) assume the attention needed to answer reliably, and the MMSE and MoCA (B, C) do not by themselves diagnose delirium.

Back to question Contents

**BQ-507 · ANSWER B**

PADIS suggests propofol or dexmedetomidine rather than benzodiazepines for sedation of ventilated adults, a conditional recommendation. It suggests against haloperidol, atypical antipsychotics, ketamine, and statins for routine prevention, so C and D are wrong.

Back to question Contents

**BQ-508 · ANSWER C**

In 247 patients, symptom scores at day 3 were worse with risperidone and with haloperidol than with placebo, with more extrapyramidal effects, and survival was lower with haloperidol. A is the opposite of the result.

Back to question Contents

**BQ-509 · ANSWER E**

The ritonavir label raises quetiapine levels, suggests an alternative antiretroviral, and advises reducing quetiapine to one sixth of the dose if the combination is necessary. It does not contraindicate quetiapine (C), and it contraindicates triazolam (D).

Back to question Contents

**BQ-510 · ANSWER A**

Paroxetine is a strong inhibitor of CYP2D6, the enzyme that activates tamoxifen. In the cohort of 2,430 women, each 25-percentage-point increase in overlap was linked to a 24 percent higher risk of breast cancer death, and the other antidepressants showed no comparable increase.

Back to question Contents

**BQ-511 · ANSWER D**

NSAIDs and diuretics raise lithium levels, and the symptoms suggest toxicity. Hold lithium, obtain a level, and assess volume and kidney function. A and E add risk, and B delays care.

Back to question Contents

**BQ-512 · ANSWER B**

A positive Hoover sign is a positive sign of functional weakness. Explaining that the symptoms are real and functional, with coordinated care, avoids both dismissal (A) and repeated testing (C). A Cochrane review found small benefit from psychological therapies, mostly cognitive behavioral therapy.

Back to question Contents

**BQ-513 · ANSWER C**

The BAP guideline supports a lorazepam challenge of 1 to 2 mg intravenously with repeat rating at 5 minutes when the diagnosis is uncertain. A positive response makes catatonia more likely and predicts treatment response. A haloperidol challenge (B) could worsen catatonia.

Back to question Contents

**BQ-514 · ANSWER E**

The BAP guideline advises ECT when benzodiazepines do not produce remission, and a lorazepam trial is considered adequate at 16 mg a day or when side effects limit titration. Adding an antipsychotic (A) is controversial and not advised here.

Back to question Contents

**BQ-515 · ANSWER B**

Inducible clonus, hyperreflexia, sweating, and agitation after stacking serotonergic drugs fit serotonin syndrome. Stop the drugs and give supportive care and benzodiazepines. Physical restraint (C) can worsen hyperthermia, and bromocriptine and dantrolene (D, E) are not recommended.

Back to question Contents

**BQ-516 · ANSWER A**

For a temperature of 38.5 degrees C or more the label says to interrupt clozapine and obtain an absolute neutrophil count, and with a normal count it refers to its NMS section; fever and rigidity on clozapine call for an NMS evaluation. C treats neutropenia that the normal count excludes.

Back to question Contents

**BQ-517 · ANSWER D**

The AHA and American College of Cardiology Foundation statement calls for prompt action when QTc exceeds 500 msec or rises 60 msec or more: stop the offending drugs, replace potassium, give magnesium, and monitor. Continuing both drugs (A) leaves the risk in place.

Back to question Contents

**BQ-518 · ANSWER C**

The 2022 CDC guideline asks for particular caution when opioids and benzodiazepines are combined, offering naloxone, and avoiding abrupt discontinuation of long-term opioids. Stopping the opioid abruptly (A) is the error the guideline warns against.

Back to question Contents

# Forensic Psychiatry, Ethics, and the Law

Answers BQ-519 to BQ-534

**BQ-519 · ANSWER B**

*Addington v. Texas* set clear and convincing evidence as the constitutional minimum for indefinite civil commitment. Preponderance (A) was found too low, and the criminal standard (D) was not required.

Back to question Contents

**BQ-520 · ANSWER D**

The annotations allow psychiatrists to share expertise about psychiatric issues in general but call it unethical to offer a professional opinion about someone not examined and without proper authorization. A, B, and C all offer opinions about the candidate, and E treats a staff's approval as the required authorization when no examination occurred.

Back to question Contents

**BQ-521 · ANSWER A**

Commitment authorizes detention under the statute, not medication over objection, and a committed patient is not presumed incapable. Calm and cooperative, she presents no emergency, so assess capacity and use the state's non-emergency process. B and C are the tempting errors.

Back to question Contents

**BQ-522 · ANSWER E**

*Canterbury* measures materiality by what a reasonable person in the patient's position would likely consider significant, and it rejected medical custom as the sole measure (A). Prevalence thresholds (B) and the patient's questions alone (C) do not define the duty.

Back to question Contents

**BQ-523 · ANSWER C**

With no designated agent, the AMA opinion points to a family member or close associate under applicable law, applying substituted judgment based on the patient′s known wishes and values. Waiting (D) is impractical for a time-sensitive decision, and administrators (A) are not surrogates.

Back to question Contents

**BQ-524 · ANSWER B**

A PAD applies only when the person lacks capacity, and the SAMHSA guide notes involuntary treatment or emergencies can override it, though it should still inform the team. D reverses the trigger, and A ignores the document′s value.

Back to question Contents

**BQ-525 · ANSWER E**

Psychotherapy notes record or analyze the content of a counseling conversation and are kept separate. Medication monitoring, test results, treatment plan summaries, and session times are expressly excluded and belong in the chart.

Back to question Contents

**BQ-526 · ANSWER A**

A patient with capacity who objects blocks sharing under the family-involvement provision, but the clinician can still listen. Confirming a diagnosis (B) or medications (C) discloses protected information over his objection.

Back to question Contents

**BQ-527 · ANSWER D**

AAPL guidance advises a treating psychiatrist to avoid serving as the forensic expert for a current patient, because the roles conflict. Share only what the patient authorizes and point to an independent evaluator. B violates the rule against fees contingent on outcome.

Back to question Contents

**BQ-528 · ANSWER C**

*Sell* requires an important governmental interest, substantial likelihood that the drug restores competence without side effects that significantly impair the defense, necessity, and medical appropriateness. A, B, and D are not legal findings. Courts should first consider other grounds such as dangerousness.

Back to question Contents

**BQ-529 · ANSWER A**

The APA annotation states that sexual activity with a current or former patient is unethical. The AMA opinion allows narrower analysis for former patients, but the APA text is categorical, so waiting periods (B) and patient initiation (C) do not change the answer.

Back to question Contents

**BQ-530 · ANSWER E**

Under *Daubert* and Rule 702, the judge acts as gatekeeper, and the proponent must show it is more likely than not that the requirements are met. A removes the screening role of the court.

Back to question Contents

**BQ-531 · ANSWER B**

State boards generally expect licensure where the patient receives care, so confirm location and the relevant state's licensure and prescribing rules. A is the common assumption and is wrong.

Back to question Contents

**BQ-532 · ANSWER D**

The temporary rule extends the flexibilities through December 31, 2026 for DEA-registered practitioners who meet the listed conditions. It does not remove the need for registration (E), and the special registration rule was still a proposal in that notice.

Back to question Contents

**BQ-533 · ANSWER C**

Mental health professionals are often mandated reporters, the usual trigger is suspicion, and most states limit privilege so it cannot excuse silence. Honoring the mother's request (B) or waiting (A) would not satisfy the duty.

Back to question Contents

**BQ-534 · ANSWER B**

*Panetti* requires a rational understanding of the reason for execution, not just awareness of the State's stated reason, and *Madison* held that dementia can matter although memory loss alone does not bar execution. A is wrong because memory loss alone is insufficient, and D is the position *Panetti* rejected.

Back to question Contents

SUBJECT 25

# Professionalism, Communication, Cultural Care, and Systems of Practice

Answers BQ-535 to BQ-550

**BQ-535 · ANSWER B**

Teach-back asks the patient to restate the plan in her own words, and for a changed medicine AHRQ also recommends show-me. A is the tempting distractor because a yes is easy to give and tells you little. D checks the husband, not the patient.

Back to question Contents

**BQ-536 · ANSWER D**

In the SHARE approach, after exploring and comparing options the clinician assesses the patient's values and preferences. A removes him from the decision. B comes later, after the decision is reached.

Back to question Contents

**BQ-537 · ANSWER A**

A reflection that holds both sides of the ambivalence is the core MI skill. B and C argue for change, which tends to evoke defense; E is advice that dismisses his stress.

Back to question Contents

**BQ-538 · ANSWER E**

After setting up the interview, SPIKES asks about the patient's perception of the situation. A skips the perception and invitation steps. C is wrong because the patient decides who attends.

Back to question Contents

**BQ-539 · ANSWER C**

The HHS letter states that self-identified bilingual ability does not establish that someone is a qualified interpreter; the person must be proficient and interpret accurately and impartially. A is wrong for this reason. D is wrong because machine translation of critical content needs qualified review.

Back to question Contents

**BQ-540 · ANSWER A**

Measurement-based care uses a validated scale at each visit, reviewed with the patient and used to adjust treatment. In one trial of 120 outpatients, remission was 73.8 versus 28.8 percent with standard care. B stops short of using the score.

Back to question Contents

**BQ-541 · ANSWER C**

The APA guideline recommends assertive community treatment (1B) for patients whose poor service engagement leads to frequent relapse or social disruption. A and B are lower-graded or insufficient for this pattern.

Back to question Contents

**BQ-542 · ANSWER E**

Coordinated specialty care is the team-based model that RAISE found more effective than typical care and that the APA guideline recommends (1B). D lacks the family, work, and case management components.

Back to question Contents

**BQ-543 · ANSWER B**

The guideline recommends psychoeducation for all patients (1B) and suggests family interventions for patients with ongoing family contact (2B). The Cochrane review found fewer relapses with family intervention, so E is wrong.

Back to question Contents

**BQ-544 · ANSWER D**

The 988 Lifeline is free and confidential, reachable by call, text, or chat, and answered by more than 200 local centers. C is wrong because routing uses local centers.

Back to question Contents

**BQ-545 · ANSWER E**

Licensure follows the patient's physical location, and a safety plan requires an address and call-back number. A uses the clinician's location. D delays assessment of a possibly unsafe patient.

Back to question Contents

**BQ-546 · ANSWER A**

The CANDOR approach pairs prompt disclosure with investigation and support. B and C hide the error and delay care.

Back to question Contents

**BQ-547 · ANSWER D**

Under the contractor's guidance, only the clinician's own time on the date of the encounter counts, including qualifying non-face-to-face work. Staff time does not count (A, E).

Back to question Contents

**BQ-548 · ANSWER C**

The APA opposes conversion therapies as unsupported and potentially harmful, and supports psychotherapies that affirm orientation. A defers to a harmful request; E abandons the family.

Back to question Contents

**BQ-549 · ANSWER B**

"Neither for nor against" usually reflects a thin trial base, not evidence of harm or a recommendation against. A and E overread it.

Back to question Contents

**BQ-550 · ANSWER E**

The law requires comparable financial requirements and limits but does not itself require coverage, and does not directly govern Medicare or Medicaid. A and B overstate its scope.

Back to question Contents